

Title Page

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Protocol Title:

A Phase 3 Randomized, Double-Blind, Placebo-Controlled Study to Investigate the Efficacy and Safety of Once Weekly Eloralintide in Adult Participants with Obesity or Overweight, and Type 2 Diabetes (ENLIGHTEN-2)

Protocol Number: J3R-MC-YDAF

Amendment Number: YDAF(a)

Compound: Eloralintide

Brief Title:

Efficacy and Safety of Eloralintide in Participants with Obesity or Overweight, and Type 2 Diabetes

Study Phase: 3

Acronym: ENLIGHTEN-2

Sponsor Name: Eli Lilly and Company

Legal Registered Address: Indianapolis, Indiana, USA 46285

Regulatory Agency Identifier Number(s):

IND: 158631

EU trial number: 2025-523658-15-00

Approval Date: Protocol Amendment (a) Electronically Signed and Approved by Lilly on date provided below.

Document ID: VV-CLIN-198891

Medical Monitor Name and Contact Information will be provided separately.

Protocol Amendment Summary of Changes Table

DOCUMENT HISTORY	
Document	Date
Original Protocol	09-Sep-2025

Amendment [a]

This amendment is considered to be nonsubstantial.

Overall rationale for the amendment

The overall rationale for the changes implemented in this protocol amendment is to address feedback received from an internal safety committee.

Section # and Name	Description of Change	Brief Rationale
Section 5.2. Exclusion Criteria	Exclusion Criterion #16: removed “Exception: Short-term treatment with insulin for <14 days is allowed for certain clinical situations, such as, elective surgery, during hospitalization, hyperosmolar states, or acute illness.” Edited Exclusion Criterion #36 to remove “, except for the use of insulin for treatment of gestational diabetes or acute, temporary use of insulin (≤14 days), for example, for acute illness, hospitalization, elective surgery.” Edited Exclusion Criterion #40 to add “Note: Insulin therapy is not allowed within 90 days prior to Visit 1.”	Updated internal safety guidance.
Section 6.9.1. Initiation of new Antihyperglycemic Medications	Added “Insulin use for ≥14 consecutive days is prohibited.”	Updated internal safety guidance.
Section 6.9.2. Rescue Therapy for Severe, Persistent Hyperglycemia	Updated from “Participants should” to “Participants must” continue administering assigned study intervention, and added clarification “unless insulin is used as short-term rescue therapy.” Updated Recommended glycemic rescue therapy section to state that urgent use of short-term insulin in a hospital or closely monitored setting may be used as rescue therapy, and	Updated internal safety guidance.

Section # and Name	Description of Change	Brief Rationale
	added respective instructions to temporarily interrupt study intervention. Removed “Short-term treatment with insulin not as rescue therapy” section.	
Section 6.9.3. Prohibited Medications	Updated Note regarding short-term insulin use to clarify that it is allowed for urgent use.	Updated internal safety guidance.

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1. Protocol Summary

1.1. Synopsis

Protocol Title:

A Phase 3 Randomized, Double-Blind, Placebo-Controlled Study to Investigate the Efficacy and Safety of Once Weekly Eloralintide in Adult Participants with Obesity or Overweight, and Type 2 Diabetes (ENLIGHTEN-2)

Brief Title:

Efficacy and Safety of Eloralintide in Participants with Obesity or Overweight, and Type 2 Diabetes

Regulatory Agency Identifier Number(s):

IND: 158631

EU trial number: 2025-523658-15-00

Rationale:

This Phase 3 multicenter, randomized, double-blind study is designed to assess the safety and efficacy of once weekly eloralintide treatment compared with placebo in participants with obesity or overweight, with T2D.

Objectives, Endpoints, and Estimands:

Objectives	Endpoints
Primary	
To demonstrate that eloralintide 3 mg, 6 mg and/or 9 mg QW is superior to placebo for change in: body weight	From baseline to Week 64 percent change in body weight
Key Secondary	
To demonstrate that eloralintide 1.5 mg QW is superior to placebo for change in: body weight	From baseline to Week 64 percent change in body weight
To demonstrate that eloralintide 3 mg, 6 mg, and/or 9 mg QW is superior to placebo for change in the following: body weight	From baseline to Week 64 - the achievement (Y/N) of body weight reduction of ≥5% ≥10% ≥15% ≥20%
- body composition via DXA - central adiposity measurement	- percent change in total body fat mass - change in waist circumference (cm)

Objectives	Endpoints
glycemic control	- change in HbA1c (%) - change in fasting glucose (mg/dL) - change in HbA1c(%) in participants with baseline HbA1c $\geq 7\%$
To demonstrate that pooled eloralintide 1.5 mg, 3 mg, 6 mg, and 9 mg QW is superior to placebo for change in the following: - lipid parameters - blood pressure	From baseline to Week 64 - percent change in - triglycerides - non-HDL cholesterol - change in SBP (mmHg)

Abbreviations: DXA = dual energy x-ray adiposity; HbA1c = hemoglobin A1c; HDL = high density lipoprotein; QW = weekly; SBP = systolic blood pressure; Y/N = yes/no.

Estimands

The study will use two estimands for the primary and key secondary objectives: a treatment-regimen estimand for the treatment policy strategy, and an efficacy estimand for the hypothetical strategy. These estimands have distinct interpretations and serve different purposes.

Treatment policy strategy

The occurrence of the ICE is considered irrelevant in defining the treatment effect of interest; the values for the variable of interest are used regardless of whether the ICE occurs.

Hypothetical strategy

A scenario is envisaged in which the ICE would not occur. The value of the variable to reflect the clinical question of interest is the value that the variable would have taken in the hypothetical scenario defined.

Treatment-regimen estimand

The primary clinical question of interest is as follows:

What is the treatment difference in the percent change in body weight from baseline at 64 weeks between eloralintide 3 mg, 6 mg and 9 mg and placebo, as an adjunct to healthy nutrition and physical activity, in participants with YDAF study disease/conditions criteria regardless of adherence to study intervention or initiation of prohibited weight management treatments?

Rationale for the estimand: This estimand aims to evaluate the efficacy of eloralintide that reflects the real-life behavior of the target population.

The “treatment regimen” estimand is described by the following attributes:

- Population:** Participants with YDAF study disease/conditions. Further details can be found in Sections 5 and 9.

- **Endpoint:** Percent change from baseline in body weight at 64 weeks.
- **Treatment condition:** The randomized treatment with allowance for potential dose interruptions and modifications, regardless of adherence to study intervention or initiation of prohibited weight management treatments.
- **Intercurrent events:** No ICEs are defined since treatment adherence, and the initiation of prohibited weight management treatments is a part of the treatment condition.
- **Population-level summary and treatment effect of interest:** Difference in mean percent change from baseline in body weight at 64 weeks between eloralintide and placebo.

Efficacy estimand

The clinical question of interest is as follows:

What is the treatment difference in the percent change in body weight from baseline at 64 weeks between eloralintide 3 mg, 6 mg and 9 mg and placebo, as an adjunct to healthy nutrition and physical activity, in participants with YDAF study disease/conditions if they would remain on their randomly assigned treatment for 64 weeks and would not initiate prohibited weight management treatments?

Rationale for the estimand: This estimand aims to evaluate the efficacy of eloralintide under the ideal condition that all participants would adhere to the randomly assigned study intervention without being confounded by the initiation of prohibited weight management treatments.

The “efficacy” estimand is described by the following attributes:

- **Population:** Participants with YDAF study disease/conditions. Further details can be found in Sections 5 and 9.
- **Endpoint:** Percent change from baseline in body weight at 64 weeks.
- **Treatment condition:** The randomized treatment with allowance for potential dose interruptions and modifications.
- **Intercurrent events:**

Intercurrent Event	Handling Strategy
permanent discontinuation of study intervention	hypothetical strategy
Initiation of weight management treatments	hypothetical strategy

- **Population-level summary and treatment effect of interest:** Difference in mean percent change from baseline in body weight at 64 weeks between eloralintide and placebo.

Overall Design:

Study J3R-MC-YDAF (YDAF) is a Phase 3 randomized, double-blind, placebo-controlled study to investigate the efficacy and safety of subcutaneously administered once weekly eloralintide in adult participants with obesity or overweight, and T2D.

Brief Summary:

Study YDAF is designed to investigate the efficacy and safety of eloralintide administered SC QW at doses of 1.5 mg, 3 mg, 6 mg, or 9 mg for 64 weeks compared with placebo administered QW for body weight reduction in participants with overweight or obesity and T2D.

This study consists of 3 periods:

- up to a 35 day screening period
- treatment: 64 weeks
 - dose-escalation period: 12 weeks, and
 - treatment maintenance period: 52 weeks, and
- post-treatment follow-up: 6 weeks.

The visit frequency is every 4 to 12 weeks.

Study Population:

In general, an individual may participate in this study if they

- are at least 18 years of age and the legal age of consent in the jurisdiction in which the study is taking place at the time of signing the informed consent
- have a BMI ≥ 27 kg/m²
- have a clinical diagnosis of T2D for at least 6 months prior to screening, and
- have a history of at least 1 self-reported unsuccessful dietary effort to reduce body weight.

Number of Participants:

Approximately 1,035 participants will be randomly assigned to study intervention.

Participants will be randomized in a 2:1:2:2:2 ratio to treatments arms of placebo, eloralintide 1.5 mg, eloralintide 3 mg, eloralintide 6 mg, and eloralintide 9 mg.

Intervention Groups:

Intervention Name	Eloralintide	Eloralintide Placebo
Dosage Level(s)	1.5 mg 3 mg 6 mg 9 mg	Placebo
Route of Administration	Subcutaneous using a PFS	Subcutaneous using a PFS
Frequency of Administration	Once weekly	Once weekly
Authorized as defined by EU Clinical Trial Regulation	Not authorized in EU	Not authorized

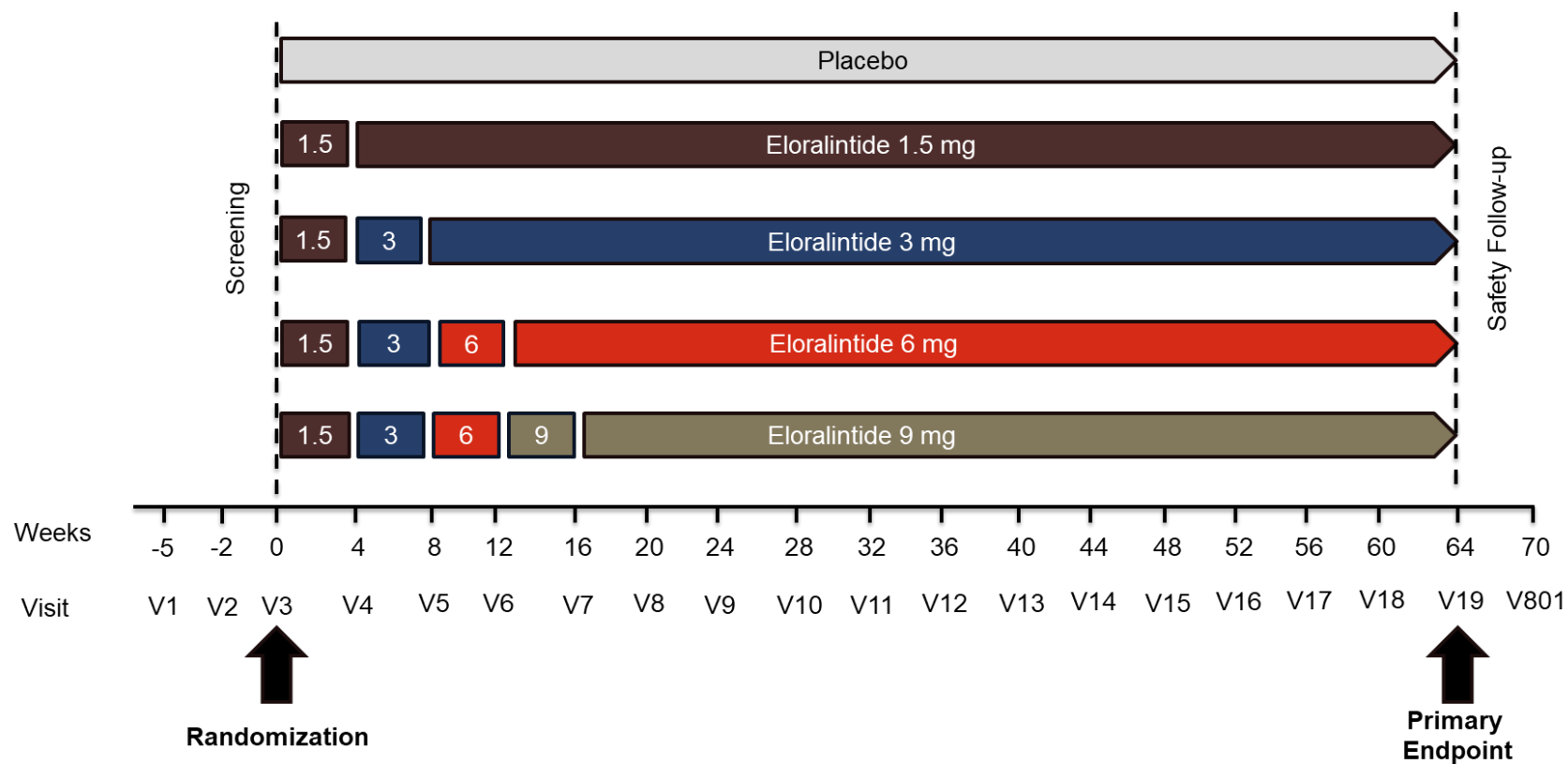
Abbreviations: PFS = prefilled syringe.

Ethical Considerations of Benefit/Risk:

The safety and efficacy profile seen to date for eloralintide supports the overall benefit-risk profile for participants in this study. Considering the measures taken to minimize risk to participants in this study, the potential risks identified in association with eloralintide are outweighed by the anticipated benefits that may be afforded to participants living with obesity or overweight and T2D.

Data Monitoring Committee: Yes

1.2. Schema



1.3. Schedule of Activities

The SoA described in this section should be followed for all participants enrolled in Study YDAF.

Visit information

Visit procedures may be conducted over more than 1 day if all activities are completed within the allowed visit tolerance period of each visit. Participants should not initiate first dose of study intervention or escalate study intervention until all required activities for that visit are complete.

Fasting visits

On all clinic visits, except PK only visits, remind participants to report to the site in a fasting condition. Fasting is defined as a period of approximately 8 hours without eating and drinking, except water, or performing any significant physical activity. Participants should not take their study intervention prior to the fasting visit. If a participant is adversely affected by the fasting condition, they are allowed to eat; however, specific study procedures need to be completed while fasting.

If a participant attends a fasting visit in the non-fasting state, reschedule a visit to complete the required fasting activities within the allowable visit interval.

Screening

All screening activities must take place within the 35-day window.

Participants should not attend Visit 3 until results from Visit 1 and Visit 2 are received and initial assessments of eligibility are completed. At Visit 2, screening laboratory results will be reviewed to confirm eligibility. At Visit 2 and randomization, all screening results will be reviewed to confirm eligibility.

Baseline fundus photography should be evaluated by a qualified eye care professional once initial assessments of eligibility are completed. Fundus photographs obtained per the protocol requirements and within 90 days before Visit 2 may be used for eligibility determination. Assessment of results is required prior to randomization at Visit 3 to confirm eligibility for enrollment.

Visit 3 (randomization to treatment)

Visit 3 should be scheduled as soon as all screening activities have been performed and results are available to assess eligibility. Participants will be randomly assigned to the treatment arms at Visit 3 and will receive their first dose of study intervention after completing all baseline assessments including labs.

Telehealth or remote visits

Telehealth visits may be by telephone or other technology.

PK only

The only procedure performed for these visits is PK sample collection.

Early discontinuation visits

ED visits will be completed by participants permanently discontinuing early from the study. These participants should also complete posttreatment follow-up visit(s). The SoA reflects the procedures that may occur during these visits.

If the participant is discontinuing at the time of a scheduled visit, perform that visit as an ED visit. Participants who withdraw from the study after signing the informed consent but who have not taken a dose of study intervention do not need to complete ED procedures.

Unscheduled visits

An unscheduled visit may occur per investigator discretion for clinical oversight. The study site will determine if fasting status is needed prior to the visit. Concomitant medications, AEs, and endpoint events are required and should be recorded in the CRF.

Visit 801

A posttreatment safety follow-up visit will occur approximately 6 weeks following the last treatment period visit or final treatment visit. All participants are required to complete the safety follow-up visit. The SoA reflects the procedures that may occur during these visits.

1.3.1. Schedule of Activities– Period I Screening and Period II Dose Escalation

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Consent and demographics							
Informed consent	X						ICF must be signed before any protocol specific tests/procedures are performed for main study and substudy, if applicable.
Inclusion/exclusion criteria; confirmation of eligibility	X	X	X				Confirm inclusion and exclusion criteria prior to randomization and administration of first dose of study intervention.
Demographics	X						Includes year of birth, sex, ethnicity (where permissible) and race.
Pre-existing conditions, family history, and medical history, including relevant surgical history	X						Collect all ongoing conditions and relevant past surgical and medical history, including menopausal status if AFAB.
Prespecified medical history (indication and history of interest)	X						Should include, but not be limited to, collecting diagnosis of obesity or overweight-related health problems.
Prior treatments for indication	X						Include treatments for obesity-related conditions, including T2D.
Substance use (alcohol, tobacco, recreational drug use)	X						
Education level	X						
Treatment goal assessment			X				Participants will be asked to provide a personal goal which could include BMI, weight, and other weight-related comorbidities.

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Concomitant medications	X	X	X	X	X	X	Include doses required for antihypertensive, antihyperglycemic, and lipid-lowering medications. Refer to Section 6.9.1, 5.3.1, and 5.3.2 for additional details.
Adverse events (AEs)	X	X	X	X	X	X	Collect additional data for AESIs and other safety topics. See Section 8.3.
Physical Evaluation							
Height	X						
Weight	X		X	X	X	X	Must be measured in a fasting state. See Sections 10.7.
Waist circumference			X	X	X	X	See instructions in Section 10.7.
Vital signs	X	X	X	X	X	X	Collect 3 measures of sitting vital signs (BP and pulse rate) before obtaining ECG and before collection of blood samples for laboratory testing. See Section 8.2.2.
Physical examination	X						See Section 8.2.1. Visit 1 requires a complete physical examination. At all other visits, additional symptom directed physical assessment may be conducted at the discretion of the investigator.
12-lead ECG (central)	X		X				Collect ECG before blood sample collection for laboratory testing if these procedures occur at the same visit. See Section 8.2.3 for details.
Fundus Photography and Further Ophthalmological Assessment							
Fundus photography for all participants		X					See Section 8.2.11.

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Participant Diary (electronic)							
Provide diary device, instruct in-use		X					An electronic diary device will be provided and training conducted. Participants will use this device to record hypoglycemic events, 7-point SMBG, and weekly study medication injections (Section 6.1). All training should be repeated as needed to encourage participant compliance.
Assist in the study medication injection diary			X				Site personnel should assist the participant in completing the first entry of the study medication injection diary.
Review diaries			X	X	X	X	Includes weekly study medication injection diary (Section 6.1), hypoglycemia diary, 7-point SMBG diary and other data as applicable. Site personnel should review study medication injection diary for completeness at every visit. Provide retraining to participant and help them catch up during visit if data entry is behind
Participant Education and Supplies							
Diabetes education (includes BG meter dispensing and SMBG training)		X					
Dispense BG meter/supplies as needed		X	X	X	X	X	

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Remind participants about 7-point SMBG		X					Participant is required to collect two 7-point SMBGs on non-consecutive days within 2 weeks prior to the next visit. A 7-point SMBG consists of measurements before and 2 hr after each of 3 main meals within the same day and at bedtime
Review 7-point SMGB values collected in the diary			X				
Lifestyle counseling			X	X	X	X	Lifestyle counseling may occur on a separate day from the rest of that visit's study procedures. Counseling may be delivered by telehealth. See Section 5.3.
Injection training			X				Repeat as needed.
Participant Log (Paper)							
Nutrition and physical activity log			X	X	X	X	Includes dispensing, collection, and review as applicable. Monitor for and encourage adequate nutritional intake and hydration throughout the study. See Section 5.3.1 and 5.3.2.
Patient-reported Outcomes (Electronic)							
SF-36v2			X				
IWQOL-Lite-CT			X				
EQ-5D-5L			X				
PGIS Physical Function Weight			X				
CoEQ			X				
FNQ			X				

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Mental Health Assessments							
Patient Health Questionnaire-9 (PHQ-9)	X		X		X		Collect AEs before the PHQ-9 collection. See Sections 8.2.7 and 8.2.7.1 .
C-SSRS Screening/ Baseline (Category Version)	X						Collect AEs before the C-SSRS collection.
C-SSRS Since Last Assessed (Clinical Categories Version)		X	X	X	X	X	Collect AEs before the collection of the C-SSRS.
Laboratory Tests and Sample Collection							
Hematology	X		X	X		X	
Hemoglobin A1c (HbA1c)	X		X			X	
Clinical chemistry	X		X	X	X	X	
Lipid panel	X		X		X		
Urinalysis	X		X			X	
Serum pregnancy	X						For IOCBP only. Definition in Section 10.4.1 . If the initial serum pregnancy test at V1 is performed more than 30 days before the first dose (Visit 3), a second negative serum test by the central lab, confirmed by the Principal Investigator, must be completed before Visit 3 and dosing. See section 8.2 for details.
Urine pregnancy test (local)			X	X	X	X	For IOCBP only. Definition in Section 10.4.1 . Week 0 (Visit 3): Results must be available before the first administration of study intervention. Perform additional pregnancy tests at any time during the study if needed. See Section 8.2.5 .

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Follicle-stimulating hormone (FSH)	X						Collect FSH only as required to confirm postmenopausal state. See Section 10.4.
Thyroid-stimulating hormone (TSH)	X						
Insulin			X		X		
C-peptide			X		X		
Free Fatty Acids			X		X		
Apolipoprotein B (ApoB)			X		X		
Apolipoprotein C-3			X		X		
Intact Proinsulin			X		X		
hsCRP			X		X		
Urinary albumin/creatinine ratio (UACR)			X				
Estimated glomerular filtration rate (eGFR)	X		X		X		Calculated by the central lab using CKD-EPI creat-cystC (2021 version).
Cystatin-C	X		X		X		
Pancreatic Amylase			X		X		
Renin			X				
Aldosterone			X				
Lipase			X		X		
Micronutrient panel			X				Micronutrient deficiencies identified via laboratory tests should be repleted as clinically appropriate. See Section 5.3.2.
Immunogenicity (ADA) samples (predose)			X	X			Collect additional unscheduled samples if systemic drug hypersensitivity reactions occur (see Section 8.3.5.9 and 10.2.1). Collect immunogenicity and PK samples for immunogenicity before dosing

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
PK sample (predose)			X	X			PK sample to be taken prior to study intervention administration. Site should confirm that participant has accurately entered the dose date and time in their diary.
PK (2 to 6hr post dose)			X				This sample should be taken on the same day after dosing of study intervention.
PK Random (at any time during visit)						X	PK sample can be taken at any time during the visit.
Stored Samples							
Genetics sample			X				Collect sample at or after the specified visit.
Exploratory biomarker samples			X				
Epigenetics sample			X				
Imaging							
Schedule DXA	X						
DXA Scan		X					See Section 8.1.2. Perform baseline scan between Visits 2 and 3 after all eligibility criteria are confirmed.

	Period I – Screening		Period II – Dose Escalation Treatment				Comments
Visit Number	1	2	3	4	5	6	
Weeks from randomization	-5	-2	0	4	8	12	
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	
Visit Detail	F		F	F	F	F	F = fasting T = telehealth
Randomization and Dosing							
Register visit with IWRS	X	X	X	X	X	X	
Randomization to treatment via IWRS			X				
Dispense study intervention via IWRS			X	X	X	X	
Observe participant administer study intervention			X				
Dispense supplies to participant			X	X	X	X	Ancillary supplies: dispense as needed.
Participant returns study interventions supplies				X	X	X	
Assess study intervention(s) compliance				X	X	X	See Section 6.5.

Abbreviations: ADA = anti-drug antibodies; AE = adverse event; ApoB = apolipoprotein B; AESI = adverse events of special interest; BG = blood glucose; BMI = body mass index; BP = blood pressure; CKD-EPI = CKD Epidemiology Collaboration; CoEQ = control of eating questionnaire; C-SSRS = Columbia-Suicide Severity Rating Scale; ECG = electrocardiogram; FNQ = food noise questionnaire; DXA = dual-energy x-ray absorptiometry eGFR = estimated glomerular filtration rate; FSH = follicle stimulating hormone; HbA1c = hemoglobin A1c; hsCRP = high sensitivity C-reactive protein; ICF = informed consent form; IOCBP = individuals of childbearing potential; IWQOL-Lite-CT = Impact of Weight on Quality of Life-Lite Clinical Trials Version; IWRS = interactive web response system; PGIS = Patient Global Impression of Status; PHQ-9 = patient health questionnaire 9; PK = pharmacokinetics, sF-36v2 = Short Form 36 Version 2 Health Survey, Acute Form; SMBG = self-monitoring blood glucose; T2D = type 2 diabetes; TSH = thyroid stimulating hormone; UACR = urinary albumin creatinine ratio.

1.3.2. Schedule of Activities– Period III Maintenance

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Consent and Demographics																		
Substance use (alcohol, tobacco, recreational drug use)	X							X							X	X		
Treatment goal assessment															X			Participants will be asked to provide a personal goal which could include BMI, weight, and other weight-related comorbidities
Concomitant medications	X	X	X	X		X	X	X		X	X	X	X	X	X	X	X	Include doses required for antihypertensive, antihyperglycemic, and lipid-lowering medications. Refer to Section 6.9.1, 5.3.1, and 5.3.2 for additional details.
Assess medication intensity (dyslipidemia, hypertension, hyperglycemia)				X											X	X		As applicable for participants receiving these medications. Evaluate the changes in treatment intensity, within each therapeutic area, from randomization until the time of evaluation.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Adverse events (AEs)	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X	Additional data are collected for certain AEs. Collect additional data for AESIs and other safety topics. See Section 8.3.
Physical evaluation																		
Weight	X			X				X				X			X	X	X	Must be measured in the fasting state. See Section 10.7.
Waist circumference	X			X				X				X			X	X	X	See instructions in Section 10.7.
Vital signs	X			X				X				X			X	X	X	Collect 3 measures of sitting vital signs (BP and pulse rate) before obtaining ECG and before collection of blood samples for laboratory testing. See Section 8.2.2.
Physical examination															X	X		See Section 8.2.1. Visits 19 (or ED if applicable) require a complete physical examination. At all other visits, symptom directed physical assessment can be conducted at the discretion of the investigator.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
ECG 12-lead (central)	X							X							X	X		Collect ECG before blood sample collection for laboratory testing if these procedures occur at the same visit. See Section 8.2.3.
Fundus photography and further ophthalmological assessment (if applicable)																		Repeat at ED visit if more than 6 months from last assessment.
Fundus photography for participants with retinopathy present at baseline				X								X				X		Repeat the fundus photography every 6 months. See Section 8.2.11 for additional requirements.
Fundus photography for participants without retinopathy present at baseline												X				X		Repeat fundus photography annually. See Section 8.2.11 for additional requirements.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Participant diary																		
Review diaries	X	X	X	X		X	X	X		X	X	X	X	X	X	X	X	Includes weekly study intervention injection diary (Section 6.1), hypoglycemia diary, 7-point SMBG diary and other data as applicable. Site personnel should review study medication injection diary for completeness at every visit. Provide retraining to participant and help them catch up during visit if data entry is behind.
Diary Device Return																X	X	Participants will return their device during their last visit.
Participant education and supplies																		
Dispense BG meter/supplies as needed	X			X				X				X			X			

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Remind participants about 7-point SMBG			X											X				Participant is required to collect two 7-point SMBGs on non-consecutive days within 2 weeks prior to the next visit. See Section 8.2.10 .
Review 7-point SMGB values collected in the diary				X											X			
Lifestyle counseling	X	X		X		X		X		X		X	X		X	X		Lifestyle counseling may occur on a separate day from the rest of that visit's study procedures. Counseling may be delivered by telehealth. See Section 5.3 .
Participant log (paper)																		
Nutrition and physical activity log	X			X				X				X			X	X		Includes dispensing, collection, and review as applicable. Monitor for and encourage adequate nutritional intake and hydration throughout the study. See Section 5.3.1 and 5.3.2 .

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Patient -reported outcomes (electronic)																		
SF-36v2 acute form	X							X							X	X		
IWQOL-Lite-CT	X							X							X	X		
EQ-5D-5L	X							X							X	X		
PGIS Physical Function Weight	X							X							X	X		
PGIC Physical Function Weight	X							X							X	X		
CoEQ	X							X							X	X		
FNQ	X							X							X	X		
Mental health assessments																		Mental health assessments should be completed during the visits.
Patient Health Questionnaire-9 (PHQ-9)	X		X			X		X			X	X		X	X	X	X	Collect AEs before the PHQ-9 collection. See Sections 8.2.7 and 8.3.3.4 .
C-SSRS Since Last Assessed (Clinical Categories Version)	X	X	X	X		X	X	X		X	X	X	X	X	X	X	X	Collect AEs before the collection of the C-SSRS.
Laboratory test and sample collection																		
Hematology				X				X				X			X	X	X	
HbA1c				X				X				X			X	X	X	
Clinical Chemistry	X			X				X				X			X	X	X	

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Lipid panel	X			X				X							X	X	X	
Urinalysis	X			X				X				X			X	X	X	
Urine pregnancy (local)	X	X	X	X				X				X			X	X	X	For IOCBP only. Definition in Section 10.4.1. Perform additional pregnancy tests at any time during the study if there is clinical suspicion of pregnancy, or as required by local law or regulation See Section 8.2.5.
Remind Urine Pregnancy (at home test)						X	X			X	X		X	X			X	For IOCBP only. Recommend participants take additional home UPT 4 weeks after V801. See Section 8.2.5.
Insulin	X			X				X							X	X	X	
C-peptide	X			X				X							X	X	X	
Free Fatty Acids	X			X				X							X	X	X	
Apolipoprotein B (ApoB)	X														X	X	X	
Apolipoprotein C-3 (ApoC3)	X														X	X	X	
Intact Proinsulin	X														X	X	X	

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
C-Reactive Protein, high-sensitivity (hsCRP)	X							X							X	X	X	
Urinary albumin/creatinine ratio (UACR)	X			X				X				X			X	X	X	
Estimated glomerular filtration rate (eGFR)	X			X				X				X			X	X	X	Calculated using CKD-EPI _{creat-cystC} (2021 version).
Cystatin-C	X			X				X				X			X	X	X	
Pancreatic Amylase	X			X				X				X			X	X	X	
Renin	X			X				X							X	X		
Aldosterone	X			X				X							X	X		
Lipase	X			X				X				X			X	X	X	
Micronutrient panel	X							X							X	X		Micronutrient deficiencies identified via laboratory tests should be replenished as clinically appropriate. See Section 5.3.2.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Immunogenicity (ADA) samples (predose)	X			X								X				X	X	Collect immunogenicity and PK samples for immunogenicity before dosing. Collect additional unscheduled samples if systemic drug hypersensitivity reactions occur (see Section 8.3.5.9 and 10.2.1).
PK sample (predose)	X			X				X				X						PK sample to be taken predose, followed by study intervention administration. Site should confirm that participant has accurately entered the dose date and time in their diary.
PK (2 to 6hr post dose)	X																	This sample should be taken on the same day after the W16 dose.
PK (12 to 48hr postdose)					X													This would be an additional PK sampling only visit at 12 to 48hr after the W28 dose.
PK (48 to 120hr postdose)									X									This would be an additional PK sampling only visit at 48 to 120hr after the W40 dose.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
PK Random (at any time during visit)															X	X	X	PK sample can be taken at any time during the visit.
Stored samples																		
Exploratory biomarker samples	X			X				X				X			X	X		
Epigenetics sample															X			
Imaging																		
Schedule DXA			X											X				
DXA Scan				X											X	X		Post baseline DXA scans may be performed within ± 7 days of Visit 10 and Visit 19 or ED. See Section 8.1.2.
Randomization and Dosing																		
Register visit with IWRS	X	X	X	X				X				X			X	X		For Unscheduled Visits: Only needed when dispensing study intervention.
Dispense study intervention via IWRS	X	X	X	X				X				X						For Unscheduled Visits: Only needed when dispensing study intervention.
Dispense supplies to participant	X			X				X				X						Ancillary supplies: dispense as needed.

	Period III – Maintenance Treatment																	Comments
Visit Number	7	8	9	10	-	11	12	13	-	14	15	16	17	18	19	ED	V801	
Weeks from randomization	16	20	24	28	28	32	36	40	40	44	48	52	56	60	64		6 wks after last tx dose	
Visit Interval Tolerance (+/- days)	3	3	3	3	-	3	3	3	-	3	3	3	3	3	3		5	
Visit Detail	F			F	PK	T	T	F	PK	T	T	F	T	T	F	F	F	F = fasting T = telehealth
Participant returns study intervention supplies	X	X	X	X				X				X			X	X		See Section 6.5.
Assess study intervention(s) compliance	X	X	X	X		X	X	X		X	X	X	X	X	X	X		See Section 6.5.

Abbreviations: ADA = anti-drug antibodies; AE = adverse event; ApoB = apolipoprotein B; ApoC3 = apolipoprotein C; AESI = adverse events of special interest; BG = blood glucose; BMI = body mass index; BP = blood pressure; CKD-EPI = CKD Epidemiology Collaboration; CoEQ = control of eating questionnaire; C-SSRS = Columbia-Suicide Severity Rating Scale; ECG = electrocardiogram; ED = early discontinuation; DXA = dual-energy x-ray absorptiometry eGFR = estimated glomerular filtration rate; FNQ = food noise questionnaire; HbA1c = hemoglobin A1c; hsCRP = high sensitivity C-reactive protein; IOCBP = individuals of childbearing potential; IWQOL-Lite-CT = Impact of Weight on Quality of Life-Lite Clinical Trials Version; IWRS = interactive web response system; PGIC = Patient Global Impression of Change; PGIS = Patient Global Impression of Status; PHQ-9 = patient health questionnaire 9; PK = pharmacokinetics, sF-36v2 = Short Form 36 Version 2 Health Survey, Acute Form; SMBG = self-monitoring blood glucose; T2D = type 2 diabetes; tx = treatment; UACR = urinary albumin creatinine ratio; UPT = urine pregnancy test; V = visit; W = week; Wks = weeks.

2. Introduction

2.1. Study Rationale

This Phase 3 multicenter, randomized, double-blind study is designed to assess the safety and efficacy of once weekly eloralintide treatment compared with placebo in participants with obesity or overweight, with T2D.

2.2. Background

2.2.1. Obesity or Overweight

The ongoing global obesity epidemic is associated with multiple comorbidities, including

- rising incidence of T2D
- increased risk for premature death, and
- increased risk for some cancers (AMA 2013; CSAPH 2013; Lauby-Secretan et al. 2016).

Although lifestyle intervention is a cornerstone of treatment of obesity and overweight, efforts focused on reduced calorie diets and increased physical activity have been shown to prompt physiological counterregulatory mechanisms that make it difficult to achieve and maintain weight reduction (Leibel et al. 1995; Sumithran et al. 2011). Lifestyle interventions alone are typically insufficient to achieve sustainable weight reduction in a majority of patients with obesity (Dombrowski et al. 2014).

Bariatric surgery has been considered the most potent clinical intervention for durable weight reduction. However, only about 1% of eligible patients with obesity are treated with surgical intervention (ASMBS 2018; Campos et al. 2020). Barriers to bariatric surgery include limited access, limited reversibility, and the potential risk of periprocedural complications.

For many individuals with obesity or overweight, pharmacological management may be a preferred or medically indicated option. With the advent of injectable incretin-based therapies, efficacious medications, with acceptable safety profile, are increasingly available for people with obesity or overweight (with at least one weight-related medical condition). These include the GLP-1 receptor agonists liraglutide 3.0 mg (Saxenda® package insert 2025) and semaglutide 2.4 mg (Wegovy® package insert 2025) and the GIP/GLP-1 receptor agonist tirzepatide (Zepbound® package insert 2025).

To expand treatment options for individuals living with obesity or overweight, Eli Lilly and Company (Lilly) is developing eloralintide.

2.2.2. Obesity and Type 2 Diabetes

T2D is a metabolic condition characterized by impaired glycemic control, caused by increased insulin resistance and progressive beta-cell failure, and consequently inadequate insulin secretion. Excess adiposity is also considered a core driver of T2D pathophysiology.

T2D is frequently associated with comorbidities, such as:

- overweight or obesity
- hypertension

- increased blood lipoprotein concentrations, and
- a higher risk for macro- and microvascular complications.

There is strong and consistent evidence that management of obesity and overweight is beneficial in the treatment of T2D and other weight-related comorbidities (American Diabetes Association 2025a; Davies et al. 2022). Modest and sustained weight reduction can produce clinically meaningful improvements in blood glucose, HbA1c, and triglycerides. Greater weight reductions produce even greater metabolic benefits, including reductions in blood pressure, improvements in low-density lipoprotein and high-density lipoprotein cholesterol, and reductions in the need for medications to control blood glucose, blood pressure, and lipids, and may even result in achievement of glycemic goals in the absence of glucose-lowering agent use in some patients (UKPDS Group 1990; Pastors et al. 2002; Wing et al. 2011; Rothberg et al. 2017; American Diabetes Association 2023). Therefore, for patients with T2D who have obesity or overweight, efforts targeting weight reduction, including lifestyle, medication, and surgical interventions are recommended (Davies et al. 2022).

2.2.3. Eloralintide Treatment and Unmet Needs

Eloralintide is a selective, long-acting amylin receptor agonist being developed as a once-weekly SC injection for the treatment of obesity and overweight in individuals with and without T2D.

Compared to people without diabetes, patients with T2D and obesity or overweight often have a less favorable weight reduction response to pharmacologic treatments for weight management (Jastreboff et al. 2022; Garvey et al. 2023; Bays 2023). Therefore, there remains an unmet medical need for pharmacologic treatments that are safe, well-tolerated, and highly efficacious in patients with obesity or overweight who have T2D.

2.3. Benefit/Risk Assessment

Detailed information about the known and expected benefits and risks and reasonably expected adverse events of eloralintide may be found in the Investigator's Brochure.

2.3.1. Risk Assessment

To date, the described potential risks associated with eloralintide have been similar to those of newly approved obesity management medications. The most commonly reported TEAEs observed in eloralintide clinical studies were

- decreased appetite
- headache
- fatigue
- diarrhea, and
- nausea.

2.3.2. Benefit Assessment

The potential benefits from participation in this study include body weight reduction and continued expert medical care for the study duration.

Participants may benefit by receiving personal health information, routine safety assessments, healthy nutrition and physical activity counseling, and frequent engagement with healthcare professionals during the study, including opportunities for lifestyle counseling and support.

Assessment of potential benefits can be found in Section 2 (Introduction) of the IB.

2.3.3. Overall Benefit Risk Conclusion

The safety and efficacy profile seen to date for eloralintide supports the overall benefit risk for participants in this study.

Given the measures taken to minimize participant risk in this study, the potential risks associated with eloralintide are justified by the anticipated benefits for individuals living with obesity or overweight with T2D.

3. Objectives, Endpoints, and Estimands

Objectives	Endpoints
Primary	
To demonstrate that eloralintide 3 mg, 6 mg and/or 9 mg QW is superior to placebo for change in: body weight	From baseline to Week 64 percent change in body weight
Key Secondary	
To demonstrate that eloralintide 1.5 mg QW is superior to placebo for change in: body weight	From baseline to Week 64 percent change in body weight
To demonstrate that eloralintide 3 mg, 6 mg, and/or 9 mg QW is superior to placebo for change in the following: - body weight - body composition via DXA - central adiposity measurement - glycemic control	From baseline to Week 64 - the achievement (Y/N) of body weight reduction of ≥5% ≥10% ≥15% ≥20% - percent change in total body fat mass - change in waist circumference (cm) - change in HbA1c (%) - change in fasting glucose (mg/dL) - change in HbA1c (%) in participants with baseline HbA1c ≥7%
To demonstrate that pooled eloralintide 1.5 mg, 3 mg, 6 mg, and 9 mg QW is superior to placebo for change in the following:	From baseline to Week 64
- lipid parameters - blood pressure	- percent change in - triglycerides - non-HDL cholesterol - change in SBP (mmHg)
Secondary	
To assess the effect of eloralintide dose relative to placebo for change in: - body weight - central adiposity measurement - body composition via DXA	From baseline to Week 64: - change in body weight (kg) - change in WHtR - total lean mass, and - fat-to-lean mass ratio

Objectives	Endpoints
• BMI • glycemic control • lipid parameters • blood pressure • inflammatory biomarkers • biomarkers of insulin sensitivity • medication use • renal function • health-related quality of life • health status	• change in BMI (kg/m²) • the achievement (Y/N) of HbA1c target values in participants who do not meet the target value at baseline, of ○ <7% ○ ≤6.5% ○ <5.7% • change in 7-point SMBG • percent change in ○ triglycerides ○ non-HDL cholesterol ○ VLDL cholesterol ○ LDL cholesterol ○ HDL cholesterol ○ total cholesterol • change in SBP (mmHg) • change in DBP (mmHg) • percent change in hsCRP • percent change in fasting insulin • a change in: ○ antihyperglycemic medications ○ antihypertensive medications ○ lipid-lowering medications • change in eGFR • change in ○ SF-36 v2 acute form domain and summary scores ○ IWQOL-Lite CT scores • change in: ○ EQ-5D-5L health state utility and VAS
• eating behavior	• change in: ○ CoEQ scores ○ FNQ scores

Objectives	Endpoints
physical function	- the achievement (Y/N) of improved categorical shift in PGIS Physical Function Weight At Week 64: - the achievement (Y/N) of improvement in PGIC Physical Function Weight
To demonstrate that eloralintide 1.5 mg QW is superior to placebo for change in the following: - body weight - body composition via DXA - central adiposity measurement - glycemic control	From baseline to Week 64 - the achievement (Y/N) of body weight reduction of ≥5% ≥10% ≥15% ≥20% - total body fat mass - change in waist circumference (cm) - change in HbA1c (%) - change in fasting glucose (mg/dL)
To describe the safety of eloralintide compared to placebo	Incidence of: - treatment-emergent adverse events - nausea, vomiting, and diarrhea events - adjudicated clinical events (CV events, neurologic events, pancreatic events; see section 8.3.4)
To characterize eloralintide PK and evaluate dose and/or exposure-response relationships between eloralintide and key efficacy, safety and tolerability measures.	PK and PD parameters

Abbreviations: BMI = body mass index; CoEQ = control of eating questionnaire; CV = cardiovascular; DBP = diastolic blood pressure; DXA = dual energy x-ray adiposity; eGFR = estimated glomerular filtration rate; EQ-5D-5L = European quality of life-5 dimensions-5 level; FNQ = food noise questionnaire; HbA1c = hemoglobin A1c; HDL = high density lipoprotein; hsCRP = high sensitivity c-reactive protein; IWQOL-Lite CT = impact of weight on quality of life-lite clinical trials version; LDL = low density lipoprotein; PD = pharmacodynamic; PGIC = patient global impression of change; PGIS = patient global impression of severity; PK = pharmacokinetic; QW = weekly; SBP = systolic blood pressure; SF-36 = short form-36; SMBG = self-monitoring of blood glucose; VAS = visual analog scale; VLDL = very low density lipoprotein; WHtR = waist to height ratio; Y/N = yes/no.

For Japan, please refer to the objectives and endpoints in Appendix 11 (Section 10.11).

Estimands

The study will use two estimands for the primary and key secondary objectives: a treatment-regimen estimand for the treatment policy strategy, and an efficacy estimand for the hypothetical strategy. These estimands have distinct interpretations and serve different purposes.

Treatment policy strategy

The occurrence of the ICE is considered irrelevant in defining the treatment effect of interest; the values for the variable of interest are used regardless of whether the ICE occurs.

Hypothetical strategy

A scenario is envisaged in which the ICE would not occur. The value of the variable to reflect the clinical question of interest is the value that the variable would have taken in the hypothetical scenario defined.

Treatment-regimen estimand

The primary clinical question of interest is as follows:

What is the treatment difference in the percent change in body weight from baseline at 64 weeks between eloralintide 3 mg, 6 mg, and 9 mg and placebo, as an adjunct to healthy nutrition and physical activity, in participants with YDAF study disease/conditions criteria regardless of adherence to study intervention or initiation of prohibited weight management treatments?

Rationale for the estimand: This estimand aims to evaluate the efficacy of eloralintide that reflects the real-life behavior of the target population.

The “treatment regimen” estimand is described by the following attributes:

- **Population:** Participants with YDAF study disease/conditions. Further details can be found in Sections 5 and 9.
- **Endpoint:** Percent change from baseline in body weight at 64 weeks.
- **Treatment condition:** The randomized treatment with allowance for potential dose interruptions and modifications, regardless of adherence to study intervention or initiation of prohibited weight management treatments.
- **Intercurrent events:** No ICEs are defined since treatment adherence, and the initiation of prohibited weight management treatments is a part of the treatment condition.
- **Population-level summary and treatment effect of interest:** Difference in mean percent change from baseline in body weight at 64 weeks between eloralintide and placebo.

Efficacy estimand

The clinical question of interest is as follows:

What is the treatment difference in the percent change in body weight from baseline at 64 weeks between eloralintide 3 mg, 6 mg, and 9 mg and placebo, as an adjunct to healthy nutrition and physical activity, in participants with YDAF study disease/conditions if they would remain on their randomly assigned treatment for 64 weeks and would not initiate prohibited weight management treatments?

Rationale for the estimand: This estimand aims to evaluate the efficacy of eloralintide under the ideal condition that all participants would adhere to the randomly assigned study intervention without being confounded by the initiation of prohibited weight management treatments.

The “efficacy” estimand is described by the following attributes:

- **Population:** Participants with YDAF study disease/conditions. Further details can be found in Sections 5 and 9.
- **Endpoint:** Percent change from baseline in body weight at 64 weeks.
- **Treatment condition:** The randomized treatment with allowance for potential dose interruptions and modifications.
- **Intercurrent events:**

Intercurrent Event	Handling Strategy
permanent discontinuation of study intervention	hypothetical strategy
Initiation of weight management treatments	hypothetical strategy

- **Population-level summary and treatment effect of interest:** Difference in mean percent change from baseline in body weight at 64 weeks between eloralintide and placebo.

4. Study Design

4.1. Overall Design

Study J3R-MC-YDAF (YDAF) is a Phase 3 randomized, double-blind, placebo-controlled study to investigate the efficacy and safety of subcutaneously administered once weekly eloralintide in adult participants with obesity or overweight, and T2D.

The study consists of

- up to a 35 day screening period (Visit 1 to Visit 3)
- Treatment: 64 weeks
 - Dose escalation period: 12 weeks, and
 - Treatment maintenance period: 52 weeks, and
- Post-treatment follow-up: 6 weeks.

The visit frequency will be every 4 to 12 weeks.

The maximum duration of treatment is 64 weeks.

The study schema is presented in Section 1.2. Procedures and assessments for each visit are presented in the SoA in Section 1.3.

A substudy of Protocol YDAF to evaluate the 24 hour glucose profile captured with CGM for eloralintide versus placebo in individuals with obesity and or overweight with T2D is described in Appendix 9 (Section 10.9).

Number of Participants:

Approximately 1035 participants will be randomly assigned to treatment arms of placebo, eloralintide 1.5 mg, eloralintide 3 mg, eloralintide 6 mg, and eloralintide 9 mg at a randomization ratio of 2:1:2:2:2. An upper limit of approximately 70% enrollment of AFAB participants will be used to ensure a sufficient number of AMAB participants.

4.2. Scientific Rationale for Study Design

Choice of primary endpoint

In Study YDAF, the primary endpoint of percent change in body weight is a commonly used endpoint in clinical studies evaluating the efficacy and safety profile of obesity management medications in participants with obesity or overweight and T2D.

Choice of comparator

A placebo comparator was selected for this study in accordance with regulatory guidance (FDA 2025; EMA 2016) and to evaluate weight maintenance and dynamic changes in body composition during weight regain. Unfortunately, obesity management medication discontinuation remains common after achieving initial BW reduction, and thus the placebo arm serves as data to represent this real-world scenario (Gleason et al. 2024). In addition, all participants, regardless of treatment assignment, will continue to receive lifestyle-modification counseling consistent with current guidelines and local standards for weight management (AACE-ACE 2016; Jensen et al. 2014; Apovian et al. 2015). Specifically, participants will consult with a dietitian or equivalent qualified delegate throughout the study to focus on a

healthy diet and physical activity. Participants with T2D will also receive diabetes education consistent with guidelines for T2D management (American Diabetes Association-EASD 2022).

Study duration

The planned duration of treatment for the primary endpoint at 64 weeks allows for at least a 52-week treatment period at the randomly assigned dose (1.5 mg, 3 mg, 6 mg, or 9 mg). This duration is considered appropriate to assess the effects and benefit-risk of eloralintide compared with placebo on body weight and is consistent with regulatory guidelines (FDA 2025; EMA 2016).

The effects of study intervention cessation will be assessed in the 6-week posttreatment follow-up period.

Concomitant medications

To minimize the potential confounding effect of changes to concomitant medications, participants will be permitted to use concomitant medications that do not interfere with the assessment of efficacy or safety characteristics of the study intervention.

Refer to Section 6.9 for the concomitant medications relevant to the study.

Collection of race and ethnicity data

In this study, collection of demographic information includes race and ethnic origin. The scientific rationale is based on the need to assess variable response in safety or efficacy based on race or ethnic origin. This question can be answered only if all the relevant data are collected.

4.3. Justification for Dose

Lilly proposes final treatment doses of 1.5 mg, 3 mg, 6 mg, and 9 mg SC QW to be evaluated in this study. To reach the doses of 3 mg, 6 mg, and 9 mg QW, a dose-escalation regimen stepping through each of these doses every 4 weeks will be employed.

The justification for eloralintide Phase 3 doses and the dose-escalation scheme is based on totality of evidence from the Phase 1 single and multiple ascending dose study YDAA and Phase 2 dose-finding studies LAA1 and LAA2 (preliminary data in a subset from approximately 50% of participants up to Week 12 of treatment), including safety, tolerability, PK, PD and efficacy (weight reduction and glycemic control) data.

- In summary, In Part B (multiple ascending dose) of Study YDAA, a dose range of eloralintide from 1.2 to 12 mg SC QW treatment (without dose escalation) in participants with overweight or obesity, showed a weight reduction trend that was dose- and time-dependent. At the highest dose of 12 mg QW, the maximal LS mean percent change from baseline in weight and 90% CI weight reduction observed on Day 85, were -11.3% (-12.5, -10.0). The placebo-adjusted LS mean percent change and 90% CI were -11.5% (-13.3, -9.6).
- Eloralintide was well-tolerated in participants with overweight or obesity without adopting a gradual dose-escalation approach even for the highest dose of 12 mg QW.
- In the Phase 2 parallel-group, double-blind study (LAA1) to investigate weight management with eloralintide SC administered QW compared with placebo in

participants with obesity or overweight and without diabetes mellitus, eloralintide showed a robust dose-related weight reduction relationship covering the doses 1 mg, 3 mg, 6 mg, 9 mg, 6/9 mg, or 3/6/9 mg compared to placebo.

- Similarly, in a second Phase 2 parallel-group, double blind study (LAA2) to investigate weight management with eloralintide and tirzepatide, alone or in combination, in adult participants with obesity or overweight with T2D, preliminary data from an interim analysis of approximately 50% of participants reaching at least 12 weeks of eloralintide treatment, found eloralintide to be safe and tolerable.
- While a non-dose-escalated dosing approach to 9 mg showed acceptable tolerability, data from treatment arms in Study LAA1 which utilized a dose-escalation approach to reach 9 mg suggested that tolerability, in particular, gastrointestinal adverse events, can be further improved with a 3/6/9 mg dose-escalation approach. This has potential to enhance patient experience and therefore adherence to treatment leading to the dose-escalation approach proposed for this study.
- In general, small reductions in heart/pulse rate, systolic and diastolic blood pressures have been observed. There are no other AEs noted across studies that would preclude evaluation of eloralintide in a Phase 3 clinical program.

4.4. End of Study Definition

The end of the study is defined as the date of the last visit or last scheduled procedure shown in the SoA for the last participant in the trial globally.

5. Study Population

Participant eligibility for enrollment in the study is based on the criteria listed in this section. The inclusion and exclusion criteria used to determine eligibility should only apply at screening or other specified visits, and not continuously throughout the study.

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.1. Inclusion Criteria

Participants are eligible to be included in the study only if all of the following criteria apply:

Age

1. Participant must be at least 18 years of age and of the legal age of consent in the jurisdiction in which the study is taking place at the time of signing the informed consent.

Type of participant and disease characteristics

2. Have a clinical diagnosis of T2D according to the World Health Organization (WHO) classification or other locally applicable standards for at least 6 months prior to screening (Visit 1), with HbA1c $\leq 10.0\%$ (86 mmol/mol) at screening (Visit 1)
3. Are on stable treatment for T2D for at least 90 days prior to screening (Visit 1), consisting of
 - either diet/exercise alone or
 - up to 3 oral AHMs (excluding incretins).**Note:** DPP-4i are allowed.

Weight

4. Have a BMI ≥ 27 kg/m² at Visit 1.
Note: In participants with limb amputation or limb loss, the investigator should reach out to the Lilly medical monitor for information regarding calculation of BMI.
5. Have a stable body weight ($<5\%$ body weight change) for 90 days prior to screening at Visit 1.
6. Have a history of at least 1 self-reported unsuccessful dietary effort to reduce body weight

Sex assigned at birth and contraceptive/barrier requirements

7. IOCBP, INOCBP, and individuals AMAB who agree to abide by the reproductive and contraceptive requirements provided in Section 10.4. may participate in this trial. See Section 10.4 for definitions and additional requirements related to contraception.

Note: Contraceptive use by participants should be consistent with local regulations regarding the methods of contraception for those participating in clinical studies. For the contraception requirements of this protocol, see Section 10.4.

Informed Consent

8. Are capable of giving signed informed consent as described in Appendix 1, which includes compliance with the requirements and restrictions listed in the ICF and in this protocol.

Other Inclusion Criteria

9. Are reliable and willing to make themselves available for the duration of the study and attend required study visits, and are willing and able to follow study procedures as required, such as
 - self-inject study intervention
 - store and take the provided study intervention as directed
 - maintain electronic study diary and paper log
 - complete the required questionnaires, and
 - undergo DXA procedure.

Note: if a person is unable to undergo the DXA due to physical limitations of the DXA machine, or because they have implants, hardware, or other foreign materials that may interfere with the scan according to the DXA manufacturer's specification, this will not be considered an exclusionary criterion. See Section 8.1.2 for further details.

5.2. Exclusion Criteria

Participants are excluded from the study if any of the following criteria apply:

Medical Conditions***Obesity related***

10. Have a prior or planned surgical treatment for obesity.

Exception: The following are allowed if performed >1 year before screening (Visit 1):

- liposuction
- cryolipolysis, or
- abdominoplasty.

11. Have a prior or planned endoscopic procedure and/or device-based therapy for obesity, such as, but not limited to

- mucosal ablation
- gastric artery embolization
- laparoscopic adjustable gastric banding
- intragastric balloon, or
- duodenal-jejunal endoluminal liner.

Exception: Prior device-based therapy is acceptable if device removal was more than 6 months prior to screening (Visit 1).

12. Have obesity induced by other endocrinologic disorders, for example, Cushing Syndrome, or are diagnosed with monogenetic or syndromic forms of obesity, for example, Melanocortin 4 Receptor deficiency or Prader Willi Syndrome.

Diabetes related

13. Have type 1 diabetes mellitus, latent autoimmune diabetes, or history of ketoacidosis or hyperosmolar coma
14. Have had 1 or more episode of severe hypoglycemia and/or 1 or more episode of hypoglycemia unawareness within the 180 days prior to Visit 1.
Note: Any participant that the investigator feels will not be able to communicate an understanding of hypoglycemic symptoms and the appropriate treatment of hypoglycemia should also be excluded.
15. Have fasting glucose >270 mg/dL (>15.0 mmol/L) confirmed by the central laboratory at screening (Visit 1).
16. Have taken within 90 days before screening (Visit 1) any of the following AHMs:
 - amylin analogs
 - GLP-1 receptor agonists
 - GIP/GLP receptor agonists, or
 - insulin.
17. Are currently receiving or planning to receive treatment for diabetic retinopathy and/or macular edema, for example, laser photocoagulation or intravitreal injections of antivascular endothelial growth factor inhibitors.

Note: All participants will have a baseline fundus photography assessment prior to randomization (Visit 3). Qualifying fundus photographs obtained within 90 days before Visit 1 may be used for eligibility determination. See Section 8.2.11.

Renal

18. Have renal impairment measured as eGFR <30 mL/min/1.73 m², calculated by eGFR_{cr-cys} formula as per Chronic Kidney Disease Epidemiology Collaboration as determined by central laboratory during screening (Visit 1).

Autoimmune

19. Have evidence of significant, active autoimmune abnormality, for example, lupus or rheumatoid arthritis, that, in the opinion of the investigator, is likely to require concurrent treatment with systemic glucocorticoids during the course of the study.

Cardiovascular

20. Have had within 90 days prior to screening (Visit 1):
 - acute myocardial infarction

- cerebrovascular accident (stroke)
- coronary artery revascularization
- unstable angina, or
- hospitalization due to congestive heart failure.

21. Have a history or diagnosis of New York Heart Association Functional Classification Class IV congestive heart failure.

22. Have either

- ongoing asymptomatic bradycardia defined as <50 beats per minute at V1, or
- ECG findings at V1 of 2nd degree AV block or higher, or
- history of symptomatic bradycardia with uncorrected etiology, including diagnosis of sick sinus syndrome.

Note: Historical symptomatic bradycardia or AV nodal block due to a corrected transient etiology will not be considered exclusionary (for example, previous beta-blocker, calcium channel blocker, or other medication that can lead to AV block; or conduction disorder corrected with cardiac device).

Hepatic

23. Have acute or chronic hepatitis, including a history of autoimmune hepatitis, signs, or symptoms of any other liver disease other than nonalcoholic fatty liver disease, or any of these laboratory values confirmed by central lab at screening (Visit 1)

- ALT or AST level $\geq 3.0\times$ the ULN

Note: Participants with nonalcoholic fatty liver disease may participate in the study if their ALT level is $<3.0\times$ the ULN.

- ALP level $\geq 1.5\times$ the ULN
- total bilirubin level $\geq 1.5\times$ the ULN, except for participants with history of Gilbert's Syndrome.

Endocrine

24. Have evidence of significant, uncontrolled endocrine abnormality, in the opinion of the investigator; for example, thyrotoxicosis or adrenal crisis.

25. Have thyroid stimulating hormone levels outside the normal reference range for the central laboratory at Visit 1.

Exception: Participants with hypothyroidism who are clinically euthyroid and on stable thyroid replacement therapy for at least 60 days prior to Visit 1 are acceptable exceptions to this criterion.

Malignancy

26. Have a family or personal history of medullary thyroid carcinoma or multiple endocrine neoplasia syndrome type 2.

27. Have a history of an active or untreated malignancy or are in remission from a malignancy for less than 5 years (Visit 1).

Exceptions:

- basal or squamous cell skin cancer
- in situ carcinomas of the cervix
- Grade 1 (for example, Gleason 6 or lower) prostate cancer.

Hematology

28. Have any hematological condition that may interfere with HbA1c measurement, for example, hemolytic anemias or sickle cell disease.

Psychobehavioral

29. Have a history of significant active or unstable major depressive disorder or other severe psychiatric disorder, such as schizophrenia, bipolar disorder, or other serious mood or anxiety disorder, within the last 2 years,

OR

In the investigator's opinion, have any significant mental health disorder that may put the individual at higher risk of study participation.

30. Have a PHQ-9 score of 15 or more at any screening visit (Visit 1) or at randomization (Visit 3).
31. Are, in the judgment of the investigator, deemed to be at significant risk for suicide
32. Have answered "yes" to either Question 4 or Question 5 on the "Suicidal Ideation" portion of the C-SSRS and the ideation occurred within the past 3 months

OR

Have answered "yes" to any of the suicide-related behaviors (actual attempt, interrupted attempt, aborted attempt, preparatory act or behavior) on the "suicidal behavior" portion of the C-SSRS, and the behavior occurred within the past 5 years

Note: Non-suicidal self-injurious behavior is not considered a suicide-related behavior.

Gastrointestinal

33. Have a known clinically significant gastric emptying abnormality, such as severe diabetic gastroparesis or gastric outlet obstruction, or chronically take drugs that directly affect GI motility.
34. Have had a transplanted organ or awaiting an organ transplant.
- Exception:** Corneal transplants or keratoplasty are allowed.
35. At the time of screening, have a planned surgery, except for minor surgical procedures, to occur during the study.

Prior/Concomitant Therapy

36. Have taken any prohibited glucose-lowering medications (see Section 6.9.3) within 90 days of Visit 1 or have a history of insulin therapy within 90 days prior to Visit 1.

37. Are receiving or have received within 90 days prior to screening chronic (>2 weeks) systemic glucocorticoid therapy (excluding topical, intraocular, intranasal, single intraarticular injections, single localized injections of skin lesions or tendon, or inhaled preparations).
38. Have known or suspected hypersensitivity to amylin or study product.
39. Have taken within 90 days prior to screening (Visit 1) prescribed or over-the-counter medications, or alternative remedies, including herbal or nutritional supplements, intended to promote body weight reduction.
- Examples include, but are not limited to
 - GLP-1RA
 - GIP/GLP-1RA
 - orlistat
 - sibutramine
 - phenylpropanolamine
 - mazindol
 - phentermine
 - lorcaserin
 - phendimetrazine
 - phentermine/topiramate
 - naltrexone/bupropion, and
 - ingested material that transiently occupies space in the stomach, for example, hydrogel capsules made of cellulose and citric acid.

Exception: AHMs for the management of T2D, for example, SGLT-2 inhibitors, are permitted, with the exception of the examples listed above used as AHM (for example, GLP-1RA, GIP/GLP-1RA).

40. Are receiving or have received within 90 days of screening, medications that may cause significant weight gain – including but not limited to tricyclic antidepressants, atypical antipsychotics, and mood stabilizers – unless at a stable dose for more than 6 months, with no planned dose changes, and documented weight stability (see Appendix 8: Section 10.8).

Note: Insulin therapy is not allowed within 90 days prior to Visit 1.

41. Have started subcutaneous/intramuscular or implantable/injectable contraceptives such as Depo-Provera®, Nexplanon, within 18 months prior to screening (Visit 1).

Note: Intrauterine devices are acceptable.

Prior/Concurrent Clinical Study Experience

- 42. Are currently enrolled in any other clinical study involving an investigational product or any other type of medical research judged not to be scientifically or medically compatible with this study.
- 43. Within the last 90 days before screening (Visit 1), have participated in a clinical study and received active treatment or unknown if they received active treatment.
- 44. Have previously completed or withdrawn from this study or any other study investigating eloralintide after receiving at least 1 dose of intervention.

Diagnostic Assessments

DXA related

- 45. Have had a procedure (within 10 days of screening [Visit 2]) where oral contrast or radionuclides were administered, for example, CT with contrast or nuclear medicine.

Other Exclusion Criteria

- 46. Are pregnant, intending to breastfeed, or intending to conceive.
- 47. Are investigator site personnel directly affiliated with this study and/or their immediate family. Immediate family is defined as a spouse, legal partner, parent, child, or sibling, whether biological or legally adopted.
- 48. Have any condition, unwillingness, or inability, not covered by any of the other exclusion criteria, which in the investigator's opinion might jeopardize the participant's safety, for example, hypersensitivity or contraindication.
- 49. Have any medical condition that, in the opinion of the investigator, would be a contraindication to participation in the trial.
- 50. Are Lilly employees or are employees of any third party involved in the study who require exclusion of their employees.

5.3. Lifestyle Considerations

Per the SoA (Section 1.3), participants will meet a dietitian, or equivalent qualified delegate, according to local standards, to receive lifestyle counseling.

Healthy nutrition patterns and physical activity goals established during the lifestyle consultation and the importance of adherence to the lifestyle component of the study will be reinforced at each visit by site staff. Ensuring participants have optimal nutritional intake is highly recommended. In addition to Section 5.3.1, nutrition support and lifestyle resources are available in the investigator resources.

To encourage adherence, it is recommended that the participant complete their 3-day nutrition and physical activity log before each counseling visit per the SoA (Section 1.3) During each visit, the participant's log is reviewed and advice may be provided as needed to maximize adherence, nutrition, and hydration.

5.3.1. Meals and Nutrition

At the randomization visit and subsequent visits, study participants will receive individualized counseling regarding a healthy nutrition pattern with the goal of optimizing nutritional status. The counseling will be performed by a dietitian or nutritionist, or equivalent qualified designee, according to the local standard.

Counseling considerations

The focus of the counseling should facilitate balanced and sufficient food and beverage choices for adequate nutritional intake and hydration. Counseling should include instruction on customized nutrient-dense food and beverage choices to reflect personal preferences, cultural traditions, and budgetary considerations.

The 3-day nutrition and physical activity log should be reviewed at lifestyle consultations per the SoA (Section 1.3). It is recommended that the dietitian or equivalent qualified delegate communicate with the investigator regarding participant intake and/or physical activity. Any concerns should be discussed with the investigator. At the discretion of the investigator, to provide additional support of nutrition and physical activity goals, the 3-day nutrition and physical activity log can be completed more frequently than what is indicated in the SoA.

The table below summarizes the recommendations for both general dietary and fluid intake (Almandoz et al. 2024; US Dietary Guidelines 2020-2025) and minimum dietary and fluid intake. Adequate nutritional intake and hydration recommendations, in the setting of weight management, may be individualized based on the participant's health status, including age and comorbidities, appetite levels, and GI tolerability. Recommendations should focus on prioritizing intake of healthy carbohydrate, protein, and fluid.

	General Intake Recommendations	Minimum Intake Recommendations
Fluids (that is, water or decaffeinated and non-sugary beverages)	2000-3000 ml/day	2000 ml/day
Energy	1200-1500 kcal/day (women) 1500-1800 kcal/day (men)	800-1000 kcal/day
Protein	0.8-1.2 g/kg/day (in some cases, calculation may be based on ideal body weight)	60 g/day
Carbohydrates	45%-65% of energy intake	50 g/day
Fat	20%-35% of energy intake	15 g/day (<10% saturated fat/day)

Consider the following principles to guide participant education in counseling sessions:

- the importance of meeting nutritional and hydration recommendations
- shifting the focus of obesity treatment from “eating less” to “eating enough” each day for optimal health

- decreased appetite and increased fullness after eating typically drive a reduction in food intake. Additional voluntary food restriction, when appetite is reduced, is unnecessary and may be harmful
- avoid skipping meals
- eating smaller and more frequent (every 3 to 4 hours) meals or snacks
- focus on nutrient-rich, minimally processed foods. Limit foods high in solid fats, added sugar, and salt
- include fiber-rich foods
- for each meal (on an 8- to 10-inch [20-25 cm] plate), consider including approximately half plate of fruits or vegetables, quarter plate of whole grains, and quarter plate of protein-rich foods. Incorporate 1 to 2 servings of healthy, unsaturated fats on most days (for example, nuts, avocado, olive oil, and salmon).

In addition, the following strategies are encouraged to facilitate adequate nutritional intake and hydration:

- multivitamin and/or micronutrient supplementation
- meal replacements (for example, protein- and micronutrient-rich shakes or bars) that can be sipped or eaten slowly over several hours, particularly for participants with markedly reduced appetite and poor intake, and
- more frequent nutritional counseling if needed.

5.3.2. Monitoring Nutritional Needs

As with other obesity management medications, there is potential that a small number of participants on study intervention will experience a significantly reduced caloric intake.

Individual risk factors for inadequate nutrition or dehydration should be taken into consideration, including, but not limited to, age, medical history/comorbidities, concomitant medications, clinical conditions (for example, infection), and GI symptoms (for example, nausea, vomiting, diarrhea). Clinically monitor the nutritional and hydration status of participants and assess for any signs or symptoms of inadequate nutrition or dehydration, including

- vital signs
- targeted physical examination
- recent central laboratory data, and
- nutrition and physical activity log (per SoA).

Obtain additional laboratory tests as clinically appropriate via the central laboratory. Use local laboratory tests as needed, for example for acute management, or if lab tests are not available centrally. See Sections [8.3.5.13](#), [10.2](#), and [10.2.2](#) for more information on clinical laboratory tests.

By recognizing early signs of poor nutrition and dehydration, preventive actions can be taken to reduce the risk of potential complications.

Monitor for the following which could be risk factors/signs for poor nutrition or dehydration.

Potential concern	Early signs include, but are not limited to
Poor nutrition	• carbohydrate intake <50 g/day • protein intake <60 g/day • fat intake <15 g/day • caloric intake <800-1000 kcal/day, and • skipping multiple meals
Dehydration	• low fluid intake (<2000 ml/day) • volume loss (for example, diuretic or laxative use, diarrhea, and vomiting) • elevated pulse rate • decreased or low blood pressure, and • high blood urea nitrogen to creatinine (BUN:Cr) ratio

If there are concerns of inadequate nutrition or dehydration, mitigation strategies should be guided by an assessment of the severity of the participant's clinical status, potential underlying etiologies, and GI symptoms. See Sections 6.6.2 and 10.2.2 for the recommended steps to follow.

5.3.3. Diabetes Education

Diabetes education will be provided by personnel who are qualified to educate participants on symptoms and management of hyperglycemia and hypoglycemia, SMBG, and diabetes management according to American Diabetes Association Standards of Medical Care in Diabetes and/or a Consensus Report by the American Diabetes Association and the European Association for the Study of Diabetes (American Diabetes Association 2025a, Davies et al. 2022) or local guidelines.

All participants will be provided with glucometers and related testing supplies for use during the study.

Site personnel will train the participant on correct use of the glucometer for self-monitoring blood glucose and reporting hypoglycemia.

5.3.4. Healthy Physical Activity

At the randomization visit and all subsequent visits, participants will receive guidance on achieving and maintaining a healthy physical activity level. A minimum of 150 minutes per week of moderate intensity aerobic exercise, along with a minimum of 3 muscle strengthening (resistance) workouts per week is recommended.

Lifestyle modification counseling should be completed according to the SoA and must be documented in the participant's medical record.

Activity before blood collections

Participants should abstain from significant exercise for 24 hours before each blood collection for clinical laboratory tests.

5.3.5. Other Restrictions**Activity before blood collections**

Participants will abstain from significant exercise for 24 hours before each blood collection for clinical laboratory tests.

Blood Donation

Study participants should not donate blood or blood products during the study or for 10 weeks following their last study visit.

5.4. Screen Failures

A screen failure occurs when a participant who consents to participate in the clinical study is not subsequently assigned to study intervention. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the Consolidated Standards of Reporting Trials publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, and any SAE.

Individuals who do not meet the criteria for participation in this study (screen failure) may not be rescreened.

5.5. Criteria for Temporarily Delaying Randomization of a Participant

Participants may have their randomization temporarily delayed if additional clinical information, including laboratory testing, is needed to be repeated.

6. Study Intervention(s) and Concomitant Therapy

Study intervention is defined as any medicinal product(s) or medical device(s) intended to be administered to or used by a study participant according to the study protocol.

6.1. Study Intervention(s) Administered

Study intervention will be dispensed at the study visits summarized in the SoA (Section 1.3).

Returned study intervention should not be re-dispensed to the participants.

This table lists the interventions used in this clinical study.

Intervention Name	Eloralintide	Eloralintide Placebo
Dosage Level(s)	1.5 mg 3 mg 6 mg 9 mg	Placebo
Route of Administration	Subcutaneous using a PFS	Subcutaneous using a PFS
Frequency of Administration	Once weekly	Once weekly
Authorized as defined by EU Clinical Trial Regulation	Not authorized in EU	Not authorized

Abbreviations: PFS = prefilled syringe.

Eloralintide dose escalation

Participants randomly assigned to eloralintide will start with a 1.5 mg QW dose and will then undergo dose escalation every 4 weeks until their randomized dose (1.5 mg, 3 mg, 6 mg, or 9 mg) is reached.

Allowed randomized doses

Allowed randomized doses are 1.5 mg, 3 mg, 6 mg, or 9 mg.

Participants randomized to placebo

To maintain blinding, participants randomly assigned to the placebo treatment group will also receive study intervention every 4 weeks during the dose-escalation period, following a sham dose-escalation schedule to match the eloralintide 9 mg treatment group.

Timing of dose administration

Participants should try to administer the SC injections on the same day of the week and similar time of day, but there are no restrictions on the time of day for each weekly dose.

The participant will record the actual date and time of all dose administrations in the diary.

Missed doses

If a dose is missed, the participant should take it as soon as possible unless it is within 72 hours of the next dose, in which case that dose should be skipped. If dose is skipped, the next dose should be taken at the appropriate time and skipped dose should be recorded in diary.

Site staff should document the planned treatment date for the dose and record a reason the dose was missed in the eCOA portal.

The day of weekly administration can be changed, if necessary, if the last dose was administered 72 or more hours before.

Anatomical location of injections

Acceptable locations for injection by the participant include the abdomen or thigh, or upper arm if another person gives the injection.

Participants should rotate injection sites from 1 injection to the next, even when injecting within the same region.

A new prefilled syringe will be used for each injection.

The participant will record the injection-site location of all dose administrations in the diary.

Packaging and labeling

Study interventions will be supplied by the Sponsor or its designee in accordance with current Good Manufacturing Practice. Study interventions will be labeled as appropriate for country requirements.

6.2. Preparation, Handling, Storage, and Accountability

The investigator or designee must confirm appropriate storage conditions have been maintained during transit for all study intervention received and any discrepancies are reported and resolved before use of the study intervention.

Only participants enrolled in the study may receive study intervention. Only authorized study personnel may supply, prepare, or administer study intervention.

All study intervention must be stored in a secure, environmentally controlled, and monitored (manual or automated) area in accordance with the labeled storage conditions with access limited to the investigator and authorized study personnel.

The investigator or authorized study personnel are responsible for study intervention accountability, reconciliation, and record maintenance, that is, receipt, reconciliation, and final disposition records.

Further guidance and information for the final disposition of unused study interventions are provided in the Pharmacy Manual.

Participant responsibilities

Study participants will be trained on the proper storage and handling of the study intervention and should follow in-use storage conditions according to the instructions for use provided by the Sponsor.

6.3. Assignment to Study Intervention

Participants who meet all criteria for enrollment will be randomly assigned to 1 of the study intervention groups. Assignment to treatment groups will be determined by a computer-generated, random sequence using an IWRS.

A total of approximately 1035 participants will be randomly assigned in a 2:1:2:2:2 ratio to received weekly dose of placebo, eloralintide 1.5 mg, 3 mg, 6 mg, and 9 mg.

The randomization will be stratified by

- sex (AFAB, AMAB)
- type of baseline antihyperglycemic medication (weight gain, weight reduction, or weight neutral), and
- screening HbA1c ($\leq 6.5\%$, $>6.5\%$).

Stratification based on the use of antihyperglycemic medications

Stratum	Potential Body Weight Effect	Participants to be Included in the Corresponding Stratum
1	Associated with weight reduction	• The use of an SGLT-2 inhibitor alone or in combination with any other AHMs except for an SU or TZD
2	Associated with weight gain	• The use of a SU alone or in combination with any other AHMs except for an SGLT-2 inhibitor OR • The use of a TZD alone or in combination with any other AHMs except for an SGLT-2 inhibitor
3	Weight neutral and others	• Lifestyle management only: no background AHMs OR • Metformin monotherapy OR • The use of an SGLT-2 inhibitor in combination with a SU and/or TZD, with or without other AHMs

Stratum	Potential Body Weight Effect	Participants to be Included in the Corresponding Stratum
		Note: The body weight effect of SGLT-2 inhibitor may be offset by SU/TZD, and vice versa, when used in combination. OR All other monotherapy (such as glinides and AGIs) or combination therapy which does not fall under stratum 1 or 2.

Abbreviations: AHM = antihyperglycemic medication; SGLT = sodium-glucose cotransporter; SU = sulfonylurea; TZD = thiazolidinedione.

6.4. Blinding

This is a double-blind study. Study participants and study personnel are blinded to study intervention. Blinding will be maintained throughout the conduct of the study, as described in the separate Blinding and Unblinding Plan.

To maintain the blind, a minimum number of Lilly personnel will see the randomization table and treatment assignments before the study is complete.

If an investigator, site personnel performing assessments, or participant is unblinded, the participant must be discontinued from the study. In cases where there are ethical reasons to have the participant remain in the study, the investigator must obtain specific approval from a Sponsor medical monitor for the participant to continue in the study.

Emergency unblinding

In case of an emergency, the investigator has the sole responsibility for determining if unblinding of a participant's treatment assignment is warranted for medical management of the event. The participant's safety must always be the first consideration in making such a determination. If a participant's treatment assignment is unblinded, Lilly must be notified immediately.

If the investigator decides that unblinding is warranted, it is the responsibility of the investigator to promptly document the decision and rationale and notify Lilly as soon as possible.

Emergency unblinding may be performed through the IWRS. This option may be used ONLY if the participant's well-being requires knowledge of the participant's treatment assignment. All unblinding events are recorded and reported by the IWRS.

Discontinuation after unblinding

If an investigator, site staff performing assessments, or participant is unblinded, the participant must be permanently discontinued from study intervention but should continue in the study for efficacy and safety endpoint evaluations and monitoring for all visits.

6.5. Study Intervention Compliance

Participant compliance with study intervention will be assessed at visits summarized in the SoA (Section 1.3). Compliance will be assessed by direct questioning and counting of unused study

intervention returned and documented in the source documents. Study intervention compliance will be determined by the following:

- Study intervention administration data will be recorded by the participant in the diary and reviewed for completeness and accuracy by the investigator **at each study visit**.
- The participants will be instructed to return any unused study intervention at the next visit to the study site for the purpose of performing study intervention accountability.
- Treatment compliance is defined as taking at least 75% of the required SC doses of study intervention. Similarly, a participant will be considered significantly noncompliant if the investigator determines that they have intentionally or repeatedly taken more than the prescribed amount of medication (125% or more of the required SC doses of the study intervention).

Other aspects of compliance with the study interventions will be the participant's adherence to the visit schedule, study diaries, and any other parameters the investigator considers necessary.

Participants considered poorly compliant will receive additional training and instruction as required. Additional unscheduled visits may be scheduled if study site staff determine that a participant requires additional training.

Site staff will capture all dose modifications in the CRF.

6.6. Dose Modification

All participants will undergo each dose-escalation step regardless of randomized treatment assignment. These dose-escalation steps will be handled in a blinded fashion using the IWRS. During the dose-escalation period, the investigator should make every effort to proceed through each dose-escalation step per the study schedule in order for participants to achieve their randomized dose. Participants should continue this dose for the duration of the treatment period. To maintain blinding, participants randomly assigned to placebo or to 1.5 mg, 3 mg, and 6 mg will follow a sham dose-escalation schedule to match the eloralintide 9 mg treatment group.

Allowed dose modifications

Dose modification is only allowed for the management of

- intolerable GI symptoms (Sections 6.6.1)
- inadequate nutrition or dehydration (Sections 6.6.2), or
- participants reaching a BMI ≤ 22 kg/m² (Sections 6.6.3).

6.6.1. Management of Participants with Intestinal Symptoms

All efforts should be made to prevent permanent discontinuation of study intervention. For participants who report intolerable GI symptoms during the study, the investigator should implement the following steps:

STEP 1	The initial step is based on principal investigator's assessment of predominant gastrointestinal symptoms with an approach that includes dietary adjustments, lifestyle modifications, with or without symptomatic medications. Counsel participant on dietary behaviors that may help mitigate gastrointestinal symptoms, such as for nausea, ▪ eating smaller meals ▪ splitting 3 daily meals into 4 or more smaller ones, and ▪ stopping eating when they feel full. Also, at the investigator's discretion, symptomatic medication, for example, antiemetic or antidiarrheal medication can be prescribed.
STEP 2	If Step 1 does not alleviate intolerable gastrointestinal symptoms, a temporary interruption of study intervention for 1 weekly dose is permitted within each dose-escalation level. Study intervention should be resumed immediately after temporary interruption, either alone or in combination with symptomatic medication, which can also be utilized to manage symptoms.
STEP 3	If significant GI symptoms persist despite the above measures (Step 1 and Step 2) the investigator should contact the Lilly medical monitor to consider a temporary dose de-escalation.

Abbreviations: GI = gastrointestinal

Temporary dose de-escalation

If intolerable GI symptoms persist despite the measures outlined in Steps 1 to 2 above, the investigator should contact the Lilly medical monitor to consider de-escalating the dose to the next lower dose for 4 weeks.

All dose modifications will be handled by the IWRS in a blinded fashion.

The table below provides guidance for de-escalating the dose.

If a participant is currently taking blinded...	Then de-escalate the dose to...
1.5 mg	discontinue the study intervention and continue in the study
3 mg	blinded 1.5 mg
6 mg	blinded 3 mg
9 mg	blinded 6 mg

Note: To maintain blinding, only participants that do not tolerate the initial dose of blinded 1.5 mg will be permanently discontinued from the study intervention. All others will dose de-escalate to the next lower dose or placebo, as appropriate based on the dose being taken at the time.

Dose re-escalation

After temporary dose de-escalation, if intolerable GI symptoms do not stop and dose re-escalation is not deemed appropriate, the investigator should contact the Lilly medical monitor to discuss next steps.

If GI symptoms become tolerable or resolve with dose de-escalation, the investigator should initiate re-escalation after 4 weeks have passed.

If the re-escalation attempt is tolerated, then the participant should continue to complete the remaining dose-escalation steps.

If the re-escalation attempt is not tolerated or if intolerable GI symptoms recur at any subsequent time point following the re-escalation, then, depending on which dose level is achieved, the participant should undergo a permanent dose reduction to the next lower dose level, or receive placebo in a blinded fashion.

The table below provides guidance if the re-escalation attempt was not tolerated.

If the re-escalation attempt is not tolerated with this dose...	then the dose will be reduced for the remainder of the study to...
blinded 3 mg	blinded 1.5 mg
blinded 6 mg	blinded 3 mg
blinded 9 mg	blinded 6 mg

Note: If a participant does not tolerate blinded 1.5 mg, then the participant will discontinue the study intervention and continue in the study.

Please note the following

- A maximum of one dose re-escalation is allowed per participant for the duration of the study.
- Following a de-escalation/re-escalation cycle, any additional dose modification to a lower dose will be considered a permanent dose reduction.

If intolerable GI symptoms persist despite symptomatic treatment, temporary drug interruption, temporary dose de-escalation, dose re-escalation, and permanent dose reduction, the participant should be permanently discontinued from the study intervention. All participants who permanently discontinue study intervention should be encouraged to continue to attend all scheduled study visits.

All dose adjustments, for example, dose de-escalation or re-escalation, are to be recorded in the CRF.

6.6.2. Management of Participants with Inadequate Nutrition or Dehydration

Follow these steps (and refer to Section 10.2.2) if a participant is at risk of inadequate nutrition or dehydration based on clinical judgement and criteria described in Sections 5.3.1 and 5.3.2:

1. Assess if there is an intercurrent illness (for example, systemic infection, cholecystitis, and so forth.) that could be causing or contributing to the inadequate oral intake.
 - a. If yes, consider temporary interruption of study intervention per Section 7.1.2.
 - b. If no, go to Step 2.
2. Assess if increased oral intake is likely to trigger GI symptoms.
 - a. If no
 - i. Provide nutritional counseling and consider strategies to increase oral intake. Refer to Sections 5.3.1 and 5.3.2.
 - ii. Review concomitant medications and adjust as appropriate; consider relevant laboratory tests (see Sections 6.9 and 10.2).
 - b. If yes
 - i. Temporarily interrupt study intervention for 1 to 2 doses per Section 7.1.2.
 - ii. Provide nutritional counseling and consider strategies to increase oral intake. Refer to Sections 5.3.1 and 5.3.2.
 - iii. Review concomitant medications and adjust as appropriate; consider relevant laboratory tests (see Sections 6.9 and 10.2).
 - iv. Reassess if nutritional and hydration concerns persist
 1. If no, restart study intervention per Section 7.1.2.
 2. If yes, contact the Lilly medical monitor to request a permanent dose reduction to the next lower dose level or discontinue study intervention. However, the participant should remain in the study.
 - v. Assess if nutritional and hydration concerns persist despite dose reduction
 1. If no, continue study intervention at the reduced dose.
 2. If yes despite dose reduction, contact the Sponsor for further guidance.

Note: Dose re-escalation is not permitted.

6.6.3. Management of Participants with BMI ≤ 22 kg/m²

If...	Then...
a participant has a BMI ≤ 22 kg/m ² at any time during the study	Step 1: adjust nutritional intake to avoid ongoing weight reduction and contact the Lilly medical monitor. Step 2: the participant should be assessed at least every 2 to 4 weeks for weight stabilization with either an in-office weight check or self-report via telehealth. Step 3: if body weight reduction continues despite nutritional intake guidance, contact the Lilly medical monitor to consider a permanent dose reduction to the next lower dose level.
body weight reduction continues to decline despite a permanent dose reduction and the investigator or participant wants to consider discontinuing study intervention	contact the Lilly medical monitor for further discussion.
a participant has a BMI ≤ 18.5 kg/m ² at any time during the study	dose interrupt and contact Lilly medical monitor.

Abbreviations: BMI = body mass index.

All participants who discontinue study intervention are encouraged to continue to attend the remaining scheduled study visits (see Section 7.1).

6.6.4. Management of Participants with Perceived Excessive Weight Reduction

If there are concerns that the degree of body weight reduction may lead to a participant's decision to discontinue from study intervention, the investigator should contact the Lilly medical monitor.

6.7. Continued Access to Study Intervention after the End of the Study

Eloralintide will not be made available to participants after completion of the study.

6.8. Treatment of Overdose

For this study, any total dose of study intervention within a 72-hour time period greater than the dose assigned by IWRS for that participant will be considered an overdose and should be assessed for AE reporting as per criteria described in Section 10.3.

In the event of an overdose, the investigator should:

- contact the medical monitor immediately
- evaluate the participant to determine, in consultation with the medical monitor, whether study intervention should be interrupted or whether the dose should be reduced
- closely monitor the participant for any AE/SAE and laboratory abnormalities as medically appropriate
- if indicated, initiate supportive treatment according to the participant's clinical signs and symptoms, and

- on a case-by-case basis, it may be recommended to obtain a plasma sample for PK analysis within 5 days from the date of the last dose of study intervention if requested by the medical monitor.

6.9. Prior and Concomitant Therapy

Participants are permitted to use concomitant medications that they require during the study except medications that may interfere with the assessment of efficacy and safety characteristics of the study interventions.

Participants must consult with the investigator or a designated site staff member if they are prescribed any new medications during the study. If this is not possible due to treatment of medical emergencies, the participant will inform the investigator or a designated site staff member as soon as possible.

The initiation of weight gain-promoting medications is discouraged, and alternate therapies not associated with weight gain should be considered whenever possible (refer to Section 5.2, exclusion criterion [40] and Appendix 8: Section 10.8).

In this study population, many study participants will be taking medications to treat weight-related comorbidities, such as antihyperglycemic, antihypertensive, diuretic, and lipid-lowering agents; or medications to mitigate GI tolerability, including laxatives, and anti-emetics; or medications with weight-based dosing such as thyroid hormone.

Investigators are responsible for closely monitoring the need to adjust these medications throughout the study. Discuss medication adjustment as soon as possible with primary care physician or specialist, as applicable.

Recording Concomitant Medications

Site staff will record changes in intensity of medications of special interest on the level of medication intensity CRF at intervals specified in the SoA. These changes should also be recorded in the concomitant medication CRF. Medications of special interest that should be recorded in both CRFs include

- antihyperglycemic
- antihypertensive, and
- lipid-lowering medications.

Any medication or vaccine, continuous positive airway pressure, over the counter, or prescription medicine (including hormonal contraceptives), vitamins or herbal supplements, or other specific categories of interest that the participant is receiving at the time of enrollment or receives during the study must be recorded in the CRF along with

- reason for use
- dates of administration, including start and end dates, and
- dosage information, including dose and frequency only for concomitant medications of special interest, for example antihypertensive, antihyperglycemic, and lipid-lowering medications.

Non-study medications taken by participants who are screened but not randomly assigned to treatment will not be reported to the Sponsor unless an SAE or AE occurs that the investigator believes may have been caused by a study procedure.

6.9.1. Initiation of new Antihyperglycemic Medications

The introduction of new AHMs other than study intervention is expected during the study ONLY in the following situations:

- as a rescue therapy for the management of severe, persistent hyperglycemia as defined in Section 6.9.2,
- in those participants who require permanent discontinuation of study intervention but remain in the study as defined in Section 7.1, and
- during the posttreatment safety follow-up period.

Note: GLP-1 RAs, GIP/GLP-1 RA, and amylin analogues/agonists are not allowed at any time during study. Insulin use for ≥ 14 consecutive days is prohibited.

6.9.2. Rescue Therapy for Severe, Persistent Hyperglycemia

Criteria for initiation of glycemic rescue therapy for severe persistent hyperglycemia

Investigators will be trained on how to apply decision criteria for the timing and method of intervention in participants who do not reach glycemic targets during the treatment period. Glycemic rescue criteria will be primarily determined from fasting SMBG values recorded in the participant's glucometer or central laboratory values, including fasting blood glucose or HbA1c.

Participants must continue administering assigned study intervention unless insulin is used as short-term rescue therapy.

Glycemic rescue therapy should be considered in participants who meet the following criteria in this table.

If a participant has...	
≥ 2 confirmed fasting SMBG/fasting glucose values over at least 2 consecutive weeks of	
>270 mg/dL (15.0 mmol/L)	at any time from Week 0 and prior to Week 4
>240 mg/dL (13.3 mmol/L)	at any time from Week 4 to Week 8
>200 mg/dL (11.1 mmol/L)	at any time after Week 12
HbA1c $\geq 8.5\%$ (69 mmol/mol)	on and after Week 16

Note: Investigators should first confirm that the study participant is fully compliant with the assigned study intervention and that he or she does not have an acute condition causing severe hyperglycemia.

Abbreviations: HbA1c = hemoglobin A1c; SMBG = self-monitoring of blood glucose.

Recommended glycemic rescue therapy

Participants who develop severe persistent hyperglycemia during the treatment period may be candidates for glycemic rescue and should be considered for addition of or dose increase in antihyperglycemic therapies.

The choice of new antihyperglycemic therapy or amount of dose increase will be at the discretion of the investigator or the participant's usual diabetes care physician. Additionally, national standards of care or guidelines for T2D management (for example, American Diabetes Association Standard of Care 2025 or American Diabetes Association/EASD Consensus Report 2022) should be followed, and avoid the prohibited medications listed in Section 6.9.3.

Urgent use of short-term insulin in a hospital or closely monitored setting may be used as rescue therapy and should be administered according to the American Diabetes Association 2025, American Diabetes Association/EASD 2022, or local standards or guidelines, and for as short a duration as possible. Study intervention should be temporarily interrupted and once the acute event resolves, the principal investigator should discuss with the Sponsor to determine if it is appropriate to restart the participant on the study intervention.

The use of rescue therapy should be recorded on a specific CRF page.

6.9.3. Prohibited Medications

Regardless of indication of use, these medications are prohibited during the study, including after permanent discontinuation of study intervention, as rescue therapy, or during the posttreatment safety follow-up period

- amylin analogs/agonists
- GLP-1R agonists
- GIP/GLP-1R agonist, or
- insulin.

Note: Short-term insulin for urgent use is allowed as described in Section 6.9.2.

6.9.4. Prohibited Surgical Treatments, Procedures, or Medications for Weight Management

Any planned elective major surgery during the study should be discussed with the Sponsor.

Bariatric surgical treatments are prohibited during the study. This includes, but is not limited to, gastric bypass, sleeve gastrectomy, and so on.

Procedures for weight management are prohibited during the study. This includes, but is not limited to, liposuction, abdominoplasty, cryolipolysis, and so on.

Endoscopic or device-based therapy for obesity is prohibited. Examples include

- mucosal ablation
- laparoscopic adjustable gastric banding
- gastric artery embolization
- intragastric balloon, or
- duodenal-jejunal endoluminal liner.

Prohibited medications include those intended to promote weight reduction, including prescribed, over the counter, or alternative remedies (Section 5.2).

6.9.5. Management of Hypoglycemia Risk

Participants will be trained in the signs and symptoms of hypoglycemia and its treatment and instructed to record pertinent information about hypoglycemic events in their study diary. Participants who develop persistent or recurrent unexplained hypoglycemia during the treatment period will be asked to reduce the dose of or discontinue any concomitant antihyperglycemic medications, starting preferably with the ones commonly associated with hypoglycemia, for example, sulfonylurea or insulin (see Section 6.9.3). Monitoring for hypoglycemia includes capture of events as described in Sections 8.3.5.1 and 10.3.

7. Discontinuation of Study Intervention and Participant Discontinuation/Withdrawal

This section outlines

- permanent discontinuation of study intervention
- temporary interruption of study intervention
- discontinuation of participants from the study, and
- participants lost to follow-up.

Discontinuation of specific sites or of the study as a whole are handled as part of Appendix 1 (Section 10.1).

7.1. Discontinuation of Study Intervention

When necessary, a participant may be permanently discontinued from study intervention. If so, the participant should discontinue the study intervention (treatment) and will remain in the study and follow procedures for all remaining study visits, as shown in the SoA.

A participant must be permanently discontinued from study intervention if

- the participant requests to discontinue study intervention
- the participant becomes pregnant during the study
- the participant is diagnosed with
 - type 1 diabetes
 - medullary thyroid cancer or multiple endocrine neoplasia type 2 syndrome, or
 - an active or untreated malignancy.

Exception:

- basal cell or squamous cell skin cancer
- in situ prostate cancer, for example, Gleason 6 or lower, or
- Stage 0 noninvasive cervical cancer.
- the participant develops any other TEAE, SAE, or clinically significant laboratory value or other safety reasons for which the investigator believes that permanent study intervention discontinuation is appropriate
- the participant receives or needs prohibited medications during the study described in Section 6.9.2, if the participant will not or cannot discontinue them, or
- if an investigator, site staff performing assessments, or participant is unblinded.

Other possible reasons that may lead to permanent discontinuation of study intervention are as follows:

- if the participant has intolerable GI symptoms despite management as described in Sections 6.6.1. and 10.13.
- if the participant is unable to maintain adequate nutrition or hydration despite management as described in Section 6.6.2.

- if the participant has a BMI ≤ 18.5 kg/m² at any time during the treatment period

Note: The investigator should contact the Lilly medical monitor to discuss whether it is medically appropriate for the participant to continue study intervention.

- the participant has bariatric surgery, body weight reduction procedures, or medications for weight management described in Section 6.9.4. and if the participant will not or cannot discontinue them
- the participant has a systemic hypersensitivity reaction (Section 8.3.5.9.)

Note: If the investigator determines that a systemic hypersensitivity reaction has occurred related to study intervention administration, the participant may be permanently discontinued from the study intervention, and the Sponsor should be notified. If the investigator is uncertain about whether a systemic hypersensitivity reaction has occurred and whether discontinuation of study intervention is warranted, the investigator may consult the Sponsor.

- PHQ-9 score ≥ 15 , or
- answered “yes” to Question 4 or Question 5 on the “Suicidal Ideation” portion of the C SSRS, or
- answered “yes” to any of the suicide-related behaviors on the Suicidal Behavior portion of the C-SSRS.

Note: In the 3 situations above related to PHQ-9 and C-SSRS, it is recommended that the participant should be assessed by a psychiatrist or appropriately trained professional to assist in deciding whether the participant can resume the study intervention.

If it is determined that a participant will permanently discontinue the study intervention due to increased suicidal risk, the participant will be followed within the study until the event leading to discontinuation resolves, stabilizes, or the participant is referred to a mental health professional for further evaluation and care.

If a participant permanently discontinues study intervention for any reason, the participant will complete procedures for an ED of Treatment visit.

The participant should be encouraged to remain in the study and adhere to the study schedule, except in the case of pregnancy.

If the participant is unwilling or unable to return for all applicable study visits, the site should attempt to collect as much follow-up information as possible, especially data collection pertaining to primary and key secondary efficacy endpoints at Week 64 of the treatment period.

7.1.1. Liver Chemistry Stopping Criteria

See Section 8.2.9.3 for hepatic criteria for study intervention interruption or discontinuation.

7.1.2. Temporary Study Intervention Interruption

All efforts should be made to keep participants on study intervention at the randomized dose level and with minimal dose interruptions throughout the study.

In certain situations, other than GI events, after randomization, the investigator may need to temporarily interrupt study intervention. When there is temporary interruption of study intervention, it is expected that the investigator reassesses the participant regularly (in an effort to restart study intervention as soon as medically appropriate). The investigator should make every attempt to restart as soon as possible.

Clinical decision making should consider the approximate 14-day half-life of eloralintide. If 2 consecutive weekly doses are missed and restarting intervention remains medically inappropriate, the investigator must contact the Lilly medical monitor before the next scheduled dose to discuss continued interruption or dose de-escalation. Temporary interruption longer than 3 months should be discussed with the Lilly medical monitor.

Distribution of study intervention at the correct dose will be implemented by the IWRS.

If study intervention interruption is...	then...
2 consecutive doses or fewer	participant resumes the study intervention at the last administered dose level. If the dose interruption occurred within 1 to 2 weeks from a dose-escalation visit, the participant should proceed with the next escalation.
3 consecutive doses or more	Participant restarts study intervention at 1.5 mg and repeats dose-escalation scheme as appropriate Note: in the rare case that a participant has had a dose de-escalation or dose reduction to placebo due to reasons described in Section 6.6 before missing 3 or more consecutive doses, then the participant will restart on placebo in a blinded fashion.
due to an AE	the event is to be documented and followed according to the procedures in Section 8.3 of this protocol.
due to intolerable persistent GI AE	participant should be treated as suggested in Section 6.6.1.
due to poor nutrition or dehydration	participant should be treated as suggested in Section 6.6.2.
due to perceived excessive weight reduction or BMI ≤ 22	participant should be treated as suggested in Section 6.6.3.

Abbreviations: AE = adverse event; BMI = body mass index; GI = gastrointestinal.

7.2. Participant Discontinuation/Withdrawal from the Study

Discontinuation is expected to be uncommon. To minimize the amount of missing data and to enable assessment of study objectives as planned in the study protocol, every attempt will be made to keep participants in the study regardless of study intervention use.

AFAB participants will be discontinued from the study if the participant becomes pregnant.

A participant may withdraw from the study

- at any time at the participant's own request for any reason or without providing any reason
- at the request of the participant's designee, for example, parents or legal guardian
- at the discretion of the investigator for safety, behavioral, compliance, or administrative reasons, or
- if enrolled in any other clinical study involving an investigational product, or enrolled in any other type of medical research judged not to be scientifically or medically compatible with this study.

At the time of discontinuing from the study, if possible, the participant will complete procedures for an early discontinuation visit and post-treatment follow-up, if applicable, as shown in the SoA. If the participant has not already discontinued the study intervention, the participant will be permanently discontinued from the study intervention at the time of the decision to discontinue the study.

If the participant withdraws consent for disclosure of future information, the Sponsor may retain and continue to use any data collected before such a withdrawal of consent. If a participant withdraws from the study, the participant may request destruction of any samples taken and not tested, and the investigator must document this in the site study records.

Participants who agree to provide information relevant to any study endpoint at the end of the study are not considered to have discontinued from the study.

7.3. Lost to Follow up

A participant will be considered lost to follow-up if they repeatedly fail to return for scheduled visits and are unable to be contacted by the study site. Site personnel or designee are expected to make diligent attempts to contact participants who fail to return for a scheduled visit or were otherwise unable to be followed up by the site.

8. Study Assessments and Procedures

Study procedures and their timing are summarized in the SoA.

Immediate safety concerns should be discussed with the Sponsor immediately upon occurrence or awareness to determine if the participant should continue or discontinue study intervention.

Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.

All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.

8.1. Efficacy Assessments

Primary Efficacy Assessment

The primary efficacy measure is to compare the effect of the primary intervention (eloralintide) versus the primary control (placebo) for weight reduction at the primary analysis time point (64 weeks). Body weight measurements will be collected at specific clinic visits in a fasted state and as summarized in the SoA (Section 1.3). Methods for measuring body weight are described in Appendix 7 (Section 10.7).

Key Secondary Efficacy Assessments

The following key secondary efficacy measures will be collected or calculated at the times shown in the SoA:

- body weight
- change in body composition as measured by DXA (Section 8.1.2)
- lipid parameters
- blood pressure, and
- waist circumference.

Other Secondary Efficacy Assessments

The following other secondary efficacy measures will be collected or calculated at the times shown in the SoA:

- body weight
- glycemic control
- body composition measures
- lipid parameters, and
- biomarkers.

8.1.1. Patient-Reported Outcomes

The self-reported questionnaires will be administered at times noted in the SoA in the translated version for the native language of the region.

Preferred administration order is:

1. SF-36v2
2. IWQOL-Lite-CT
3. EQ-5D-5L
4. PGIS Physical Function Weight
5. PGIC Physical Function Weight
6. CoEQ
7. FNQ.

8.1.1.1. Short Form-36 Version 2 Health Survey, Acute Form (1-Week Recall Version)

The SF-36v2 assesses health-related quality of life. The SF-36v2 acute form is a 36-item generic, patient-completed measure designed to assess the following 8 health domains:

1. Physical Functioning
2. Role-Physical
3. Bodily Pain
4. General Health
5. Vitality
6. Social Functioning
7. Role-Emotional, and
8. Mental Health.

The Physical Functioning domain assesses limitations due to health “now”, whereas the remaining domains assess functioning “in the past week.” Each domain is scored individually, and information from these 8 domains is further aggregated into 2 health component summary scores: Physical Component Summary and Mental Component Summary. Items are answered on Likert scales of varying lengths (3-point, 5-point, or 6-point scales). Scoring of each domain and both summary scores are norm based and presented in the form of T-scores, with a mean of 50 and standard deviation of 10; higher scores indicate better levels of function and/or better health (Maruish 2011).

8.1.1.2. EQ-5D-5 Level

The EQ-5D-5L (EuroQol Research Foundation 2019) is a standardized, self-administered instrument for use as a measure of health outcome. It consists of the EQ-5D-5L descriptive system and the EuroQol visual analog scale (EQ VAS). It provides a simple EQ-5D-5L descriptive profile and a single EQ-5D-5L index value for health status that can be used in the clinical and economic evaluation of health care as well as population health surveys. The EQ-5D-5L descriptive system includes 5 items that assess 5 dimensions of health:

- mobility
- self-care
- usual activities
- pain/discomfort, and
- anxiety/depression.

Each dimension has 5 response levels:

- no problems
- slight problems
- moderate problems
- severe problems, and
- extreme problems.

The EQ-VAS score records the respondent's self-rated health on a vertical visual analog scale where the endpoints are labeled as "The best health you can imagine" (100) and "The worst health you can imagine" (0).

8.1.1.3. Impact of Weight on Quality of Life-Lite Clinical Trials Version

The IWQOL-Lite-CT (Kolotkin et al. 2017, 2019) is a 20-item, obesity-specific patient-reported outcomes instrument developed for use in weight management clinical trials.

The IWQOL-Lite-CT assesses 2 primary domains of obesity related health related quality of life (HRQoL): Physical (7 items) and Psychosocial (13 items). A 5-item subset of the Physical domain – the Physical Function composite – is also supported. Items in the Physical Function composite describe physical impacts related to general and specific physical activities. All items are rated on either a 5-point frequency ("never" to "always") scale or a 5-point truth ("not at all true" to "completely true") scale. The three domain scores and total score range from 0 to 100 with higher scores indicating greater functioning.

8.1.1.4. Control of Eating Questionnaire

The CoEQ (Dalton et al. 2015) is a 19-item participant-completed questionnaire that assesses the intensity of food cravings, food types craved, appetite, and mood over the past 7 days. The 11-point numeric rating scale ranges from 0 (not at all) to 10 (extremely) for each of the 19 items. Seventeen of the 19 items are grouped into the 4 domains of

- craving control
- positive mood
- craving for savory foods, and
- craving for sweet foods.

The remaining 2 items, which assess the degree of hunger and fullness, are scored individually. Higher scores represent higher levels of the concept measured in each domain or individual item (for the 2 standalone items).

8.1.1.5. Patient Global Impression of Severity-Physical Function due to Weight

The PGIS Physical Function Weight scale is designed to assess the participants' overall perception of their condition.

This is a single-global item that asks participants to rate how their weight limited their ability to perform physical activities in the past 7 days on a 5-point scale ranging from "not at all limited" to "extremely limited."

8.1.1.6. Patient Global Impression of Change-Physical Function due to Weight

The PGIC Physical Function Weight scale is designed to assess the participants' overall perception of the efficacy of treatment.

This is a single-global item that asks participants to rate the overall change in their ability to perform physical activities due to their weight since starting the study intervention. The responses are based on a 5-point scale ranging from "much better" to "much worse."

8.1.1.7. Food Noise Questionnaire

The FNQ is a 5-item participant-completed questionnaire that assesses thoughts about food in the past 2 weeks (Diktas et al. 2025). Each item of the FNQ is answered using a 5-point, Likert scale ranging from "strongly disagree" (scored as 0) to "strongly agree" (scored as 4). The FNQ Total score is the sum of the item scores, with higher scores indicating greater levels of food noise.

8.1.2. DXA Scan

DXA Total Body Composition

Dual-energy x-ray absorptiometry total body composition with regional analysis provides a measure of physiological response to obesity interventions superior to simple anthropometrics, such as BMI. It is safe and widely available in clinical practice (Kendler et al. 2013), making it a useful method to detect changes in fat and lean body mass in weight management studies.

The risk associated with DXA scan is low, with a radiation exposure (effective dose) of about 5 μ Sv or 0.005 mSv (millisievert). The typical level of background radiation to which the general population is exposed, not including radiation due to medical procedures, has been estimated to be about 2.5 mSv/year (Lewiecki et al. 2016).

DXA instrument

Investigative sites will perform the body composition DXA scans on either Hologic® or Lunar™ DXA scanners with total body composition capabilities according to the imaging guidelines.

Guidance for conducting DXA scans

To obtain consistent and acceptable quality data, investigators will receive detailed instructions on the DXA scan protocol to be used.

The scans will be read and evaluated by a central reader.

Recommendations

- Participants should fast for at least 8 hours without eating or drinking (except water) before each DXA scan. No water should be consumed within 2 hours before each DXA scan.
- Participants will abstain from significant exercise for 24 hours before each DXA scan.
- Perform all scans for a given participant using the same DXA scanner and software.
- Perform all scans for a given participant under similar circumstances, for example, using consistent scan status and performing the scan at the same time of day.

Total body composition assessments for this study

Study participants will undergo a body composition assessment via DXA scans, measured between Visits 2 and 3 prior to receiving study intervention, and Weeks 28 (Visit 10), 64 (Visit 19), or at the time of ED (if at least 1 month from the previous DXA scan), with the intent of evaluating changes in body composition associated with weight reduction, including both fat and lean mass compartments.

If a participant has a body weight, height, and/or width that prohibits the ability to obtain accurate measurements according to the DXA manufacturer's specifications, the participant may still enroll in Study YDAF after consultation with the medical monitor. In this case, missing DXA data will not be a protocol deviation.

8.2. Safety Assessments

Planned time points for all safety assessments are provided in the SoA.

8.2.1. Physical Examinations**Complete physical examination**

A complete physical examination will include, at a minimum, assessments of these systems:

- heart
- lungs
- thyroid
- abdomen
- neurological, and
- extremities, including evaluation for diabetic neuropathy.

A complete physical examination excludes pelvic, rectal, and breast examinations unless clinically indicated.

Investigators should pay special attention to clinical signs related to previous serious illnesses.

Symptom-directed physical assessment may be conducted at the discretion of the principal investigator or qualified personnel as indicated per local regulations based on participant status and standard of care.

Additional assessments

Height, weight, waist circumference, and vital signs will also be measured and recorded. Refer to Section 10.7 for further details.

8.2.2. Vital Signs

For each participant, vital signs measurements should be conducted according to the SoA (Section 1.3). See Section 10.7.

8.2.3. Electrocardiograms

Single 12-lead ECGs will be obtained as outlined in the SoA (Section 1.3). ECGs must be recorded before collecting any blood samples. Participants must be supine for at least 5 to 10 minutes before ECG collection, and remain supine and awake, during ECG collection. ECGs will be interpreted by the investigator (a physician or qualified designee) at the site as soon after the time of ECG collection as practical, and ideally while the participant is still present for immediate participant management, should any clinically relevant findings be identified. The investigator or qualified designee must document their review of the ECG printed at the time of evaluation. Any clinically significant findings from ECGs that result in a diagnosis and that occur after the participant receives the first dose of the study intervention should be reported to the Sponsor or its designee as an AE via the CRF. ECGs may be obtained at additional times when deemed clinically necessary (for example, to assess participants' safety). All digital ECGs will be obtained using centrally provided ECG machines and will be electronically transmitted to a designated central ECG laboratory. The central ECG laboratory will perform a basic quality control check (for example, demographics and study details) and then store the ECGs in a database. At a future time, the stored ECG data may be overread by a cardiologist at the central ECG laboratory for further evaluation of machine-read measurements or to meet regulatory requirements. The machine-read ECG intervals and heart rate may be used for data analysis and report-writing purposes, unless a cardiologist overreading of the ECGs is conducted prior to completion of the final study report (in which case, the overread data would be used).

8.2.4. Clinical Safety Laboratory Tests

See Appendix 2, Section 10.2 for the list of clinical laboratory tests to be performed and the SoA for the timing and frequency.

The investigator must review the laboratory results, document this review, and report any clinically relevant changes occurring during the study as an AE. The laboratory results must be retained with source documents unless a Source Document Agreement or comparable document cites an electronic location that accommodates the expected retention duration. Clinically significant abnormal laboratory findings are those which are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

All laboratory tests with values considered clinically significantly abnormal during participation in the study or within 6 weeks after the last dose of study intervention should be repeated until the values return to normal or baseline or are no longer considered clinically significant by the investigator or medical monitor.

If such values do not return to normal/baseline within a period of time judged reasonable by the investigator, the etiology should be identified and the Sponsor notified.

All protocol-required laboratory assessments, as defined in Appendix 2, Section 10.2, must be conducted in accordance with the SoA, standard collection requirements, and laboratory manual.

If laboratory values from non-protocol specified laboratory assessments performed at an investigator-designated local laboratory require a change in participant management or are considered clinically significant by the investigator, for example, SAE or AE or dose modification, then report the information as an AE.

8.2.5. Pregnancy Testing

Collection of serum pregnancy samples during screening (Visit 1) should be performed within 30 days prior to the first study intervention administration (Visit 3).

If the initial serum test during screening is performed more than 30 days prior to administration of the first dose, a second negative serum pregnancy test performed by the central lab with results confirmed by the investigator is required prior to Visit 3 and administration of the first dose.

Participants who are IOCBP will undergo routine urine pregnancy testing, beginning at study entry (Week 0) and occurring every 4 weeks as indicated in the Schedule of Activities (Section 1.3) via urine pregnancy test. Participants will be provided with at-home urine pregnancy kits with instructions to perform the test with first morning void. Site personnel will contact participants via telephone, telemedicine visits, or both at Weeks 32 (Visit 11), 36 (Visit 12), 44 (Visit 14), 48 (Visit 15), 56 (Visit 17), 60 (Visit 18). Information from these test results will be recorded in participant records.

If the urine pregnancy test is inconclusive at any visit, collect an additional serum pregnancy test and hold study intervention dosing until negative serum pregnancy test is confirmed by site via central laboratory test.

Participants should notify the investigator as soon as possible if they test positive for pregnancy and to hold the next dose of study intervention until additional testing. A positive test should be confirmed with onsite clinical serum testing.

Study drug should not be administered after positive home urine test unless that test is confirmed (at least 2 serum tests separated by 14 days) to be a false positive. Participants who become pregnant during the study should be permanently discontinued from the study (Section 7.2). Participants who become pregnant will complete procedures for an ED visit and safety follow-up, as shown in the SoA (Section 1.3).

Participants should be advised to conduct final at-home urine pregnancy test 4 weeks after V801 and to call the study site for any positive test. Contraception guidance continues 4 weeks after V801.

Details of all pregnancies in AFAB participants and, if indicated, AFAB partners of AMAB participants will be collected as outlined in Sections 8.3.1 and 8.3.2.

8.2.6. Suicidal Ideation and Behavior Risk Monitoring

Participants being treated with eloralintide should be monitored appropriately and observed closely for suicidal ideation and behavior or any other unusual changes in behavior, especially at the beginning and end of the course of intervention, or at the time of dose changes, either increases or decreases. Participants who experience signs of suicidal ideation and behavior should undergo a risk assessment. All factors contributing to suicidal ideation and behavior should be evaluated, and consideration should be given to discontinuation of the study intervention.

Baseline assessment of suicidal ideation and behavior will be monitored during YDAF using C-SSRS.

Participants will be monitored for suicidal ideation through AE collection and by using the C-SSRS.

8.2.6.1. Columbia-Suicide Severity Rating Scale

The C-SSRS is a scale that captures the occurrence, severity, and frequency of suicidal ideation and behavior during the assessment period using a semi-structured interview by a trained rater.

For this study, the Categories Version of the C-SSRS will be used.

The C-SSRS Categories Version is adapted for the assessment of the ideation and behavior categories only. The Intensity of Ideation and Lethality of Behavior sections are removed.

The trained site personnel may administer the C-SSRS and will report the emergence of suicidal ideation, specifically any participant answering “yes” to Question 4 or 5 immediately to the study investigator.

It is important to document a description of any suicidal ideation or behavior or non-suicidal self-injurious behavior captured on the C-SSRS during the participant interview.

For this study, a free text description of the ideation or behavior is documented via the eCOA system.

Timing of collection and AE monitoring

Nonleading AE collection should occur prior to the collection of the C-SSRS.

Follow standard procedures for reporting SAEs.

8.2.7. Depression Monitoring

When informed consent or assent is given, study site personnel should instruct caregivers of study participants about the need to monitor participants for the emergence of depression or unusual changes in behavior, and to report such symptoms immediately to the study investigator.

Study site personnel should monitor participants receiving study intervention for depression or any other unusual changes in behavior, especially at the beginning and end of the course of treatment, or at the time of dose changes, either increases or decreases.

8.2.7.1. Patient Health Questionnaire-9

The PHQ-9 is a validated, participant-reported instrument that assesses the specific diagnostic symptoms that determine the presence and severity of a clinical depressive disorder per the Diagnosis and Statistical Manual for Mental Disorders, 5th Edition (APA, WWW) (Spitzer et.al. 1999; Kroenke et al. 2001; Moriarty et.al. 2015). This scale is validated for use in individuals 18 years of age and older.

The questionnaire assesses the previous 2 weeks.

The PHQ-9 assesses 9 diagnostic symptoms:

- mood
- anhedonia
- sleep disturbance
- loss of energy
- appetite change
- feelings of worthlessness or guilt
- diminished concentration
- psychomotor agitation or retardation, and
- suicidal thoughts or attempts.

Each question has 4 response options, with scores ranging from 0 to 3, corresponding to categories of “not at all” to “nearly every day”. Higher numbers indicate greater dysfunction.

This table describes the interpretation of results. The total score is the sum of scores for the 9 items.

Interpretation of Depression	Total Score
Minimal to none	0-4
Mild	5-9
Moderate	10-14
Moderately severe	15-19
Severe	20-27

8.2.8. Fatigue Assessment Scale

The FAS is a validated 10-item respondent-reported questionnaire that self-assesses both the mental and physical aspects of fatigue (Michielsen et al. 2003).

Each item of the FAS is answered using a five-point, Likert-type scale ranging from 1 (“never”) to 5 (“always”).

8.2.9. Hepatic Safety Monitoring, Evaluation, and Criteria for Study Drug Interruption or Discontinuation

The following tables summarize actions to take based on abnormal hepatic laboratory or clinical changes.

Participants with normal or near normal baseline (ALT, AST, or ALP<1.5x ULN)

If this laboratory value is observed...	Then...		
	Initiate or continue close hepatic monitoring	Initiate comprehensive evaluation	Interrupt or discontinue study intervention
ALT or AST ≥ 3 x ULN	X		
ALP ≥ 2 x ULN	X		
TBL ≥ 2 x ULN ^a	X		
ALT or AST ≥ 5 x ULN	X	X	
ALP ≥ 2.5 x ULN	X	X	
ALT or AST ≥ 3 x ULN with hepatic signs or symptoms ^b	X	X	X
ALT or AST ≥ 5 x ULN for more than 2 weeks	X	X	X
ALT or AST ≥ 8 x ULN	X	X	X
ALT or AST ≥ 3 x ULN and TBL ≥ 2 x ULN ^a or INR ≥ 1.5	X	X	X
ALP ≥ 3 x ULN	X	X	X
ALP ≥ 2.5 x ULN and TBL ≥ 2 x ULN ^a	X	X	X
ALP ≥ 2.5 x ULN with hepatic signs or symptoms ^b	X	X	X

Abbreviations: ALP = alkaline phosphatase; ALT = alanine aminotransferase; AST = aspartate aminotransferase; INR = international normalized ratio; TBL = total bilirubin; ULN = upper limit of normal.

^a In participants with Gilbert's syndrome, the threshold for TBL will be TBL ≥ 3 x ULN, if baseline is <1.5x ULN, and TBL ≥ 2 x baseline, if baseline is ≥ 1.5 x ULN.

^b Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

Participants with elevated baseline (ALT, AST, or ALP $\geq$ 1.5x ULN)

If this laboratory value is observed...	Then...		
	Initiate or continue close hepatic monitoring	Initiate comprehensive evaluation	Interrupt or discontinue study intervention
ALT or AST $\geq$ 2x baseline	X		
ALP $\geq$ 2x baseline	X		
TBL $\geq$ 2x ULN ^a	X		
ALT or AST $\geq$ 3x baseline or $\geq$ 250 U/L (whichever occurs first)	X	X	
ALP $\geq$ 2.5x baseline	X	X	
ALT or AST $\geq$ 2x baseline or $\geq$ 250 U/L (whichever occurs first) with hepatic signs or symptoms ^b	X	X	X
ALT or AST $\geq$ 3x baseline or $\geq$ 250 U/L (whichever occurs first) for more than 2 weeks	X	X	X
ALT or AST $\geq$ 4x baseline or $\geq$ 400 U/L (whichever occurs first)	X	X	X
ALT or AST $\geq$ 2x baseline or $\geq$ 250 U/L (whichever occurs first) and TBL $\geq$ 2x ULN ^a or INR $\geq$ 1.5	X	X	X
ALP $\geq$ 3x baseline	X	X	X
ALP $\geq$ 2.5x baseline and TBL $\geq$ 2x ULN ^a	X	X	X
ALP $\geq$ 2.5x baseline with hepatic signs or symptoms ^b	X	X	X

Abbreviations: ALP = alkaline phosphatase; ALT = alanine aminotransferase; AST = aspartate aminotransferase; INR = international normalized ratio; TBL = total bilirubin; ULN = upper limit of normal.

^a In participants with Gilbert's syndrome, the threshold for TBL will be TBL $\geq$ 3x ULN, if baseline is <1.5x ULN, and TBL $\geq$ 2x baseline, if baseline is $\geq$ 1.5x ULN.

^b Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

8.2.9.1. Close Hepatic Monitoring

If a participant develops any one of these changes, initiate close hepatic monitoring:

Participants with normal or near normal baseline (ALT, AST, or ALP <1.5x ULN)	Participants with elevated baseline (ALT, AST, or ALP $\geq$ 1.5x ULN)
ALT or AST $\geq$ 3x ULN or	ALT or AST $\geq$ 2x baseline
ALP $\geq$ 2x ULN or	ALP $\geq$ 2x baseline
TBL $\geq$ 2x ULN ^a	TBL $\geq$ 2x ULN ^a

Abbreviations: ALP = alkaline phosphatase; ALT = alanine aminotransferase; AST = aspartate aminotransferase; TBL = total bilirubin; ULN = upper limit of normal.

^a In participants with Gilbert's syndrome, the threshold for TBL will be TBL $\geq$ 3x ULN, if baseline is <1.5x ULN, and TBL $\geq$ 2x baseline, if baseline is $\geq$ 1.5x ULN.

Close hepatic monitoring should include these actions:

- Laboratory tests (Appendix 2 [Section 10.2]), including ALT, AST, ALP, total bilirubin level, direct bilirubin, gamma-glutamyl transferase, creatine kinase, and complete blood count with differential, should be checked within 48- to 72-hours of the detection of elevated liver tests to confirm the abnormality and to determine if it is increasing or decreasing.
- If the abnormality persists, clinical and laboratory monitoring should continue at a frequency of 2 to 3 times weekly until levels start normalizing or return to approximate baseline values. Subsequently, the frequency of monitoring may be lowered to once every 1 to 2 weeks, if the participant's clinical condition and lab results stabilize.
- In addition to lab tests, basic evaluation for possible causes of abnormal liver tests should be initiated by the investigator in consultation with the Lilly-designated medical monitor. At a minimum, this evaluation should include physical examination and a thorough medical history, including current symptoms, recent illnesses (for example, heart failure, systemic infection, hypotension, or seizures), recent travel, concomitant medications (including over-the-counter), herbal and dietary supplements, history of alcohol drinking and other substance abuse.

8.2.9.2. Comprehensive Hepatic Evaluation

If a participant develops any one of the following laboratory or clinical changes, initiate a comprehensive hepatic evaluation:

Participants with normal or near normal baseline (ALT, AST, or ALP < 1.5x ULN)	Participants with elevated baseline (ALT, AST, or ALP ≥ 1.5x ULN)
ALT or AST ≥ 5x ULN or	ALT or AST ≥ 3x baseline or ≥ 250 U/L (whichever occurs first) or
ALP ≥ 2.5x ULN or	ALP ≥ 2.5x baseline or
ALT or AST ≥ 3x ULN with hepatic signs or symptoms ^a or	ALT or AST ≥ 2x baseline or ≥ 250 U/L (whichever occurs first) with hepatic signs or symptoms ^a or
ALT or AST ≥ 3x ULN and TBL ≥ 2x ULN ^b or INR ≥ 1.5	ALT or AST ≥ 2x baseline or ≥ 250 U/L (whichever occurs first) and TBL ≥ 2x ULN ^b or INR ≥ 1.5

Abbreviations: ALP = alkaline phosphatase; ALT = alanine aminotransferase; AST = aspartate aminotransferase; INR = international normalized ratio; TBL = total bilirubin; ULN = upper limit of normal.

^a Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

^b In participants with Gilbert's syndrome, the threshold for TBL will be TBL ≥ 3x ULN if baseline is <1.5x ULN, and TBL ≥ 2x baseline if baseline is ≥1.5x ULN.

Comprehensive hepatic evaluation should include these actions:

- Clinical and laboratory monitoring should continue at a frequency of 2 to 3 times weekly until levels start normalizing or return to approximate baseline values.
- At a minimum, comprehensive hepatic evaluation should include physical examination and a thorough medical history, as outlined above, as well as tests for prothrombin time-international normalized ration (PT-INR); tests for viral hepatitis A, B, C, and E; tests for autoimmune hepatitis; and an abdominal imaging study (for example, ultrasound or CT

scan). Remaining sample collected for hepatic evaluation may be stored as an exploratory biomarker sample. These stored samples will be used for the limited purpose of additional hepatic evaluations for up to 15 years.

- Based on the participant's history and initial results, further testing should be considered in consultation with the Lilly-designated medical monitor, including tests for hepatitis D virus (HDV), cytomegalovirus (CMV), Epstein-Barr virus (EBV), acetaminophen levels, acetaminophen protein adducts, urine toxicology screen, Wilson's disease, blood alcohol levels, urinary ethyl glucuronide, and blood phosphatidylethanol (PEth).
- For participants with isolated ALP elevation ≥ 2.5 x ULN without GGT elevation, consider testing for ALP isoenzymes and evaluating for extrahepatic sources such as bone disease (for example, fractures, bone metastases, osteomalacia, Paget's disease), or pregnancy.
- Based on the circumstances and the investigator's assessment of the participant's clinical condition, the investigator should consider referring the participant for a hepatologist or gastroenterologist consultation, and additional tests including magnetic resonance cholangiopancreatography (MRCP), endoscopic retrograde cholangiopancreatography (ERCP), cardiac echocardiogram, or a liver biopsy.
- All the medical information and tests results related to the hepatic monitoring and comprehensive hepatic evaluation should be collected and recorded in CRFs, including the hepatic safety CRF.

8.2.9.3. Study Drug Interruption or Discontinuation

If a participant develops any one of the following laboratory or clinical changes, interrupt the study-drug and continue close monitoring and comprehensive hepatic evaluation as described in Section 8.2.9.1 and 8.2.9.2.

Participants with normal or near normal baseline (ALT, AST, or ALP < 1.5x ULN)	Participants with elevated baseline (ALT, AST, or ALP ≥ 1.5x ULN)
ALT or AST ≥ 3x ULN with hepatic signs or symptoms ^a or	ALT or AST ≥ 2x baseline or ≥ 250 U/L (whichever occurs first) with hepatic signs or symptoms ^a or
ALT or AST ≥ 5x ULN for more than 2 weeks or	ALT or AST ≥ 3x baseline or ≥ 250 U/L (whichever occurs first) for more than 2 weeks or
ALT or AST ≥ 8x ULN or	ALT or AST ≥ 4x baseline or ≥ 400 U/L (whichever occurs first) or
ALT or AST ≥ 3x ULN and TBL ≥ 2x ULN ^b or INR ≥ 1.5 or	ALT or AST ≥ 2x baseline or ≥ 250 U/L (whichever occurs first) and TBL ≥ 2x ULN ^b or INR ≥ 1.5 or
ALP ≥ 3x ULN or	ALP ≥ 3x baseline or
ALP ≥ 2.5x ULN and TBL ≥ 2x ULN ^b or	ALP ≥ 2.5x baseline and TBL ≥ 2x ULN ^b or
ALP ≥ 2.5x ULN with hepatic signs or symptoms ^a	ALP ≥ 2.5x baseline with hepatic signs or symptoms ^a

Abbreviations: ALP = alkaline phosphatase; ALT = alanine aminotransferase; AST = aspartate aminotransferase; INR = international normalized ratio; TBL = total bilirubin; ULN = upper limit of normal.

^a Examples of hepatic signs or symptoms: severe fatigue, nausea, vomiting, right upper quadrant abdominal pain, fever, rash, and/or eosinophilia >5%.

^b In participants with Gilbert's syndrome, the threshold for TBL will be TBL ≥ 3x ULN if baseline is <1.5x ULN, and TBL ≥ 2x baseline if baseline is ≥1.5x ULN.

Interruption or discontinuation of study intervention should include these actions:

- While the participant is not receiving the study intervention, clinical and laboratory monitoring should continue at a frequency of 1 to 3 times weekly until liver tests normalize or return to approximate baseline values.
- If the hepatic event continues past the anticipated end of the study the investigator should consult with the Lilly-designated medical monitor to determine the need for further data collection beyond the end date of the study.
- All the medical information and tests results related to the close hepatic monitoring and comprehensive hepatic evaluation should be collected and recorded in CRFs, including the hepatic safety CRF.
- Resumption of the study intervention after interruption for a hepatic reason can be considered only in consultation with the Lilly designated medical monitor and only if the liver test results returned to near baseline and if a self-limited non-study-drug etiology is identified. Otherwise, the study intervention should be permanently discontinued.

8.2.10. Self-monitoring of Blood Glucose

Glucometer for participant use during the study

Participants will receive a glucometer and related testing supplies for use during the study. Participants must use only the study-provided monitors, provided per local guidelines, during the study.

Frequency of blood glucose monitoring should be individualized based on treatment regimen and participant circumstances and preferences and at the discretion of the investigator. For those who have high blood glucose values, at least twice weekly SMBG is recommended. See Section 6.9.2 for information on rescue therapy for severe, persistent hyperglycemia.

Site staff will train the participant on correct use of the glucometer for self-monitoring blood glucose and reporting of hypoglycemia data in the diary. See Section 6.9.5 for information on hypoglycemia.

Participants should use the glucometer

- whenever hypoglycemia is experienced or suspected
- when there is awareness of increased risk of hypoglycemia related to changes in dietary intake or physical activity
- to check for hyperglycemic events, or
- as directed by the investigator.

Measuring Seven-point SMBG

Participants are required to collect two 7-point SMBGs on nonconsecutive days within 2 weeks prior to specified visits per the SoA. All results will be recorded in the diary according to instructions preceding prespecified visits as shown in the SoA (Section 1.3). These results will be used for the efficacy analysis. Seven-point SMBG profiles will consist of measurements obtained on a single day

- before 3 main meals
- approximately 2 hours after 3 main meals, and
- at bedtime.

8.2.11. Diabetic Retinopathy Screening and Evaluation

Baseline fundus photography should be performed for participants who remain eligible based on screening activities performed at Visit 1, including laboratory results. The overall ophthalmological evaluation of the baseline fundus photographs must be completed prior to randomization to confirm eligibility.

Before randomization, participants currently receiving, planning to receive, or in need of treatment, that is, intravitreal injections of Vascular Endothelial Growth Factor or steroids, focal/grid macular laser surgery, panretinal photocoagulation, or vitrectomy for diabetic retinopathy at screening (Visit 2) or in the period between screening and randomization (Visit 3) will be excluded (see Section 5.2).

All participants will have follow-up fundus photography performed according to the SoA (Section 1.3). Fundus photography may be performed more frequently if warranted based on the

severity of retinopathy at baseline or during the study consistent with screening guidelines for diabetic retinopathy (American Diabetes Association 2025b). Pupilar dilatation is optional and can be done as needed. The process may differ by country; however, it should always follow the specified protocol requirements.

Fundus photography should be performed in both eyes (unless precluded due to severe cataract, prosthesis, or other conditions) using the broadest field of view available to the site or associated qualified eye care professional, including 7-standard views, 4-wide field views, or at a minimum 2-field views with at least 60-degree field of view.

Photographs may be performed at the study site if equipment and staff trained in collecting the required views are available; otherwise, they should be referred to an ophthalmologist or other suitable qualified health care provider.

All fundus photographs will be evaluated by an ophthalmologist or other qualified health care provider experienced in the assessment of diabetic retinopathy and macular edema. The evaluation will include whether diabetic retinopathy is present and whether treatment is recommended.

Fundus photographs deemed unevaluable for any reason will be repeated as recommended by the ophthalmological evaluator. Whenever possible, the same qualified evaluator should review all photographs for an individual participant throughout the study.

Any evidence of diabetic retinopathy, macular edema, or other clinically significant findings on the fundus photographs will require additional ophthalmological follow-up per local standards of care. The use of optical coherence tomography should be encouraged to confirm cases of macular edema.

Results from the overall ophthalmological evaluation, including final diagnoses of nonproliferative retinopathy, proliferative retinopathy, or diabetic macular edema, will be provided to the investigator, and recorded on a specific retinopathy CRF. Any interventions initiated for diabetic retinopathy during the study will be captured on the concomitant medication CRF.

8.3. Adverse Events, Serious Adverse Events, and Product Complaints

The definitions of the following events can be found in Appendix 3, Section 10.3:

- Adverse events (AEs)
- Serious adverse events (SAEs)
- Product complaints (PCs)

These events will be reported by the participant, or, when appropriate, by a caregiver, surrogate, or the participant's legally authorized representative.

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet these definitions and remain responsible for following up events that are serious, considered related to the study intervention or study procedures, or that caused the participant to discontinue the study intervention and study (see Section 7).

Care will be taken not to introduce bias when detecting events. Open-ended and non-leading verbal questioning of the participant is the preferred method to inquire about event occurrences.

After the initial report, the investigator is required to proactively follow each participant at subsequent visits or contacts. All SAEs and AEs of special interest (as defined in Section 8.3.3) will be followed until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3). All AEs of fatigue should be evaluated using the FAS (Section 8.3.5.11).

For product complaints, the investigator is responsible for ensuring that follow-up includes any supplemental investigations as indicated to elucidate the nature or causality. Further information on follow-up procedures is provided in Appendix 3, Section 10.3.

8.3.1. Timing and Mechanism for Collecting Events

This table describes the timing, deadlines, and mechanism for collecting events.

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
Adverse Events					
AEs including SAEs	Signing of the ICF	Participation in study has ended	As soon as possible upon site awareness	AE CRF	N/A
<i>Additional Reporting for Serious Adverse Events</i>					
SAE and SAE updates – prior to start of study intervention and deemed reasonably possibly related to study procedures	Signing of the ICF	Start of study intervention	Within 24 hours of awareness	SAE CRF	SAE paper form
SAE and SAE updates – after start of study intervention	Start of study intervention	Participation in study has ended	Within 24 hours of awareness	SAE CRF	SAE paper form
SAE† – after participant’s study participation has ended and the investigator becomes aware	After participant’s study participation has ended	N/A	Promptly	SAE paper form	N/A

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
Pregnancy					
Pregnancy in participants and partners of participants	After the start of study intervention	70 days after last dose of study intervention	Within 24 hours (see Section 8.3.2)	Pregnancy CRF	Pregnancy paper form
Product Complaints					
PC associated with an SAE or might have led to an SAE	Start of study intervention	End of study intervention	Within 24 hours of awareness	Product Complaint form	N/A
PC not associated with an SAE	Start of study intervention	End of study intervention	Within 1 business day of awareness	Product Complaint form	N/A
Updated PC information	—	—	As soon as possible upon site awareness	Originally completed Product Complaint form with all changes signed and dated by the investigator	N/A
PC (if investigator becomes aware)	Participation in study has ended	N/A	Promptly	Product Complaint form	

Abbreviations: AE = adverse event; CRF = case report form; ICF = informed consent form; N/A = not applicable; PC = product complaint; SAE = serious adverse event.

† Serious adverse events should not be reported unless the investigator deems them to be possibly related to study intervention or study participation.

8.3.2. Collection of Pregnancy Information

8.3.2.1. Participants Who Become Pregnant

The investigator will collect pregnancy information on any participant who becomes pregnant while participating in this study. The initial information will be recorded on the appropriate form and submitted to the Sponsor within 24 hours of learning of a participant's pregnancy.

The participant will be followed to determine the outcome of the pregnancy. The investigator will collect follow-up information on the participant and the neonate and the information will be forwarded to the Sponsor. Generally, follow-up will not be required for longer than 6 to 8 weeks beyond the estimated delivery date. Any termination of pregnancy will be reported, regardless of gestational age, fetal status (presence or absence of anomalies) or indication for the procedure.

While pregnancy itself is not considered to be an AE or SAE, any pregnancy complication or elective termination of a pregnancy for medical reasons will be reported as an AE or SAE.

A spontaneous abortion (occurring at <20 weeks gestational age) or still birth (occurring at ≥20 weeks gestational age) is always considered to be an SAE and will be reported as such.

Any post-study pregnancy related SAE considered reasonably related to the study intervention by the investigator will be reported to the Sponsor as described in protocol Section 8.3.1. While the investigator is not obligated to actively seek this information in former study participants, the investigator may learn of an SAE through spontaneous reporting.

Any participant who becomes pregnant while participating in the study will discontinue study intervention and be withdrawn from the study. If the participant is discontinued from the study, follow the standard discontinuation process and continue directly to the follow-up phase. The follow-up on the pregnancy outcome should continue independent of intervention or study discontinuation.

8.3.2.2. Participants with Partners Who Become Pregnant

When to collect pregnancy information

In most circumstances, the investigator will attempt to collect pregnancy information from a participant's partner who becomes pregnant while the participant is in this study.

After learning about a pregnancy in the partner of a study participant, the investigator

- will obtain permission to release information from the pregnant partner directly, and
- within 24 hours after obtaining this permission, will record pregnancy information on the appropriate form and submit it to the Sponsor.

The partner will be followed to determine the outcome of the pregnancy. Information on the status of the mother and neonate will be forwarded to the Sponsor. Generally, the follow-up will be no longer than 6 to 8 weeks after the estimated delivery date. Any termination of the pregnancy will be reported regardless of gestational age, fetal status (presence or absence of anomalies), or indication for the procedure.

When not to collect pregnancy information

It is not necessary to collect information about a pregnancy in the partner of a study participant in these circumstances

- the partner of the study participant was not exposed to the study intervention via semen during conception or pregnancy, or
- the participant did not contribute the spermatozoid or ova that resulted in the pregnancy.

8.3.3. Adverse Events of Special Interest**8.3.3.1. Hypotension and Related Neurological Signs and Symptoms**

Hypotension events may be identified by spontaneous reporting of symptoms (for example dizziness, blurred vision, fainting) and BP levels. If systolic blood pressure persisted below 90 mmHg or diastolic blood pressure persisted below 60 mmHg or recurrent symptomatic hypotension has occurred, principal investigator should reduce antihypertensive medication dose level or inform participants to consult with their physician. Changes of antihypertensive medication will be recorded in CRF.

8.3.3.2. Acute Renal Events

Renal safety will be assessed based on repeated renal functional assessment as well as assessment of AEs suggestive of acute renal events or worsening of preexisting chronic kidney disease.

Eloralintide may cause nausea, vomiting, and diarrhea. These events may lead to dehydration, which could cause a deterioration in renal function, including acute renal failure.

Participants should notify investigators in case of severe nausea, frequent vomiting or diarrhea, or symptoms of dehydration.

8.3.3.3. Suicidal Ideation and Behavior

Suicidal ideation and behavior will be monitored routinely using C-SSRS (Section 8.2.6.1). Consideration should be given to discontinuing the study medication in participants who experience signs of suicidal ideation or behavior. Refer to Section 7.1 for discontinuation criteria related to suicidal ideation and behavior. At the discretion of the investigator, a participant may be referred to a mental health professional at any time during the study regardless of CSSR-S score.

8.3.3.4. Depression

Participants will be screened at study entry and monitored during the study for depression using PHQ-9 (Section 8.2.7.1). It is recommended that participants who develop moderate depressive symptoms (increase in PHQ-9 total score to 10-14) be assessed for risk of depression. The investigator's assessment of risk should be documented including the rationale for continuing the study intervention, and the participant can be referred to a mental health professional for further evaluation at the discretion of the investigator. Refer to Section 7.1 for discontinuation criteria related to depression monitoring.

8.3.4. Adjudicated Events

The Phase 3 eloralintide program will include an independent adjudication committee to adjudicate suspected cases of the events described in this table in a blinded manner. Any completed adjudications will be utilized in safety analyses.

CV events
Death (CV and non-CV)
Myocardial infarction
Hospitalization for unstable angina
Hospitalization or urgent visit for heart failure
Neurologic events
Stroke
Transient ischemic attack
Pancreatic events
Pancreatitis (acute or chronic)
Pancreatic hyperenzymemia with symptoms
Pancreatic hyperenzymemia that undergoes further diagnostic follow-up
Severe or serious abdominal pain of unknown etiology

Abbreviations: CV = cardiovascular

8.3.5. Safety Topics for Monitoring

These are safety topics of interest that may be anticipated based on published information, potential findings based on drug class even if not observed with the specific investigational molecule (eloralintide), preclinical results, or early clinical results. They can also be events that regulatory bodies recommend be monitored. Such topics would not be recorded as notable, unless fitting one of the notable criteria as defined by the ICH Guidance (E3) on “Structure and Content of Clinical Study Reports”: deaths, SAEs, and AEs leading to permanent discontinuation of the investigational drug.

8.3.5.1. Hypoglycemia

Distribution of glucometers and study diaries

All participants will be provided with glucometers.

All participants will receive a study diary. Participants will be trained about the signs and symptoms of hypoglycemia and its treatment and instructed to record relevant information about hypoglycemic events in their study diary.

Responding to recurrent hypoglycemia in participants taking concomitant antihyperglycemic medication

If a participant develops recurrent unexplained hypoglycemia during the treatment period, the investigator should consider reducing the dose of or discontinuing any concomitant antihyperglycemic medication commonly associated with hypoglycemia, for example, sulfonylurea.

Recording hypoglycemic episodes

Participants will be trained to record hypoglycemic episodes in the study diary.

Site staff should ONLY record hypoglycemia as an AE if the event meets serious criteria. A hypoglycemic event meets serious criteria if it meets the Level 3 Severe definition stated below.

If a hypoglycemic event meets the criteria of severe hypoglycemia, the investigator must record the event as serious on the AE and SAE CRFs and report it to the Sponsor as an SAE.

To avoid duplicate reporting, all consecutive blood glucose values <70 mg/dL (3.9 mmol/L) occurring within a 1-hour period are considered a single hypoglycemic event (Weinberg et al. 2010; Danne et al. 2013).

Investigators should use the following classification of hypoglycemia:

Level 1 hypoglycemia:

Glucose <70 mg/dL (3.9 mmol/L) and $\geq$ 54 mg/dL (3.0 mmol/L): Level 1 hypoglycemia can alert a person to take action such as treatment with fast-acting carbohydrates. Providers should continue to counsel participants to treat hypoglycemia at this glucose alert value.

Level 2 hypoglycemia:

Glucose <54 mg/dL (3.0 mmol/L): Level 2 hypoglycemia is also referred to as documented or blood glucose confirmed hypoglycemia with glucose <54 mg/dL (3.0 mmol/L). This glucose threshold is clinically relevant regardless of the presence or absence of symptoms of hypoglycemia.

Level 3 hypoglycemia:

Severe hypoglycemia (in adults): A severe event characterized by altered mental and/or physical status requiring assistance for treatment of hypoglycemia. For example, participants had altered mental status, and could not assist in their own care, or were semiconscious or unconscious, or experienced coma with or without seizures, and the assistance of another person was needed to actively administer carbohydrate, glucagon, or other resuscitative actions. Glucose measurements may not be available during such an event, but neurological recovery attributable to the restoration of glucose concentration to normal is considered sufficient evidence that the event was induced by a low glucose concentration

- The determination of a hypoglycemic event as an episode of severe hypoglycemia, as defined above, is made by the investigator based on the medical need of the participant to have required assistance and is not predicated on the report of a participant simply having received assistance.
- If a hypoglycemic event meets the criteria of severe hypoglycemia, the investigator must record the event as serious on the AE CRF and report it to Lilly as an SAE.

8.3.5.2. Severe/Serious Gastrointestinal Adverse Reactions

Severe GI AEs, like nausea, vomiting, and diarrhea, may occur. Information on severe GI AEs and antiemetic/antidiarrheal use will be recorded in the AE and concomitant medication CRFs. For details on managing GI AEs, refer to Section 6.6.1.

8.3.5.3. Pancreatitis Events

The diagnosis of acute pancreatitis requires 2 of the following 3 features (Banks 2006, Koizumi 2006):

- abdominal pain, characteristic of acute pancreatitis (that is, epigastric pain radiating to the back, often associated with nausea and vomiting)
- serum amylase (total, pancreatic, or both) and/or lipase $\geq 3 \times$ ULN
- characteristic findings of acute pancreatitis on CT scan or MRI.

If acute pancreatitis is suspected, the investigator should

- obtain appropriate laboratory tests, including pancreatic amylase (p-amylase) and lipase
- perform imaging studies, such as abdominal CT scan with or without contrast, or abdominal MRI, and
- evaluate for possible causes of acute pancreatitis, including alcohol use, gallstone/gall bladder disease, hypertriglyceridemia, and concomitant medications.

Discontinuation for acute pancreatitis

If acute pancreatitis is suspected by the investigator, the participant must temporarily discontinue the use of the study intervention and notify the Sponsor.

When it is deemed clinically safe and the Sponsor has been notified, study participants may resume study intervention at the discretion of the investigator. Resumption of study intervention is not dependent on adjudication outcome.

Case adjudication and data entry

An independent CEC will adjudicate all suspected cases of acute pancreatitis.

Asymptomatic elevation of serum amylase and/or lipase

Serial measures of pancreatic enzymes have limited clinical value for predicting episodes of acute pancreatitis in asymptomatic patients (Nauck et al. 2017; Steinberg et al. 2017a, 2017b). Therefore, further diagnostic follow-up of cases of asymptomatic elevation of pancreatic enzymes (lipase and/or p-amylase $\geq 3 \times$ ULN) is not mandated but may be performed based on the investigator's clinical judgment and assessment of the participant's overall clinical condition.

8.3.5.4. Hepatobiliary Disorders

In cases of elevated liver laboratory tests, initiate hepatic monitoring as outlined in Section 8.2.9.

All events of treatment emergent biliary colic, cholecystitis, cholelithiasis, or other suspected events related to gallbladder disease should be evaluated and additional diagnostic tests performed, as needed.

8.3.5.5. Adjudicated Cardiovascular Events

Nonfatal CV AEs and all deaths will be adjudicated by a committee of physicians external to Lilly. This committee will be blinded to treatment assignment. The nonfatal CV AEs to be adjudicated include

- myocardial infarction
- hospitalization for unstable angina
- hospitalization or urgent visit for heart failure, and
- cerebrovascular events, including cerebrovascular accident (stroke) and transient ischemic attack.

Case adjudication and data entry

An independent CEC will adjudicate all suspected cases of major adverse cardiovascular events. The investigator must first report these events as an AE as described in Section 8.3 and then report them as an endpoint on the CRF with all required source documents provided for adjudication to the CEC (Section 8.3.4). Clinical event reporting begins after randomization.

8.3.5.6. Supraventricular Arrhythmias and Cardiac Conduction Disorders

Participants who develop any event from these groups of disorders should undergo an ECG, which should be submitted to the central reading center. Additional diagnostic tests to determine exact diagnosis should be performed, as needed.

Record the specific diagnosis as an AE.

Record events that meet criteria for serious conditions, as described in Section 10.3.2 as SAEs.

8.3.5.7. Malignancies

Evaluate all events of malignancy or other suspected events related to malignancy and perform additional diagnostic tests as needed.

8.3.5.8. Abuse/Liability Potential

All events of abuse potential should be evaluated, and additional investigations performed as needed.

8.3.5.9. Hypersensitivity Reactions

Many drugs, including oral agents and biologic agents, carry the risk of systemic hypersensitivity reactions. If such a reaction occurs, additional data should be provided to the Sponsor's medical in the designated CRFs.

Sites should have appropriately trained medical staff and appropriate medical equipment available when study participants are receiving study intervention. It is recommended that participants who experience a systemic hypersensitivity reaction be treated per national and international guidelines.

In the case of a suspected systemic hypersensitivity event, additional blood samples should be collected, as described in Section 10.2.1. Laboratory results are provided to the Sponsor's medical monitor via the central laboratory.

8.3.5.10. Injection Site Reactions

Symptoms and signs of a local injection site reaction may include erythema, induration, pain, pruritus, and edema (See also Section 8.3.5.9).

If an injection site reaction is reported by a participant or site staff, it will be reported as an AE and a specific injection site reaction CRF will be used to capture additional information about this reaction for example, injection site pain, degree and area of erythema, induration, pruritus, and edema.

8.3.5.11. Fatigue

The investigator should evaluate for contributors to fatigue including clinical, lifestyle, and concomitant medication related.

During maintenance phase, if fatigue is persistent, lasting longer than 4 weeks, the investigator should evaluate participant's tolerability to fatigue and if needed consider a temporary dose interruption of 1 to 2 weeks. After a 1 to 2-week dose interruption, participant should be clinically re-evaluated and, if medically appropriate, restart previous dose. In cases of persistent or recurrent intolerable fatigue, the investigator should contact Lilly medical monitor.

If AEs related to fatigue, including the AEs lethargy, asthenia, and malaise, are reported by the participant, the site should trigger the FAS (Section 8.2.8) in the eCOA system for participant completion.

8.3.5.12. Bradycardia

Eloralintide may decrease heart rate. TEAEs related to bradycardia and sinus bradycardia should be evaluated. Development of symptomatic bradycardia should be managed at the discretion of the Investigator.

8.3.5.13. Inadequate Nutrition

Investigators should monitor participants for signs and symptoms of inadequate nutrition and provide nutritional guidance. By recognizing early signs of inadequate oral intake, preventative actions can be implemented to reduce the risk of potential complications. Supplementation and treatment of deficiencies should be provided as clinically indicated.

See Section 10.3 for any additional samples and data collections when certain AEs occur.

8.4. Pharmacokinetics

Pharmacokinetic samples will be collected from all randomized participants.

Whole blood samples will be collected for measurement of plasma concentrations of eloralintide will be collected at timepoints as indicated in the table below. Blood samples collected from participants in the placebo arms will not be included in the bioanalyses of drug concentrations.

A maximum of 3 samples may be collected at additional time points during the study if warranted and agreed upon between the investigator and the Sponsor. The timing of sampling may be altered during the course of the study based on newly available data (for example, to obtain data closer to the time of peak plasma concentrations) to ensure appropriate monitoring.

Instructions for the collection and handling of biological samples will be provided by the Sponsor. The actual date and time (24-hour clock time) of every dose and each sample collected will be recorded on the appropriate forms.

Samples will be used to evaluate the PK of eloralintide. Samples collected for analyses of eloralintide plasma concentrations may also be used to evaluate safety or efficacy aspects related to concerns arising during or after the study.

Refer to Section 9.3.5 for population PK and PK/PD analysis.

Study intervention concentration information that may unblind the study will not be reported to investigative sites or blinded personnel until the study has been unblinded.

Pharmacokinetic Sampling Times

Week from Randomization	PK Sample Relative to Last Study Intervention Dose
Week 0	Predose and 2 to 6 hour postdose
Week 4	Predose ^a
Week 12	Random (at any time during visit)
Week 16	Predose ^a and 2 to 6 hour postdose
Week 28	Predose ^a and 12 to 48 hour postdose
Week 40	Predose ^a and 48 to 120 hour postdose
Week 52	Predose ^a
Week 64	Random (at any time during visit)
Early discontinuation ^b	Random (at any time during visit)
6 weeks after last treatment dose	Random (at any time during visit)

Abbreviations: PK = pharmacokinetics.

^a Participants should take study intervention only after PK samples are taken on the predose PK sample visits.

^b Early discontinuation PK samples should be collected regardless of study week at which the discontinuation occurs (even if after Week 64).

8.4.1. Bioanalysis

Plasma samples will be analyzed at laboratories approved by the Sponsor and stored at facilities designated by the Sponsor.

Concentrations of eloralintide will be analyzed using validated liquid chromatography mass spectrometry (LC/MS) methods. Analyses of samples collected during placebo treatment are not planned.

Bioanalytical samples collected to measure eloralintide concentrations will be retained for a maximum of 1 year following last participant visit for the study. During this time, samples remaining after the bioanalyses may be used for exploratory analyses such as metabolism work, protein binding, and/or bioanalytical method cross-validation.

8.5. Pharmacodynamics

Samples to assess the PD properties of eloralintide are included under efficacy or other laboratory measures.

8.6. Genetics

A whole blood sample will be collected from participants to enable DNA isolation for exploratory pharmacogenetics analysis as specified in the SoA, where local regulations allow.

Samples will not be used to conduct unspecified disease or population genetic research either now or in the future.

Samples may be used to investigate variable exposure or response to eloralintide and to investigate genetic variants thought to play a role in obesity, diabetes mellitus, and related clinical traits or complications. Assessment of variable response may include evaluation of AEs or differences in PD, mechanistic, safety, or efficacy measures.

See Appendix 5 (Section 10.5) for Information regarding genetic research and Appendix 1 (Section 10.1.12) for details about sample retention and custody.

8.7. Biomarkers

Stored whole blood (epigenetics), serum and plasma samples for exploratory biomarker research will be collected at times specified in the SoA, where local regulations allow.

All samples will be used but not limited to develop or validate new assays, research methods or diagnostic tools, or conduct research on the drug targets, disease process, variable response to eloralintide, pathways associated with obesity or overweight and T2D, and related clinical traits, or complications, mechanism of action of eloralintide and/or other amylin agonists, or other relevant disease states.

All samples will be coded with the participant number. These samples and any data generated can be linked back to the participant only by the investigative site staff. The cadence of collection is described in the Schedule of Activities (Section 1.3).

See Section 10.1.12 for details about sample retention.

8.8. Immunogenicity Assessments

At visits and time specified in the SoA, venous blood samples will be collected to determine antibody production against eloralintide. To interpret the results of immunogenicity, a PK sample will be collected at the same time points as the immunogenicity sample. All samples for immunogenicity should be taken predose where applicable and possible. In the event of drug hypersensitivity events, additional samples will be taken as specified in Section 10.2.1.

The detection and characterization of antibodies to eloralintide will be performed using a validated assay under supervision of the Sponsor. ADA may be further characterized for cross-reactivity to native amylin, their ability to neutralize eloralintide, and/or their ability to neutralize native amylin. Immunogenicity samples may be used for control or validation of the ADA assay.

TE ADA are defined in section 9.3.7.

See Section 10.1.12 for details on sample retention.

8.9. Health Economics

Health economics or medical resource utilization and health economics parameters are not evaluated in this study.

9. Statistical Considerations

The statistical analysis plan will be finalized prior to unblinding, and it will include a more technical and detailed description of the statistical analyses described in this section. This section is a summary of the planned statistical analyses of the most important endpoints, including primary and key secondary endpoints.

Unblinding details are specified in the blinding and unblinding plan.

9.1. Statistical Hypotheses

The null hypotheses corresponding to the primary objective are as follows:

- $H_{3,1,0}$, $H_{6,1,0}$, $H_{9,1,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean percent change in body weight from baseline at Week 64.

The null hypotheses corresponding to the key secondary objectives are as follows:

- $H_{1.5,1,0}$: No difference in 1.5 mg QW eloralintide compared to placebo with respect to the mean percent change in body weight from baseline at Week 64.
- $H_{3,2,0}$, $H_{6,2,0}$, $H_{9,2,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the percentage of participants achieving body weight reduction of 5% from baseline at Week 64.
- $H_{3,3,0}$, $H_{6,3,0}$, $H_{9,3,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the percentage of participants achieving body weight reduction of 10% from baseline at Week 64.
- $H_{3,4,0}$, $H_{6,4,0}$, $H_{9,4,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the percentage of participants achieving body weight reduction of 15% from baseline at Week 64.
- $H_{3,5,0}$, $H_{6,5,0}$, $H_{9,5,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the percentage of participants achieving body weight reduction of 20% from baseline at Week 64.
- $H_{3,6,0}$, $H_{6,6,0}$, $H_{9,6,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean percent change in total body fat mass from baseline to Week 64.
- $H_{3,7,0}$, $H_{6,7,0}$, $H_{9,7,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean change in waist circumference from baseline at Week 64.
- $H_{\text{pooled},8,0}$: No difference in 1.5 mg, 3 mg, 6 mg, 9 mg QW eloralintide pooled compared to placebo with respect to the mean percent change in triglyceride from baseline at Week 64.
- $H_{\text{pooled},9,0}$: No difference in 1.5 mg, 3 mg, 6 mg, 9 mg QW eloralintide pooled compared to placebo with respect to the mean percent change in non-HDL from baseline at Week 64.

- $H_{\text{pooled},10,0}$: No difference in 1.5 mg, 3 mg, 6 mg, 9 mg QW eloralintide pooled compared to placebo with respect to the mean change in systolic blood pressure from baseline at Week 64.
- $H_{3,11,0}$, $H_{6,11,0}$, $H_{9,11,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean change in HbA1c from baseline at Week 64.
- $H_{3,12,0}$, $H_{6,12,0}$, $H_{9,12,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean change in HbA1c from baseline at Week 64 in participants with baseline HbA1c $\geq 7\%$
- $H_{3,13,0}$, $H_{6,13,0}$, $H_{9,13,0}$: No difference in 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean change in fasting glucose (mg/dL) from baseline at Week 64.

9.1.1. Multiplicity Adjustment

Multiplicity-adjusted analyses will be performed on the primary and key secondary objectives in the study to control the overall family-wise type I error rate at a 2-sided alpha level of 0.05. The graphical multiple testing procedure described by Bretz et al. (2009, 2011) will be used. This approach is a closed testing procedure; hence, it strongly controls the family-wise error rate across all hypotheses tested (Alosh et al. 2014).

Unless otherwise specified, the “treatment regimen” estimand is planned to support future product registration and the “efficacy estimand” will be considered for use in scientific disclosures and other purposes. Since these 2 types of estimands are intended for distinct purposes, no multiplicity adjustment will be made for conducting separate analyses on the same objectives.

The detailed graphical testing procedure will be included in the statistical analysis plan prior to the unblinding.

9.2. Analyses Sets

The following participant analysis sets are defined:

Participant Analysis Set	Description
Entered participants	All participants who sign informed consent.
Randomized participants	All participants who are randomly assigned a study intervention.
Safety participants	All participants who are randomly assigned a study intervention and who take at least 1 dose of study intervention.

The following data points sets are defined:

Data Points Sets	Description
Treatment regimen estimand data points set	All data points obtained during the treatment period defined as at or after baseline and up to the last visit within the planned treatment period, regardless of study intervention discontinuation or initiation of prohibited weight management treatments.

Data Points Sets	Description
Safety data points set	All data points obtained during the intervention period and the follow-up period defined as at or after baseline and up to the date of study withdrawal, including the follow-up period and regardless of study intervention discontinuation or initiation of prohibited weight management treatments.
Efficacy estimand data points set	All data points obtained during the treatment period defined as at or after baseline and up to the earliest date of study intervention discontinuation or initiation of prohibited weight management treatments.

The following data points sets are defined for all glycemic-related parameters. The parameters in scope include fasting glucose and HbA1c.

Data Points Sets	Description
Treatment regimen estimand data points set	All data points obtained during the treatment period defined as at or after baseline and up to the last visit within the treatment period, regardless of study intervention discontinuation or initiation of prohibited weight management treatments or glycemic rescue therapies
Safety data points set	All data points obtained during the treatment period and the follow-up period defined as at or after baseline and up to the date of study withdrawal, including the follow-up period and regardless of study intervention discontinuation or initiation of prohibited weight management treatments or glycemic rescue therapies
Efficacy estimand data points set	All data points obtained during the treatment period defined as at or after baseline and up to the earliest date of study intervention discontinuation or initiation of prohibited weight management treatments or glycemic rescue therapies.

9.3. Statistical Analyses

9.3.1. General Considerations

Statistical analysis will be the responsibility of Lilly or its designee. Any change to the data analysis methods described in the protocol will require an amendment ONLY if it changes a principal feature of the protocol. Any other change to the data analysis methods described in the protocol, and the justification for making the change, will be described in the SAP or the clinical study report. Additional exploratory analyses of data will be conducted, as deemed appropriate.

Unless otherwise noted, all tests for superiority will be conducted at the 2-sided alpha level of 0.05, and CIs will be calculated at 95%, 2-sided. For objectives controlled for type 1 error, the alpha will be allocated by the graphical testing scheme.

Baseline is defined as the last available non-missing measurement prior to the first dosing of study intervention, unless otherwise specified. If there are no doses of study intervention, baseline will be defined as the last available non-missing measure on or prior to randomization.

General descriptions of analyses

For continuous measures, summary statistics will include sample size, mean, standard deviation, median, minimum, and maximum for both the actual and the change from baseline measurements, model-based estimates (MBE) and standard errors derived from the analysis models will also be displayed for the change from baseline measurements. Treatment comparisons will be displayed showing the treatment difference model-based estimates and the 95% CIs for the treatment differences, along with the p-values for the treatment comparisons.

The analysis model to make comparisons between treatment groups relative to continuous efficacy measurements with only 1 postbaseline assessment or under treatment regimen estimand will be an ANCOVA using robust inference (Ye et al. 2022, FDA 2023). The model will include treatment group, strata, the continuous covariate of baseline values and their interactions with treatment group.

For continuous measures evaluated at multiple postbaseline visits and analysis guided by efficacy estimand, a restricted maximum likelihood based-MMRM analysis (Wang and Du 2022) will be used. The analysis model to make comparisons between treatment groups relative to continuous measurements assessed over time will include fixed effects of visit, and treatment, stratification factors, and baseline measurement all nested within visits. The variance-covariance matrix will be estimated using sandwich variance estimators (Tsiatis 2007).

For categorical measures, summary statistics will include sample size, frequency, and percentages. For safety categorical measures, risk difference and its 95% CI will be provided. Fisher's exact test or Pearson's chi-square test will be used for treatment comparisons without covariate adjustment, if applicable.

For binary measures, a logistic regression model with treatment group and strata as fixed effects and the continuous baseline value and their interactions with treatment group as a covariate will be used to examine the treatment difference with missing endpoints imputed. The unconditional treatment group effect will be assessed by risk difference and relative risk using the marginal standardization method, where the treatment group-specific risk will be derived from the counterfactual risks for each participant that are predicted with the fitted logistic model (FDA 2023; Ye et al. 2023). The estimated treatment group-specific risk, risk difference, relative risk, p-value, and 95% CI will be presented.

For stratification factors at baseline, a strata variable is defined for statistical modeling to consist of joint levels of all stratification factors.

For time-to-event measures, Kaplan–Meier method will be used for estimation of cumulative event-free survival rates over time and Cox proportional hazards regression analysis will be used to compare hazard rates among treatments.

Unless specified otherwise, safety and tolerability assessments will be guided by an estimand comparing safety of eloralintide doses with placebo in safety participants using safety data point set.

Missing data should be minimized as the best precaution. Handling of missing, unused, and spurious data is addressed prospectively in the overall statistical methods described in the protocol and in the SAP, where appropriate. Adjustments to the planned analyses will be described in the final clinical study report.

Participants will be analyzed according to the treatment group to which they were randomly assigned.

In case of mis-stratification, actual strata variable value collected at baseline, instead of as-randomized value will be used for covariate adjustment.

Other statistical methods may be used, as appropriate, and details will be described in the SAP.

9.3.2. Treatment Group Comparability

9.3.2.1. Participant Disposition

A detailed description of participant disposition will be provided.

Frequency counts and percentages of all randomly assigned participants will be presented by treatment groups. A listing of randomly assigned participants not receiving study intervention will be provided. All participants who discontinue the study will be identified, and the extent of their participation in the study will be reported. If known, a reason for their discontinuation will be given. The primary reasons for discontinuation will be listed and will be summarized by treatment groups. Kaplan–Meier analyses of time from randomization to premature discontinuation from study and premature discontinuation from study intervention by treatment group will be provided.

9.3.2.2. Participant Characteristics

Demographics and other baseline characteristics will be summarized by treatment group for all randomly assigned participants.

9.3.2.3. Concomitant Therapy

Concomitant medications, including previous therapy, will be summarized descriptively by treatment group for the randomly assigned participants during treatment period.

9.3.2.4. Treatment Compliance

Treatment compliance is defined as taking at least 75% and no more than 125% of scheduled study intervention during the treatment period. Frequency counts and percentages of participants compliant to study intervention will be summarized by treatment group using the safety participants during the treatment period.

9.3.3. Primary Endpoint(s)/Estimand(s) Analysis

There will be 2 estimands of interest in comparing efficacy of eloralintide doses with placebo. No multiplicity adjustment is planned between these two estimands.

“Treatment regimen” estimand:

Primary analysis aligned to treatment regimen estimand will be conducted using the randomly assigned participants with treatment regimen estimand data points set. The primary analysis

model will be ANCOVA characterizing percent change in body weight from baseline at the 64-week visit. The model includes the following as covariates:

- treatment group as a factor variable
- strata as a factor variable
- baseline value (of the dependent variable)
- interaction between strata variable and treatment group, and
- interaction between baseline value and treatment group.

The estimated treatment group effect and comparison between eloralintide 3 mg, 6 mg, 9 mg, and placebo will be reported together with the variability estimated using the robust inference (Ye et al. 2022). The associated 2-sided 95% CI and corresponding p-values will also be reported.

Missing data at Week 64 will be imputed in a fashion consistent with what the values would likely have been had they been collected, namely using multiple imputation based on data retrieved from participants who permanently discontinued study intervention but continued in the study with non-missing measurements (that is, retrieved dropouts). The statistical inference over multiple imputed datasets will be guided by the method proposed by Rubin (1987). The details of the imputation strategy will be specified in the statistical analysis plan.

A multiple imputation-based tipping-point analysis is planned as a sensitivity analysis to assess the robustness of primary efficacy results. Additional details on sensitivity analyses will be provided in the SAP.

“Efficacy” estimand:

Primary analysis aligned to efficacy estimand will be conducted using the randomly assigned participants with efficacy data points set, that is, data collected before the occurrence of any ICEs (Section 9.2). All the longitudinal observations at each scheduled postbaseline visit will be included in the analysis. The primary analysis model will be a mixed model for repeated measures model characterizing percent change in body weight over time.

The MMRM model will include the following terms:

- treatment group as a factor variable
- visit as a factor variable
- strata as a factor variable
- baseline value (of the dependent variable)
- interaction between treatment group, strata variable, and visit, and
- interaction between treatment group, baseline value, and visit.

The estimated treatment group effect and comparison between eloralintide 3 mg, 6 mg, 9 mg, and placebo at the scheduled visits will be reported together with the variability estimated using the robust inference (Wang and Du 2023). The addition of 3-way interaction terms is not intended to estimate the heterogeneity effect but to provide robustness and efficiency for the estimate of treatment comparisons on the unconditional effect. The associated 2-sided 95% CI and corresponding p-values will also be reported. An unstructured covariance matrix by each treatment group will be used to model the within-participant errors, assuming heteroscedasticity and the measurements for different participants are independent.

Through the MMRM, the efficacy measures (after the ICEs) will be implicitly imputed as if participants did not have ICEs.

Additional details of the statistical modeling will be provided in the SAP.

9.3.4. Secondary Endpoint(s)/Estimand(s) Analysis

All these key secondary analyses will be addressed using two estimands as mentioned in Section 9.3.3. Additional details, including the strategy for controlling overall Type 1 error rate at a 2-sided alpha of 0.05 of primary and key secondary endpoint evaluation for each of the estimand and analysis details for all secondary endpoints and exploratory endpoints, will be provided in the SAP.

Continuous endpoints for secondary endpoints will be analyzed using similar methods as primary endpoint outlined in Section 9.3.3, aligning with respective estimands of interest.

The binary outcomes will be analyzed using the following procedure:

- Impute missing underlying continuous data that determines the binary value at visit of interest,
 - **“Treatment regimen” estimand** - using the same imputation strategy on the underlying continuous endpoint that determines the binary value.
 - **“Efficacy” estimand** - Imputing the missing continuous-valued measurements at the scheduled visits using the randomized participants with efficacy estimand data points set assuming “missing at random” with multiple imputation.
- Transforming the observed and imputed continuous value to the binary value (Ma et al. 2022).
- Fitting a logistic regression model to the data with the following terms:
 - treatment group as a factor variable
 - strata as a factor variable
 - baseline value (of the dependent variable)
 - interaction between strata variable and treatment group, and
 - interaction between baseline value and treatment group.
- Providing for the visit of interest estimates and inferences of unconditional treatment effects defined by the risk difference and/or risk ratio based on the delta method using the formula provided (Ye et al. 2023).
- Deriving the final inference using Rubin’s rule by combining estimates from multiply imputed datasets.

A strata variable is defined for statistical modeling to consist of joint levels of stratification factors.

Details for additional secondary analyses will be provided in the SAP.

9.3.5. Pharmacokinetic/Pharmacodynamic Analyses

Eloralintide concentration data will be analyzed using a population PK approach using nonlinear mixed-effects modeling techniques implemented in the NONMEM® or equivalent software.

The relationships between eloralintide doses and/or concentration and selected efficacy, tolerability, and safety endpoints may be characterized. Additionally, the impact of intrinsic and extrinsic participant factors such as age, weight, gender, and renal function on eloralintide PK and/or PD parameters may be evaluated.

If ADA titers are detected from immunogenicity testing, then the impact of immunogenicity on eloralintide PK or any relevant PD parameters may also be evaluated via PK/PD analysis or graphically. Additional analyses may also be conducted if they are deemed appropriate. Further details will be provided in the PK/PD analysis plan.

9.3.6. Safety Analyses

Safety analyses will be conducted using the safety participants and the safety data points set, unless otherwise specified.

AEs will be coded from the actual term using the Medical Dictionary for Regulatory Activities and reported by preferred terms within system organ class. Selected notable AEs of interest may be reported using high-level terms or Standardized Medical Dictionary for Regulatory Activities Queries. Summary statistics will be provided for incidence of

- TEAEs
- SAEs
- study discontinuation due to AEs
- study intervention discontinuation due to AEs
- deaths, and
- other CV endpoints.

Counts and percentages of participants experiencing AEs will be reported for each treatment group. Details regarding safety analysis will be provided in the SAP and program safety analysis plan.

9.3.6.1. Adverse Events of Special Interest and Other Safety Topics

The analysis details for the AEs of special interest and other safety topics (as defined in Section 8.3.3) will be provided in the SAP.

9.3.6.2. Gastrointestinal Events

Summaries and analyses for incidence, prevalence, and severity of the GI events of nausea, vomiting, constipation, and diarrhea will be provided by treatment group.

9.3.6.3. Depression, Suicidal Ideation, and Behavior

In addition to the summary of TEAEs, suicide-related thoughts and behaviors occurring during treatment will be summarized based on responses to the C-SSRS consistent with the C-SSRS Scoring and Data Analysis Guide [C-SSRS]. Depression-related symptoms will be summarized based on responses to the PHQ-9.

The analysis details will be provided in the SAP.

9.3.6.4. Central Laboratory Measures, Vital Signs, and Electrocardiograms

Actual and change from baseline to postbaseline values of central laboratory measures, vital signs, and selected ECG parameters will be summarized at each scheduled visit. Continuous variables, as well as the change from baseline for these variables, will be analyzed by MMRM models, as described in Section 9.3.1. The percentages of participants with treatment-emergent abnormal, high, or low measures (including laboratory, vital, and ECG parameters) will be summarized and compared between treatment groups using Fisher's exact test or Pearson's Chi-square test.

9.3.7. Other Analyses

Subgroup analyses

Subgroup analyses to assess consistency of the effect of eloralintide across subgroups guided by treatment regimen estimand for the primary endpoints will be detailed in the SAP. The following subgroups will be considered (but not limited to):

- age group (<65, ≥65 years)
- sex (AMAB, AFAB)
- baseline background oral AHMs (according to potential effect on body weight change)
- baseline BMI (≤30, >30 kg/m² and <35, ≥35 and <40, ≥40 kg/m²)
- race
- baseline HbA1c (≤6.5%, >6.5%)
- ethnicity
- country/region
- prior incretin use (incretin naïve/incretin experienced), and
- menopausal status (yes/no).

Note: Menopausal status is applicable to participants AFAB only.

If the number of participants is too small (less than 10%) within a subgroup, then the subgroup categories may be redefined prior to unblinding the study. Additional subgroup analyses on the key secondary endpoints may also be performed. Further details on the statistical analysis will be provided in the SAP.

Immunogenicity

The frequency and percentage of participants with preexisting ADA and TE ADA to eloralintide may be tabulated. Treatment-emergent ADA are defined as those with a titer 2-fold (1 dilution) greater than the minimum required dilution if no ADA were detected at baseline (treatment-induced ADA) or those with a 4-fold (2 dilutions) increase in titer compared to baseline if ADA were detected at baseline (treatment-boosted ADA).

For TE ADA+ participants, the distribution of maximum titers may be reported. The frequency of TE ADA+ participants with cross-reactive antibodies against native amylin and/or neutralizing antibodies against eloralintide and/or native amylin may also be tabulated, if available.

The relationship between the presence of antibodies and the PK parameters, PD response, safety, and efficacy of eloralintide may be assessed.

9.4. Interim Analysis

No efficacy interim analyses are planned for this study. If an unplanned efficacy interim analysis is deemed necessary for reasons other than a safety concern, the protocol must be amended.

An independent DMC will be established for interim safety monitoring and have the responsibility to review unblinded interim safety analysis results. A statistical analysis center independent from the Sponsor will perform the data analysis for the DMC. The first interim analysis will be performed approximately 6 months after the first patient enters treatment of the first study in the program. These interim safety analyses will occur approximately every 6 months to monitor the safety of the participants in the study.

The study team will not have access to the unblinded data. Only the DMC is authorized to evaluate unblinded interim safety analyses. Study sites will receive information about interim safety results only if they need to know for the safety of their participants. Details of the DMC are described in Section 10.1.5.2 and the DMC charter.

The database lock will occur after all randomized participants complete the 64-week treatment period and the posttreatment safety follow-up period of the study or discontinue the study early. The primary objective of the study will be assessed following this database lock.

Details regarding maintaining the blind during the conduct of this study will be specified in the blinding and unblinding plan document.

9.5. Sample Size Determination

A sample size of 1,035 participants (230 participants for the placebo, 3 mg, 6 mg, and 9 mg eloralintide arms and 115 participants to the 1.5 mg eloralintide arm) provides more than 95% power to demonstrate superiority of 3 mg, 6 mg, and/or 9 mg eloralintide to placebo at a 2-sided significance level of 0.016 (as primary objectives), and 94% power to demonstrate superiority of 1.5 mg eloralintide to placebo at a 2-sided significance level of 0.05 (as a key secondary objective) with regards to mean percent change in body weight from baseline to Week 64, under the following assumptions:

- use of a 2-sample t-test to compare treatment means for the treatment-regimen estimand, taking 15% treatment discontinuation for eloralintide treatment groups and 30% treatment discontinuation for placebo group into consideration
- at least 6% greater mean body weight reduction for 3 mg, 6 mg, and 9 mg of eloralintide compared to placebo
- at least 4.5% greater mean body weight reduction for 1.5 mg of eloralintide compared to placebo, and
- common standard deviation of 11%.

The power calculations were performed under the treatment-regiment estimand with the software nQuery Version 8.4.1.0.

10. Supporting Documentation and Operational Considerations

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted in accordance with the protocol and with the following:

- consensus ethical principles derived from international guidelines including the Declaration of Helsinki and Council for International Organizations of Medical Sciences (CIOMS) International Ethical Guidelines
- applicable ICH Good Clinical Practice (GCP) Guidelines

The protocol, protocol amendments, ICF, Investigator Brochure, and other relevant documents, for example, advertisements, must be submitted to an IRB/IEC by the investigator and reviewed and approved by the IRB/IEC before the study is initiated.

Any amendments to the protocol will require IRB/IEC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

Protocols and any substantial amendments to the protocol will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following:

- providing written summaries of the status of the study to the IRB/IEC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/IEC
- notifying the IRB/IEC of SAEs or other significant safety findings as required by IRB/IEC procedures
- providing oversight of study conduct for participants under their responsibility and adherence to requirements of 21 Code of Federal Regulations (CFR), ICH guidelines, the IRB/IEC, European regulation 536/2014 for clinical studies (if applicable), and all other applicable local regulations
- reporting to the Sponsor or designee significant issues related to participant safety, participant rights, or data integrity.

Investigator sites are compensated for participation in the study as detailed in the clinical trial agreement.

10.1.2. Financial Disclosure

Investigators and sub-investigators will provide the Sponsor with sufficient, accurate financial information as requested to allow the Sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are

responsible for providing information on financial interests during the course of the study and for 1 year after completion of the study

10.1.3. Informed Consent Process

The investigator or the investigator's representative will explain the nature of the study, including the risks and benefits, to the potential participant or their legally authorized representative, and answer all questions regarding the study.

Potential participants must be informed that their participation is voluntary. Participants or their legally authorized representatives will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH guidelines, privacy and data protection requirements, where applicable, and the IRB/IEC or study center.

The medical record must include a statement that written informed consent was obtained before the participant was entered in the study and the date the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICF.

Revised consents must be appropriately obtained using the correct approved ICFs for applicable study participants in accordance with Sponsor and ERB consenting guidance.

A copy of the ICF(s) must be provided to the participant or the participant's legally authorized representative and is kept on file.

10.1.4. Data Protection

Participants will be assigned a unique identifier by the Sponsor to protect the participant's personal data. Any participant information, such as records, datasets, or tissue samples that are transferred to the Sponsor will contain the identifier only. Participant names or any information which would make the participant identifiable will not be transferred.

The participant must be informed that the participant's personal study-related data will be used by the Sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant who will be required to give consent for their data to be used as described in the informed consent. This is done by the site personnel through the informed consent process.

The participant must be informed through the informed consent by the site personnel that their medical records may be examined by Clinical Quality Assurance auditors or other authorized personnel appointed by the Sponsor, by appropriate IRB/IEC members, and by inspectors from regulatory authorities.

The Sponsor has processes in place to ensure information security, data integrity, and data protection. These processes address management of data transfer, and prevention and management of unauthorized access, disclosure, dissemination, alteration or loss of information or personal data. These processes include appropriate contingency plan(s) for appropriate and timely response in the event of a data security breach.

The transfer of personal data is subject to appropriate safeguards through contractual agreements and processes. The Sponsor's processes are compliant with local privacy laws and relevant legislations including the General Data Protection Regulation (GDPR).

10.1.5. Committees Structure

10.1.5.1. Clinical Endpoint Committee

An independent CEC with membership external to the Sponsor will be responsible for event adjudication in a blinded fashion.

The independent CEC will adjudicate events listed in Section 8.3.4. The CEC charter will contain the final detailed event definitions used for adjudication.

10.1.5.2. Data Monitoring Committee

An independent, external DMC will be responsible for reviewing unblinded data during the study.

The committee will include, at a minimum, a physician with appropriate expertise and a statistician.

Access to the unblinded data will be limited to the DMC and the external statistical analysis center statisticians who are providing the analysis of the data. These statisticians will be independent from the study team. The study team will not have access to the unblinded data. Only the DMC is authorized to evaluate unblinded interim safety analyses.

Details about the membership, purpose, responsibilities, and operation will be included in the DMC charter.

10.1.6. Dissemination of Clinical Study Data

Reports

The Sponsor will disclose a summary of study information, including tabular study results, on publicly available websites where required by local law or regulation.

The summary of results will be posted within the time frame specified by local law or regulation. If the study remains ongoing in some countries and a statistical analysis of an incomplete dataset would result in analyses lacking scientific rigor (for example, underpowered) or compromise the integrity of the overall analyses (for example, trial not yet unblinded), the summary of results will be submitted within 1 year after the end of the study globally or as soon as available, whichever is earlier.

Data

The Sponsor provides access to all individual participant data collected during the trial, after anonymization, with the exception of pharmacokinetic or genetic data.

Data are available to request 6 months after the indication studied has been approved in the US and EU and after primary publication acceptance, whichever is later. No expiration date of data requests is currently set once data are made available.

Access is provided after a proposal has been approved by an independent review committee identified for this purpose and after receipt of a signed data-sharing agreement.

Data and documents, including the study protocol, statistical analysis plan, clinical study report, and blank or annotated case report forms, will be provided in a secure data-sharing environment for up to 2 years per proposal.

For details on submitting a request, see the instructions provided at www.vivli.org.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRFs unless transmitted to the Sponsor or designee electronically (for example, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the CRF.

The investigator must maintain accurate documentation (source data) that supports the information entered in the CRF. This includes laboratory tests, medical records, and clinical notes.

The investigator must review and confirm that data entries are accurate and complete throughout the duration of the study, by physically or electronically signing the CRF, as instructed by the Sponsor. All completed CRFs must be signed prior to archival.

The investigator must permit study-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct or remote access to source documents.

Quality tolerance limits (QTLs) will be predefined to identify systematic issues that can impact participant safety and/or reliability of study results. These predefined parameters will be monitored during the study and important excursions from the QTLs and remedial actions taken will be summarized in the clinical study report.

Monitoring details describing strategy, for example, risk-based initiatives in operations and quality such as risk management and mitigation strategies and analytical risk-based monitoring, methods, responsibilities and requirements, including handling of noncompliance issues and monitoring techniques are provided in the Monitoring Plan.

The Sponsor or designee is responsible for the data management of this study including quality checking of the data.

The Sponsor assumes accountability for actions delegated to other individuals, for example, contract research organizations.

The Sponsor or designee will perform monitoring to confirm that data transcribed into the CRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP, and all applicable regulatory requirements.

Records and documents, including signed ICFs, pertaining to the conduct of this study must be retained by the investigator for the time period outlined in the Clinical Trial Agreement (CTA) unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the Sponsor. No records may be transferred to another location or party without written notification to the Sponsor.

In addition, the Sponsor or its representatives will periodically check a sample of the participant data recorded against source documents at the study site. The study may be audited by the Sponsor or its representatives, or regulatory agencies at any time. Investigators will be given notice before an audit occurs.

Data Capture System

The investigator is responsible for ensuring the accuracy, completeness, legibility, and timeliness of the data reported to the Sponsor.

Electronic Data Capture System

An EDC will be used in this study for the collection of CRF data. The investigator maintains a separate source for the data entered by the investigator or designee into the Sponsor-provided EDC system. The investigator is responsible for the identification of any data to be considered source and for the confirmation that data reported are accurate and complete by signing the CRF.

Clinical Outcome Assessments

Additionally, electronic Clinical Outcome Assessment (eCOA) data (participant-focused outcome instrument) will be directly recorded by the participant or investigator site personnel, into an instrument (for example, hand held smart phone or tablet). The eCOA data will serve as the source documentation and the investigator does not maintain a separate written or electronic record of these data.

Data collected via the Sponsor-provided data capture system(s) will be stored at third-party (at third parties). The investigator will have continuous access to the data during the study and until decommissioning of the data capture system(s). Prior to decommissioning, the investigator will receive or access an archival copy of pertinent data for retention.

Data managed by a central vendor, such as laboratory test data, will be stored electronically in the central vendor's database system and reports will be provided to the investigator for review and retention. Data will subsequently be transferred from the central vendor to the Sponsor data warehouse.

Data from complaint forms submitted to the Sponsor will be encoded and stored in the global product complaint management system.

10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator's site.

Data reported on or entered in the CRF and are transcribed from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

Definition of what constitutes source data can be found in the site confirmation of source data.

10.1.9. Study and Site Start and Closure

Study start

The study start date is the date on which the clinical study will be open for recruitment of participants.

First act of recruitment

The first act of recruitment is the opening of the first site.

Study or site termination

The Sponsor or Sponsor's designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the Sponsor. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time, provided there is reasonable cause and sufficient notice is given in advance of the intended termination.

Reasons for the early closure of a study site by the Sponsor or investigator may include but are not limited to

- for study termination due to discontinuation of further study intervention development
- for site termination due to
 - failure of the investigator to comply with the protocol, the requirements of the IRB/IEC or local health authorities, the Sponsor's procedures, or GCP guidelines
 - inadequate recruitment, evaluated after a reasonable amount of time, of participants by the investigator, or
 - total number of participants included earlier than expected.

If the study is prematurely terminated or suspended, the Sponsor shall promptly inform the investigators, the IECs/IRBs, the regulatory authorities, and any contract research organization(s) used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator shall promptly inform the participant and should assure appropriate participant therapy and/or follow-up.

10.1.10. Publication Policy

In accordance with the Sponsor's publication policy, the results of this study will be submitted for publication by a peer-reviewed journal.

10.1.11. Investigator Information

Researchers with appropriate education, training, and experience, as determined by the Sponsor, will participate as investigators in this clinical study.

10.1.12. Sample Retention

Sample retention enables use of new technologies, response to regulatory questions, and investigation of variable response that may not be observed until later in the development of eloralintide or after eloralintide becomes commercially available.

Sample Type	Custodian	Maximum Retention Period after the Last Participant Visit ^a
Exploratory biomarkers	Sponsor or designee	Up to 7 years
Pharmacokinetic	Sponsor or designee	1 year
Genetics	Sponsor or designee	Up to 7 years
Immunogenicity	Sponsor or designee	Up to 15 years

^a Retention periods may differ locally.

10.2. Appendix 2: Clinical Laboratory Tests

The tests detailed in the table below will be performed by the central laboratory.

Local laboratory results are only required in the event that the central laboratory results are not available in time for either study intervention administration and/or response evaluation. If a local sample is required, it is important that the sample for central analysis is obtained at the same time. Additionally, if the local laboratory results are used to make either a study intervention decision or response evaluation, the results must be recorded.

In circumstances where the Sponsor approves local laboratory testing in lieu of central laboratory testing (in the table below), the local laboratory must be qualified in accordance with applicable local regulations.

Protocol-specific requirements for inclusion or exclusion of participants are detailed in Section 5 of the protocol.

Additional tests may be performed at any time during the study as determined necessary by the investigator or required by local regulations.

Investigators must document their review of the laboratory safety results.

Laboratory or analyte results that could unblind the study will not be reported to investigative sites or other blinded personnel until the study has been unblinded.

Clinical Laboratory Tests	Comments
Hematology	Assayed by Lilly-designated laboratory
Hemoglobin	
Hematocrit	
Erythrocyte count (RBCs – red blood cells)	
Mean cell volume	
Mean cell hemoglobin	
Mean cell hemoglobin concentration	
Leukocytes (WBCs – white blood cells)	
Differential – absolute	
Neutrophils	
Lymphocytes	
Monocytes	
Eosinophils	
Basophils	
Platelets	
Clinical Chemistry Panel	Assayed by Lilly-designated laboratory
Sodium	
Potassium	
Magnesium	

Clinical Laboratory Tests	Comments
Chloride	
Bicarbonate	
Total bilirubin	
Direct bilirubin	
Alkaline phosphatase (ALP)	
Alanine aminotransferase (ALT)	
Aspartate aminotransferase (AST)	
Gamma-glutamyl transferase (GGT)	
Blood urea nitrogen (BUN)	
Creatinine	
Creatine kinase (CK)	
Total protein	
Albumin	
Calcium	
Phosphorus	
Glucose	
Uric acid	
Lipid Panel	Assayed by Lilly-designated laboratory unless stated otherwise
High-density lipoprotein cholesterol (HDL-C)	
Low-density lipoprotein cholesterol (LDL-C) (Direct measure)	Direct measurement will be performed if triglycerides exceed maximum value for calculation.
Very low-density lipoprotein cholesterol (VLDL-C)	Calculated by Lilly-designated laboratory
Total cholesterol	
Triglycerides	
Non-high-density lipoprotein cholesterol (Non-HDL-C)	Calculated by Lilly-designated laboratory
Urinalysis	Assayed by Lilly-designated laboratory
Specific gravity	
pH	
Protein	
Glucose	
Ketones	
Bilirubin	
Urobilinogen	
Blood	
Nitrite	
Urine leukocyte esterase	
Microscopic examination of sediment	Only if abnormalities are detected

Clinical Laboratory Tests	Comments
Hormones	Assayed by Lilly-designated laboratory unless stated otherwise
Serum pregnancy	
Urine pregnancy	Evaluated locally
Follicle-stimulating hormone (FSH)	
Urine Chemistry	Assayed by Lilly-designated laboratory
Albumin	
Creatinine	
Calculations	Generated by Lilly-designated laboratory
eGFR (CKD-EPI creat-cystC)	
Urinary albumin/creatinine ratio (UACR)	
Renin/aldosterone ratio	Results will not be provided to the investigative sites
Pharmacokinetic Samples – eloralintide concentration	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites
Eloralintide PK	
Other Testing	Assayed by Lilly-designated laboratory
Free fatty Acids	Results will not be provided to the investigative sites
Aldosterone	Results will not be provided to the investigative sites
Apolipoprotein C3	Results will not be provided to the investigative sites
Apolipoprotein B	Results will not be provided to the investigative sites
C-peptide	Results will not be provided to the investigative sites
C-reactive protein, high-sensitivity (hsCRP)	Results will not be provided to the investigative sites
Cystatin-C	
HbA1c	
Insulin	Results will not be provided to the investigative sites
Intact proinsulin	Results will not be provided to the investigative sites
Lipase	
Pancreatic amylase	
Renin	Results will not be provided to the investigative sites
Thyroid-stimulating hormone (TSH)	

Clinical Laboratory Tests	Comments
Micronutrient panels	Assayed by Lilly-designated laboratory
Vitamin B1 (Thiamine)	
Vitamin B9 (Folate)	
Vitamin B12	
Vitamin D (25-hydroxy vitamin D)	
Iron Panel (iron, ferritin, TIBC, transferrin saturation)	
Zinc	
Genetic Sample	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites
Whole blood	
Exploratory Biomarker Stored Samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites
Serum	
Plasma (EDTA)	
Whole blood – epigenetics	
Immunogenicity Samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites
Anti-eloralintide antibodies	
Anti-eloralintide antibodies neutralization	

Abbreviations: CKD-EPIcreat-cystC = chronic kidney disease epidemiology collaboration using creatinine and cystatin C; eGFR = estimated glomerular filtration rate; EDTA = ethylenediaminetetraacetic acid; eGFR = estimated glomerular filtration rate; HbA1c = hemoglobin A1c.

10.2.1. Laboratory Samples to be Obtained at the Time of a Systemic Hypersensitivity Event

Purpose of collecting samples after a systemic hypersensitivity event

The samples listed in this appendix are not collected for acute study participant management. The Sponsor will use the laboratory tests results from these samples to characterize hypersensitivity events across the clinical development program.

When to collect samples after a systemic hypersensitivity event occurs

Collect the samples listed below if a systemic hypersensitivity event is suspected. The timing should be as designated in the table, assuming the participant has been stabilized.

Obtain follow-up predose samples at the next regularly scheduled laboratory sample collection (ideally prior to the next dose after the event) to assess post-event return-to-baseline values.

Timing	Laboratory Test ^a
Collect from 30 minutes to 4 hours after the start of the event. Note: The optimal collection time is from 1 to 2 hours after the start of event.	total tryptase
Collect only if not already collected on the same day as the event. Note: If collecting, collect up to 12 hours after the start of the event.	eloralintide anti-drug antibodies (ADA) eloralintide concentration.

^a All samples for hypersensitivity testing will be assayed by Lilly-designated laboratory. Results will not be provided to the study site. If samples are not collected or are collected outside the specified time period, this will not be considered a protocol deviation.

What information to record

Record the date and time when the samples are collected.

Allowed additional testing for participant management

The investigator may perform additional tests locally, if clinically indicated, for acute study participant management.

10.2.2. Nutritional Monitoring Panel to be Obtained in Case of Suspected Poor Nutrition

Purpose of collecting samples in case of suspected poor nutrition

Investigators should monitor participants for signs and symptoms of poor nutrition. Refer to Sections 5.3.1, 5.3.2, and 6.6.2.

When to collect samples in case of suspected malnutrition

Collect the samples listed below if malnutrition is suspected.

Category	Laboratory Test
Special tests	Vitamin A
	Vitamin B1 (Thiamine)
	Vitamin B9 (Folate)
	Vitamin B12
	Vitamin D (25-hydroxy vitamin D)
	Vitamin K ₁ ^a
Hematology	Complete blood count ^b
Clinical chemistry	Clinical chemistry panel ^c
	Zinc
	Intact parathyroid hormone (iPTH)
Iron panel	Serum iron
	Total iron binding capacity (TIBC)
	Transferrin saturation (%)
	Ferritin

^a Results of vitamin K1 may not be available in all countries.

^b Refer to Section 10.2 for contents of complete blood count panel.

^c Refer to Section 10.2 for contents of clinical chemistry panel.

Note: Tests will be assayed by a Lilly-designated laboratory and results will be provided to the investigative sites where allowed (or appropriate).

What information to record

Record the date and time when the samples are collected.

Allowed additional testing for participant management

The investigator may perform additional tests locally, if

- clinically indicated, for acute study participant management, or
- a test is not available centrally.

10.3. Appendix 3: Adverse Events and Serious Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-up, and Reporting

10.3.1. Definition of AE

- An AE is any untoward medical occurrence in a participant administered a pharmaceutical product and which does not necessarily have a causal relationship with the study intervention. An AE can therefore be any unfavourable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of a medicinal (investigational) product, or investigational combination product, whether or not related to the medicinal (investigational) product or investigational combination product.

Events meeting the AE definition

- Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments, for example, ECG, radiological scans, and vital signs measurements, including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator, that is, not related to progression of underlying disease.
- Exacerbation of a chronic or intermittent pre-existing condition including either an increase in frequency and/or intensity of the condition.
- New condition detected or diagnosed after study intervention administration even though it may have been present before the start of the study.
- Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction.
- Medication error, misuse, or abuse of IMP, including signs, symptoms, or clinical sequelae.
- Lack of efficacy or failure of expected pharmacological action per se will not be reported as an AE or SAE. Such instances will be captured in the efficacy assessments. However, the signs, symptoms, and/or clinical sequelae resulting from lack of efficacy will be reported as AE or SAE if they fulfill the definition of an AE or SAE.

Events NOT meeting the AE definition

In line with clinical practice for this disease state, and based on the defined criteria for adverse events and related clarifications (ICH 1994, FDA 2009, EMA 2017, CFR 2024), the following are not considered adverse events:

- Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.
- The disease or disorder being studied or expected progression, signs, or symptoms of the disease or disorder being studied, unless more severe than expected for the participant's condition.
- Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital).

- Anticipated day-to-day fluctuations of pre-existing disease(s) or condition(s) present or detected at the start of the study that do not worsen.

10.3.2. Definition of SAE

An SAE is defined as any untoward medical occurrence that, at any dose, meets one or more of the criteria listed:

- Results in death
- Is life-threatening
 - The term *life-threatening* in the definition of *serious* refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event, which hypothetically might have caused death, if it were more severe.
- Requires inpatient hospitalization or prolongation of existing hospitalization
 - In general, hospitalization signifies that the participant has been admitted to hospital or emergency ward (usually involving at least an overnight stay) for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious criteria, the event is serious. When in doubt as to whether hospitalization occurred or was necessary, the AE should be considered serious.
 - Hospitalization for elective treatment of a pre-existing condition that did not worsen from baseline is not considered an AE.
- Results in persistent disability/incapacity
 - The term disability means a substantial disruption of a person's ability to conduct normal life functions.
 - This definition is not intended to include experiences of relatively minor medical significance, such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma, for example, sprained ankle, which may interfere with or prevent everyday life functions but do not constitute a substantial disruption.
- Is a congenital anomaly/birth defect
 - Abnormal pregnancy outcomes, for example, spontaneous abortion, fetal death, stillbirth, congenital anomalies, and ectopic pregnancy, are considered SAEs.
- Other situations:
 - Medical or scientific judgment should be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations, such as important medical events that may not be immediately life-threatening or result in death or hospitalization but may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed in the above definition. These events should usually be considered serious.

- Examples of such events include invasive or malignant cancers, intensive treatment in an emergency room or at home for allergic bronchospasm, blood dyscrasias or convulsions that do not result in hospitalization, or development of drug dependency or drug abuse.

10.3.3. Definition of Product Complaints

Product complaint

A product complaint is any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, durability, reliability, safety, effectiveness or performance of a study intervention. When the ability to use the study intervention safely is impacted, the following are also product complaints:

- deficiencies in labeling information, and
- use errors for device or drug-device combination products due to ergonomic design elements of the product.

Product complaints related to study interventions used in clinical trials are collected to ensure the safety of participants, monitor quality, and to facilitate process and product improvements.

If the participant identifies a product complaint or a problem with the study intervention, investigators will instruct participants to contact the site as soon as possible so that the situation can be assessed.

An event may meet the definition of both a product complaint and an AE/SAE. In such cases, it should be reported as both a product complaint and as an AE/SAE.

Device deficiencies are product complaints.

10.3.4. Recording and Follow-Up of AE and/or SAE and Product Complaints

AE, SAE, and product complaint recording

When an AE/SAE/product complaint occurs, it is the responsibility of the investigator to review all documentation (for example, hospital progress notes, laboratory reports, and diagnostics reports) related to the event.

The investigator will then record all relevant AE/SAE/product complaint information in the participant's medical records, in accordance with the investigator's normal clinical practice. AE/SAE information is reported on the appropriate CRF page and product complaint information is reported on the Product Complaint Form.

Note: An event may meet the definition of both a product complaint and an AE/SAE. In such cases, it should be reported as both a product complaint and as an AE/SAE.

It is **not** acceptable for the investigator to send photocopies of the participant's medical records to the Sponsor or designee in lieu of completion of the CRF page for AE/SAE and the Product Complaint Form for product complaints.

There may be instances when copies of medical records for certain cases are requested by the Sponsor or designee. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission to the Sponsor or designee.

The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE/SAE.

Assessment of intensity

The investigator will make an assessment of intensity for each AE and SAE reported during the study and assign it to one of the following categories:

- **Mild:** A type of AE that is usually transient and may require only minimal treatment or therapeutic intervention. The event does not generally interfere with usual activities of daily living.
- **Moderate:** A type of AE that is usually alleviated with additional specific therapeutic intervention. The event interferes with usual activities of daily living, causing discomfort but poses no significant or permanent risk of harm to the research participant.
- **Severe:** A type of AE that interrupts usual activities of daily living, or significantly affects clinical status, or may require intensive therapeutic intervention. An AE that is assessed as severe should not be confused with a SAE. Severe is a category utilized for rating the intensity of an event, and both AEs and SAEs can be assessed as severe.

An event is defined as “serious” when it meets at least one of the predefined outcomes as described in the definition of an SAE, NOT when it is rated as severe.

Assessment of causality

The investigator is obligated to assess the relationship between study intervention and each occurrence of each AE/SAE. The investigator will use clinical judgment to determine the relationship.

A “reasonable possibility” of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out.

Alternative causes, such as underlying disease(s), concomitant therapy, and other risk factors, as well as the temporal relationship of the event to study intervention administration will be considered and investigated.

The investigator will also consult the IB in their assessment.

The investigator **must** review and provide an assessment of causality for each AE/SAE and document this in the medical notes.

There may be situations in which an SAE has occurred and the investigator has minimal information to include in the initial report to the Sponsor or designee. However, it is very important that the investigator always make an assessment of causality for every event before the initial transmission of the SAE data to the Sponsor or designee.

The investigator may change their opinion of causality in light of follow-up information and send a SAE follow-up report with the updated causality assessment.

The causality assessment is one of the criteria used when determining regulatory reporting requirements.

Follow-up of AEs and SAEs

- The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations as medically indicated or as requested by the Sponsor or designee to elucidate the nature and/or causality of the AE or SAE as fully as possible. This may include additional laboratory tests or investigations, histopathological examinations, or consultation with other health care professionals.

10.3.5. Reporting of SAEs**SAE reporting via an electronic data collection tool**

The primary mechanism for reporting an SAE will be the electronic data collection tool.

If the electronic system is unavailable, then the site will use the SAE paper form (see next section) in order to report the event within 24 hours.

The site will enter the SAE data into the electronic system as soon as it becomes available.

After the study is completed at a given site, the electronic data collection tool will be taken off-line to prevent the entry of new data or changes to existing data.

If a site receives a report of a new SAE from a study participant or receives updated data on a previously reported SAE after the electronic data collection tool has been taken off-line, then the site can report this information on a SAE paper form (see next section) or to the Sponsor by telephone.

Contacts for SAE reporting can be found in study training material

SAE reporting via paper form

Facsimile transmission of the SAE paper form is the preferred method to transmit this information to the Sponsor.

Initial notification via telephone does not replace the need for the investigator to complete and sign the SAE CRF pages within the designated reporting time frames.

Contacts for SAE reporting can be found in the SAE Paper Form.

10.3.6. Regulatory Reporting Requirements**SAE regulatory reporting**

Prompt notification by the investigator to the Sponsor of a SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The Sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The Sponsor will evaluate the reported SAEs, including confirmation of relatedness and assessment of expectedness. The Sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, Institutional Review Boards (IRB)/Independent Ethics Committees (IEC), and investigators.

An investigator who receives an investigator safety report describing a SAE or other specific safety information (for example, summary or listing of SAEs) from the Sponsor will review and then file it along with the IB and will notify the IRB/IEC, if appropriate according to local requirements.

10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Definitions

Word/Phrase	Definition
Individuals assigned female at birth (AFAB)	Individuals assigned the female sex based on external genitalia and/or genetic or medical information. In addition, if these individuals are of reproductive potential, they are potentially capable of gestating a fetus, and thus are capable of exposing an egg, embryo, or fetus to study intervention or drug effects.
Individuals assigned male at birth (AMAB)	Individuals assigned the male sex based on external genitalia and/or genetic or medical information. In addition, if these individuals are of reproductive potential, they are not capable of gestating a fetus but are capable of exposing a fetus to study drug or drug effects via their semen. Individuals AMAB are considered to be not of reproductive potential if they have had bilateral orchiectomy (orchidectomy) with or without penectomy.
Individuals of childbearing potential (IOCBP) ^a	Adult individuals AFAB are considered IOCBP unless they are INOCBP. Note: Adult individuals AFAB who are receiving hormone therapy as part of gender transition are considered IOCBP unless they meet the conditions outlined below for INOCBP.
Individuals not of childbearing potential (INOCBP) ^b	Individuals AFAB are considered INOCBP if they are not capable of producing ova or embryo, and/or are not capable of potentially gestating a fetus. Such individuals include those who • have a congenital anomaly such as Müllerian agenesis, resulting in confirmed infertility • are infertile due to surgical sterilization, or • are postmenopausal. Acceptable surgical sterilization methods are hysterectomy, bilateral salpingo-oophorectomy, bilateral salpingectomy, or bilateral oophorectomy.

Postmenopausal state ^c	The postmenopausal state is defined as an individual: • at any age at least 6 weeks post-surgical bilateral oophorectomy with or without hysterectomy; or • aged at least 40 years and up to 55 years with an intact uterus, not on hormone therapy^c, who has had cessation of menses for at least 12 consecutive months without an alternative medical cause, AND with a follicle-stimulating hormone ≥ 40 mIU/mL; or • 55 years or older not on hormone therapy, who has had at least 12 months of spontaneous amenorrhea, or • aged at least 55 years with a diagnosis of menopause prior to starting hormone replacement therapy.
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- a IOCBP is inclusive of the concept of women of childbearing potential (WOCBP or WCBP), a term often used in literature and regulatory guidance documents.
- b INOCBP is inclusive of the concept of women not of childbearing potential (WNOCBP).
- c The individual **should not** be taking medications during amenorrhea such as oral contraceptives, hormone replacement therapy (HRT), gonadotropin-releasing hormone, anti-estrogens, selective estrogen receptor modulators, or chemotherapy that could induce transient amenorrhea. Individuals on HRT and those whose postmenopausal status cannot be confirmed will be required to comply with the protocol contraception requirements if they wish to continue HRT during the study. Otherwise, they must discontinue HRT to allow confirmation of postmenopausal status before study enrollment.

10.4.2. Contraception Guidance

IOCBP who are completely abstinent as their preferred and usual lifestyle or exclusively engage in sexual relations with other individual(s) who are AFAB as their preferred and usual lifestyle must follow the rules in this table.

Must...	Must not...
agree to either remain abstinent or exclusively engage in sexual relations with other individual(s) who are AFAB, and not plan a pregnancy during the study	• use periodic abstinence methods ○ calendar ○ ovulation ○ symptothermal, or ○ post-ovulation • declare abstinence just for the duration of a trial, or • use the withdrawal method.

IOCBP who are NOT completely abstinent as their preferred and usual lifestyle, or who do NOT exclusively engage in sexual relations with other individual(s) who are AFAB as their preferred and usual lifestyle, must follow the rules in this table.

Must...
Agree to use 1 highly effective method of contraception together with a barrier method of contraception.
These methods of contraception must be used during the study and for at least 70 days after the last dose of the study intervention.

The table below describes contraception guidance for individuals AMAB.

Participant Population	Contraception Guidance
All individuals AMAB...	should refrain from sperm donation for the duration of the study and for 70 days after the last dose of the study intervention.
Individuals AMAB with partner(s) who are IOCBP ^a ...	must remain abstinent (if this is their preferred and usual lifestyle), or use condoms and at least 1 additional effective method of contraception for the duration of the study and for 70 days after the last dose of the study intervention.
Individuals AMAB with partner(s) who are pregnant ^a ...	must • remain abstinent (if this is their preferred and usual lifestyle), or • use condoms for the duration of the study and for 70 days after the last dose of the study intervention.
Individuals AMAB who exclusively engage in sexual relations with other individual(s) who are AMAB, as their preferred and usual lifestyle...	are not required to use contraception.

- a Individuals AMAB who have undergone orchiectomy but not penectomy must use condoms during sex, but the partner who is IOCBP is not required to use an additional form of contraception. Individuals AMAB who have undergone orchiectomy and penectomy are not required to use condoms.

Examples of different methods of contraception^a:

Methods	Examples
Highly effective contraception (less than 1% failure rate)	• fallopian tubal sterilization methods other than bilateral salpingectomy (laparoscopic bipolar electrocoagulation, plastic ring application on the uterine tubes, fallopian tube ligation, hysteroscopic sterilization). Note: Bilateral salpingectomy is indicative of permanent sterilization. Please see the INOCBP definition above. • combined (estrogen and progestogen-containing) hormonal contraception associated with inhibition of ovulation: ○ oral ○ intravaginal (vaginal ring) ○ transdermal (for example, contraceptive patch) (only for individuals <198 pounds or 90 kg). • progestogen-only hormonal contraception associated with inhibition of ovulation

Methods	Examples
	○ oral ○ injectable ○ implantable. • total abstinence • sexual relationships exclusively between individuals who are assigned the same sex at birth • vasectomy – for individuals AMAB in clinical trials and for the partner of an individual AFAB (if only sexual partner) • fallopian tube implants (if confirmed by hysterosalpingogram), or • intrauterine devices (IUD) with or without hormone-releasing system.
Effective contraception	• progestogen-only hormonal contraception, where inhibition of ovulation is not the primary mode of action • penile condom with or without spermicide • vaginal condom with or without spermicide • diaphragm with spermicide • cervical sponge with spermicide • cervical cap with spermicide. Note: Penile and vaginal condoms should not be used in combination.
Ineffective methods of contraception whether used alone or in any combination	• spermicide alone • periodic abstinence • fertility awareness (calendar method, temperature method, cervical mucus, or symptothermal) • withdrawal • postcoital douche, or • lactational amenorrhea.

^a Ablation (endometrial or uterine) is not considered a form of contraception.

10.5. Appendix 5: Genetics

Use/Analysis of DNA

Genetic variation may impact a participant's response to study intervention, susceptibility to, and severity and progression of disease. Variable response to study intervention may be due to genetic determinants that impact drug absorption, distribution, metabolism, and excretion; mechanism of action of the drug; disease etiology; and/or molecular subtype of the disease being treated.

Therefore, where local regulations and IRB/IEC allow, a whole blood sample will be collected for DNA analysis from consenting participants.

DNA samples will be used for research related to eloralintide or Obesity or Overweight with Type 2 Diabetes and related diseases. They may also be used to develop tests/assays including diagnostic tests related to eloralintide and/or other amylin agonists and Obesity or Overweight with Type 2 Diabetes. Genetic research may consist of the analysis of one or more candidate genes or the analysis of genetic markers throughout the genome (as appropriate).

The samples may be analyzed as part of a multi-study assessment of genetic factors involved in the response to eloralintide or study interventions of this class to understand study disease or related conditions.

The results of genetic analyses may be reported in the clinical study report or in a separate study summary.

The Sponsor will store the DNA samples in a secure storage space with adequate measures to protect confidentiality.

The samples will be retained while research on eloralintide continues but no longer than 7 years or other period as per local requirements.

10.6. Appendix 6: Liver Safety: Suggested Actions and Follow-up Assessments

Hepatic Evaluation Testing

See Section 8.2.9 for guidance on appropriate test selection from the table below. Note that testing for some analytes may not be available from the central lab or local lab in certain regions due to local availability of assay or regulatory requirements. If testing is not available, this will not be considered a protocol deviation.

Local testing may be performed *in addition to central testing* when necessary for immediate participant management. If local testing is performed in lieu of central testing, the results should be recorded in the appropriate local lab CRF. For example, if immediate participant management circumstances preclude collection of central lab samples – for example, emergency room visit or hospitalization. The local laboratory must be qualified in accordance with applicable local regulations.

TESTS AVAILABLE FROM LILLY DESIGNATED CENTRAL LABORATORY	
Hepatic Clinical Chemistry Panel	Hepatitis A virus (HAV) testing:
Total bilirubin	HAV total antibody
Direct bilirubin	HAV IgM antibody
Alkaline phosphatase (ALP)	Hepatitis B virus (HBV) testing:
Alanine aminotransferase (ALT)	Hepatitis B surface antigen (HBsAg)
Aspartate aminotransferase (AST)	Hepatitis B surface antibody (anti-HBs)
Gamma-glutamyl transferase (GGT)	Hepatitis B core total antibody (anti-HBc)
Creatine kinase (CK)	Hepatitis B core IgM antibody
Hepatic Coagulation Panel	HBV DNA
Prothrombin time, INR (PT-INR)	Hepatitis C virus (HCV) testing:
Hepatic Hematology Panel	HCV antibody (total or IgG)
Hemoglobin	HCV RNA
Hematocrit	Hepatitis E virus (HEV) testing:
Erythrocytes (RBCs - red blood cells)	HEV IgG antibody
Leukocytes (WBCs - white blood cells)	HEV IgM antibody
Differential:	HEV RNA
Neutrophils	Anti-nuclear antibody (ANA)
Lymphocytes	Anti-smooth muscle antibody (ASMA) or anti-actin antibody
Monocytes	Haptoglobin
Basophils	Immunoglobulin IgA, IgG, IgM (quantitative)
Eosinophils	Urine Chemistry
Platelets	Drug Screen
Cell morphology (RBC and WBC)	

TESTS TO BE PERFORMED BY INVESTIGATOR-DESIGNATED LABORATORY, AS NEEDED	
Acetaminophen	Hepatitis D virus (HDV) testing:
Acetaminophen protein adducts	HDV total antibody
Alkaline phosphatase isoenzymes	HDV IgM antibody
Ceruloplasmin	HDV RNA
Copper	Herpes simplex virus (HSV) testing:
Cytomegalovirus (CMV) testing:	HSV (Type 1 and 2) antibody
CMV antibody	HSV (Type 1 and 2) DNA
CMV DNA	Iron
Ethyl alcohol (ethanol, EtOH)	Liver kidney microsomal type 1 (LKM-1) antibody
Ethyl glucuronide (EtG)	Phosphatidylethanol (PEth)
Epstein Barr virus (EBV) testing:	Microbiology Culture:
EBV antibody	Blood
EBV DNA	Urine
Ferritin	Total Iron Binding Capacity or Transferrin Saturation

10.7. Appendix 7: Measurement of Height, Weight, Waist Circumference, Vital Signs, and Calculation of BMI

The following information has been adapted from standardized physical measurement protocols for the World Health Organization's STEPwise approach to Surveillance (STEPS) (WHO 2017).

Height

Step 1. Ask the participant to remove their footwear and any headgear (light headgear worn for religious reasons can remain, but this should be worn by the participant at every clinic visit when their height is measured).

Step 2. Ask the participant to stand on the calibrated height measuring board (stadiometer) or against a wall with their feet together and their knees straight with their heels against the backboard, the stadiometer, or the wall.

Step 3. Ask the participant to look straight ahead without tilting their head up.

Step 4. Ask the participant to breathe in and stand tall. Measure and record the participant's height in **centimeters to 1 decimal place**.

Weight

- Body weight measurements should be done in a consistent manner using a calibrated electronic scale capable of measuring weight in kilograms to 1 decimal place.
- All weights for a given participant should be measured using the same scale, whenever possible, at approximately the same time in the morning after evacuation of bladder contents.
- Body weight will be measured in fasting state at all visits. If the participant is not fasting, the participant should be called in for a new visit within the visit window to have the fasting body weight measured.

Step 1. Ask the participant to empty their pockets, remove their footwear, outerwear (coat, jacket, and so forth), and any headgear (light headgear worn for religious reasons can remain, but this should be worn by the participant at every clinic visit when weight is measured).

Step 2. Make sure the scale is placed on a firm, flat, even surface (not on carpet, on a sloping surface, or a rough, uneven surface).

Step 3. Ask the participant to step onto the scale with 1 foot on each side of the scale.

Step 4. Ask the participant to stand still with arms by sides and then record weight in kilograms to the nearest one-tenth kilogram.

Waist Circumference

- Waist circumference should be measured in centimeters in the horizontal plane and at the midpoint between the lower margin of the last palpable rib and the top of the iliac crest.
- Measurements must be taken at the end of a normal expiration using a non-stretchable measuring tape provided. The tape should lie flat against the skin without compressing the soft tissue.
- The waist circumference should be measured twice, rounded to the nearest 0.5 cm. The measuring tape should be removed between the 2 measurements. Both measurements will be recorded in the CRF. If the difference between the 2 measurements exceeds 1 cm, this set of measurements should be discarded and the 2 measurements repeated.

Step 1. Ask the participant to wear light clothing (if available, patient gowns could also be used).

Step 2. Ask the participant to stand with their feet close together, arms at their side, body weight evenly distributed.

Step 3. Ask the participant to relax and measure the participant's waist circumference.

Guidance for vital sign measurements of blood pressure and heart rate

- measure vital signs before obtaining an ECG tracing and before collection of blood samples for laboratory testing
- have the participant sit quietly for about 15 minutes before vital signs measurements are taken
- for each parameter, take 3 measurements from the same arm, preferably the nondominant arm
- measure the recordings at least 3 minutes apart
- blood pressure must be taken with an automated BP instrument with the appropriate size cuff on the upper arm; Each measurement of sitting pulse and BP needs to be recorded in the CRF.
- heart rate is measured by pulse

Note: In the event pulse measurement cannot be taken via an automated BP instrument, the preferred location for measurement of pulse is the radial artery, and if BP and pulse measurements are taken separately, pulse should be taken prior to BP.

Any clinically significant findings from vital signs measurement that result in a diagnosis and occur during the study should be reported to the Sponsor as an AE via CRF.

Calculating BMI

Round to one decimal place. For example, a BMI of 26.6 kg/m² should **not** be rounded to 27.0 kg/m².

10.8. Appendix 8: Select Medications Associated with Weight Gain

<i>Antidepressants</i>	<i>Antihistamines</i>
Nortriptyline	Diphenhydramine
Doxepin	Cetirizine
Amitriptyline	Hydroxyzine
imipramine	Fexofenadine
Phenelzine	Meclizine
Paroxetine	Cyproheptadine
Escitalopram	<i>Antipsychotics</i>
Citalopram	Clozapine
Fluoxetine (>1 year)	Olanzapine
Sertraline (>1 year)	Risperidone
Mirtazapine	Quetiapine
<i>Antihypertensives</i>	Perphenazine
Prazosin	Aripiprazole
Doxazosin	<i>Mood Stabilizers</i>
Terazosin	Lithium
Metoprolol tartrate	<i>Steroids</i>
Propranolol	Glucocorticoids
Atenolol	Progestins
<i>Antidiabetics</i>	Corticosteroids (such as prednisone)
Insulin	<i>Contraceptives and hormones</i>
Sulfonylureas	Depo-medroxyprogesterone acetate
Thiazolidinediones	Megestrol acetate
<i>Anti-epileptics</i>	
Gabapentin	
Pregabalin	
Valproic acid	
Vigabatrin	
Carbamazepine	

Note: Adapted from Apovian et al. 2025.

10.9. Appendix 9: Continuous Blood Glucose Monitoring

Rationale for Substudy

The purpose of the CGM substudy of Protocol YDAF is to evaluate the 24-hour glucose profile captured with CGM for eloralintide versus placebo individuals with obesity and or overweight with T2D. The CGM substudy will allow for comparison of eloralintide to placebo for the percentage of time in the hyper- and hypoglycemia range, and to evaluate the glucose variability in response to therapy. The scope of this appendix includes a subset of the study population that agrees to participate in this protocol appendix.

The goal of long-acting injectable antidiabetic therapy is to maintain blood glucose levels within an acceptable range over a prolonged period. Ideally, both hyper- and hypoglycemia are avoided, as extended periods in these states are associated with significant morbidity. To confirm that the target range is achieved and maintained, monitoring is required. Monitoring can be performed through a variety of methods, each with its own advantages and disadvantages.

HbA1c reflects average glucose exposure over a period of months and is associated with risk of future diabetes complications. However, HbA1c does not discern variability of blood glucose levels or relay information on severity of hypoglycemia, both of which could translate to different risks of clinically relevant outcomes. Alternatively, frequent SMBG can provide some information on glucose levels throughout the day, but does not provide a detailed, complete picture of glucose exposure throughout the day and night.

The use of a CGM system provides a comprehensive look at glycemic control over an extended period of time. The percentage of time either within or outside of the target range may be a better indicator of overall glucose control. CGM may provide a more accurate evaluation of overall outcome in response to an intervention than that assessed by either HbA1c or SMBG.

Continuous Glucose Monitoring Procedure

The Continuous Glucose Monitoring System will be used in unblinded mode during the sub study. The CGM device will display the sensor glucose readings to the participant and investigator, and high and low glucose alerts will be available to the participant.

The CGM System will also include a study-provisioned mobile device that can remotely collect at-home CGM data. This mobile device will enable the near real-time transfer of study CGM data to a vendor-hosted web portal to facilitate glycemia monitoring and for assessing data capture compliance during each CGM session.

At Visit 2, trained medical site staff will insert the CGM sensor, and dispense the CGM System. Site staff must check expiration date on sensors prior to insertion. Patients will wear the CGM device for approximately 14 days consecutively, but no longer than 20 days prior to Visit 3. The CGM System will be returned at Visit 3. At Visits 10 and 16, the sensor will be inserted and the CGM System will be returned at Visits 13 and 19 respectively (See Section [10.9.1](#)).

Training and initiation of study CGM

Participants who fulfill eligibility criteria will be trained on the use of the Dexcom G7 System, sensor replacement and the requirements for CGM. All participants will be trained on the interpretation of CGM glucose values and alarm/alerts as applicable. Participants will be

unblinded to their CGM using the Dexcom G7 receiver. The unblinded CGM data will be available to the investigator via a web portal for monitoring glycemia.

All participants will be required to wear a study provided unblinded CGM (Dexcom G7 CGM system) at 3 time points.

Site personnel will dispense Dexcom G7 System, supplies, and initiate CGM as specified in the Appendix SoA (Section 10.9.1). Site staff will ensure that the participant can change the sensor at home as directed.

A Participant User Guide that provides an overview of Dexcom G7 System components and study specific instructions will be available for at-home use. Participants are not permitted to connect the sensor of the study-provided CGM system to a personal device, mobile application, or any other system besides the study-provided mobile device and G7 receiver.

CGM data compliance

Sites will be required to review data capture compliance following CGM data downloads using the available vendor portal reports and visualization tools.

A data compliance threshold of 70% will be used and is defined as the percentage of actual data versus expected data collected during the entirety of the wear session. Site personnel will re-educate participants on the CGM System operation and requirements when compliance is <70%.

The patient will be instructed to monitor fasting blood glucose (by finger stick) per Study YDAF protocol.

Substudy Population

The total sample size of the CGM substudy will be 203, with approximately 45 participants in the placebo, eloralintide 3 mg, 6 mg, and 9 mg arms, and approximately 23 participants in the eloralintide 1.5 mg arm. Participants are eligible to be included in the substudy only if all of the following criteria apply:

Inclusion criteria

51. Enrolled in Study YDAF.
52. Must be willing to use the CGM system, supplied for this substudy, as directed, for approximately a 14-day period on 3 occasions.
53. Must have a normal wake/sleep cycle such that midnight to 0600 hours will reliably reflect a usual sleeping period.
54. Willing to complete all procedures included in this substudy, in addition to those included in the Study YDAF, and provide written informed consent for participation in this substudy.

Exclusion criteria

- 55. Have any condition or circumstance that, in the opinion of the investigator, would interfere with the completion of required CGM procedures.
- 56. Are unwilling to stop wearing a personal CGM during the 3 timepoints specified in this appendix (see Section [10.9.1](#)).

Schedule of Activities for CGM Substudy

In this substudy, participants will follow the main protocol SoA (Section [1.3](#)) along with the added visit details specified in Section [10.9.1](#).

10.9.1. Protocol J3R-MC-YDAF CGM Substudy Schedule of Activities

	PI		PII	PIII					Comments
Visit Number	1	2	3	10	13	16	19	ED	
Weeks from randomization	-5	-2	0	28	40	52	64		
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	3		
Visit Detail	F		F	F	F	F	F	F	F = fasting
Consent and demographics									
Informed consent	X								ICF must be signed before any protocol specific tests/procedures are performed.
Inclusion/exclusion criteria; confirmation of eligibility		X							
Continuous Glucose Monitoring									
Study CGM Device Training		X							
Dispense CGM system		X		X		X			Dispense supplies as needed. The System will include the vendor-provided mobile device for remote CGM data capture.

	PI		PII	PIII					Comments
Visit Number	1	2	3	10	13	16	19	ED	
Weeks from randomization	-5	-2	0	28	40	52	64		
Visit Interval Tolerance (+/-days)	-	3	3	3	3	3	3		
Visit Detail	F		F	F	F	F	F	F	F = fasting
CGM sensor insertion and session start		X		X		X			The Study CGM must be worn for a minimum of 14 consecutive days during each session. See Section 10.8 .
CGM system return			X		X		X	X	If participant is ineligible during Visit 2, the site can use an unscheduled visit to return the CGM system prior to Visit 3. If the ED visit occurs during a CGM session, the device must be returned at that visit.

Abbreviations: CGM = continuous glucose monitoring; ED = early discontinuation; ICF = informed consent form; PI = Period I; PII = Period II; PIII = Period III; T2D = type 2 diabetes.

Objectives, Endpoints, and Estimands

Objectives	Endpoints
Secondary	
To demonstrate that pooled eloralintide (1.5 mg, 3 mg, 6 mg, 9 mg) QW is superior to placebo for: change in glycemic parameters from baseline to 28 and 52 weeks	Change in - mean glucose (mg/dL and mmol/L) - time in range (TIR) between 3.9-10.0 mmol/L (70-180 mg/dL) - glycemic variability (%CV) target <36%.

Abbreviations: %CV = coefficient of variation; QW = weekly; TIR = time in range.

10.10. Appendix 10: Region- or Country-specific Requirements

10.10.1. European Union

The study will be conducted in accordance with the Regulation (EU) No 536/2014

Regulatory reporting requirements

The Sponsor has processes for safety reports for identification, recording, and expedited reporting of SUSARs according to EU regulatory requirements. The Sponsor will comply with applicable regulatory requirements relating to safety reporting to the regulatory authority, per European Union Clinical Trial Regulation 536/2014 submission of SUSARs to the EudraVigilance database.

10.10.1.1. Germany

This section describes protocol changes applicable for adult participants in study sites in Germany.

This table describes the changes and provides a rationale for the changes.

Protocol Section Number and Name	Description of the Change	Brief Rationale
7.2. Participant Discontinuation/Withdraw from the Study	Deleted references to “legally authorized representative,” “legal guardian,” “parents”	The German Drug Law (Arzneimittelgesetz – AMG) with reference to EU Regulation 536/2014 requires that adult participants act on their own behalf and provide their own written informed consent. If written consent is not possible, verbal consent with a witness is acceptable. No legal representative consent is accepted.
8.3. Adverse Events, Serious Adverse Events, and Product Complaints		
10.1.3. Informed Consent Process		

The revised text in the following sections show the changes applicable for adult participants at study sites in Germany. Deletions are identified by strikethrough format.

7.2. Participant Discontinuation/Withdrawal from the Study

A participant may withdraw from the study

- at any time at the participant’s own request for any reason or without providing any reason
- ~~at the request of the participant’s designee, for example, parents or legal guardian~~
- at the discretion of the investigator for safety, behavioral, compliance, or administrative reasons,

- if enrolled in any other clinical study involving an investigational product or enrolled in any other type of medical research judged not to be scientifically or medically compatible with this study.

8.3. Adverse Events, Serious Adverse Events, and Product Complaints

The definitions of the following events can be found in Section 10.3:

- Adverse events (AEs)
- Serious adverse events (SAEs)
- Product complaints (PCs)

These events will be reported by the participant or, when appropriate, by a caregiver, or surrogate, ~~or the participant's legally authorized representative.~~

10.1.3. Informed Consent Process

The investigator or the investigator's representative will explain the nature of the study, including the risks and benefits, to the potential participant ~~or the potential participant's legally authorized representative~~, and answer all questions regarding the study.

Potential participants must be informed that their participation is voluntary. Participants ~~or their legally authorized representatives~~ will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH guidelines, privacy and data protection requirements, where applicable, and the IRB/IEC or study center.

The medical record must include a statement that written informed consent was obtained before the participant was entered in the study and the date the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICF.

Revised consents must be appropriately obtained using the correct approved ICFs for applicable study participants in accordance with Sponsor and ERB consenting guidance.

A copy of the ICF(s) must be provided to the participant ~~or the participant's legally authorized representative~~ and is kept on file.

10.12. Appendix 12: Provisions for Changes in Study Conduct During Exceptional Circumstances

In Germany, the temporary measures described in the "Provisions for Changes in Study Conduct During Exceptional Circumstances" (Section 10.12) are applicable only to "the COVID-19 pandemic.

10.13. Appendix 13: Abbreviations and Definitions

enter	Participants entered into a study are those who sign the informed consent form directly or through their legally acceptable representatives.
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10.10.2. India

This section describes protocol changes applicable for participants in study sites in India.

This table describes the changes and provides a rationale for the changes.

Protocol Section Number and Name	Description of the Change	Brief Rationale
8.3.1. Timing and Mechanism for Collecting Events	Revised footnote in timing and mechanism for collecting events table	ALL SAEs irrespective of the cause or causality, need to be reported to Indian Ministry of Health within 14 calendar days by the Sponsor.

The revised text in the following section shows the changes applicable only to participants in India.

All additions have been identified by the use of underscore, and all deletions have been identified by strikethroughs.

8.3.1 Timing and Mechanism for Collecting Events

This table describes the timing, deadlines, and mechanism for collecting events.

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
Adverse Events					
AEs including SAEs	Signing of the ICF	Participation in study has ended	As soon as possible upon site awareness	AE CRF	N/A
<i>Additional Reporting for Serious Adverse Events</i>					
SAE and SAE updates – prior to start of study intervention and deemed reasonably possibly related to study procedures	Signing of the ICF	Start of study intervention	Within 24 hours of awareness	SAE CRF	SAE paper form
SAE and SAE updates – after start of study intervention	Start of study intervention	Participation in study has ended	Within 24 hours of awareness	SAE CRF	SAE paper form

Event	Collection Start	Collection Stop	Timing for Reporting to Sponsor or Designee	Mechanism for Reporting	Back-up Method of Reporting
SAE† – after participant's study participation has ended and the investigator becomes aware	After participant's study participation has ended	N/A	Promptly	SAE paper form	N/A
Pregnancy					
Pregnancy in participants and partners of participants	After the start of study intervention	70 days after last dose of study intervention	Within 24 hours (see Section 8.3.2)	Pregnancy CRF	Pregnancy paper form
Product Complaints					
PC associated with an SAE or might have led to an SAE	Start of study intervention	End of study intervention	Within 24 hours of awareness	Product Complaint form	N/A
PC not associated with an SAE	Start of study intervention	End of study intervention	Within 1 business day of awareness	Product Complaint form	N/A
Updated PC information	—	—	As soon as possible upon site awareness	Originally completed Product Complaint form with all changes signed and dated by the investigator	N/A
PC (if investigator becomes aware)	Participation in study has ended	N/A	Promptly	Product Complaint form	

Abbreviations: AE = adverse event; CRF = case report form; ICF = informed consent form; N/A = not applicable; PC = product complaint; SAE = serious adverse event.

† Serious adverse events should ~~not~~ be reported ~~unless the investigator deems them to be possibly related to study intervention or study participation.~~

10.11. Appendix 11: Objectives, Endpoints, Estimands and Statistical Considerations for Japan

The primary objective for the Japan submission is consistent with that described in Section 3 of the main body.

The key secondary objectives specific to the Japan submission are summarized in the table below.

Objectives	Endpoints
Key Secondary	
To demonstrate that eloralintide 1.5 mg, 3 mg, 6 mg and 9 mg QW is superior to placebo for change in: body weight	From baseline to Week 64 - percent change in body weight in the population meeting JASSO criteria - the achievement (Y/N) of body weight reduction of $\geq 5\%$ in the population meeting JASSO criteria.

Abbreviations: JASSO = Japan Society for the Study of Obesity; QW = once weekly; Y/N = yes/no.

Japan-specific secondary endpoints are also presented in the table below.

Objectives	Endpoints
Secondary	
To demonstrate that eloralintide 1.5 mg, 3 mg, 6 mg, 9 mg QW and pooled dose is superior to placebo in change from baseline for the following: improvement of comorbidities	From baseline to Week 64 - the achievement (Y/N) of improvement in hypertension (only for participants with hypertension at baseline) - the achievement (Y/N) of improvement in dyslipidemia (only for participants with dyslipidemia at baseline) - change in anti-hypertensive medication (decrease, no change, increase) (only hypertension at baseline) - Change in lipid-lowering medication (decrease, no change, increase) (only dyslipidemia at baseline) - the achievement (Y/N) of 0, 1, 2 or 3 improvements of obesity-related health problems (only for participants with 3 obesity-related health problems) 0, 1, or 2 improvements of obesity-related health problems (only for participants with 2 obesity-related problems) 0, or 1 improvement of obesity-related health problems (only for participants with 1 obesity-related problem) (obesity-related health problems here are T2D, hypertension and dyslipidemia).

Abbreviations: QW = once weekly; T2D = type 2 diabetes; Y/N = yes/no.

10.11.1. Statistical Considerations

The following outlines the differences from the content presented in Section 9.

10.11.1.1. Statistical Hypotheses

The null hypotheses corresponding to the primary objective are same as in Section 9. The null hypotheses corresponding to the key secondary objectives are as follows:

- $H_{1.5,1,0,J}$, $H_{3,1,0,J}$, $H_{6,1,0,J}$, $H_{9,1,0,J}$: No difference in 1.5mg, 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the mean percent change in body weight from baseline at Week 64 in JASSO population.
- $H_{1.5,2,0,J}$, $H_{3,2,0,J}$, $H_{6,2,0,J}$, $H_{9,2,0,J}$: No difference in 1.5 mg, 3 mg, 6 mg, 9 mg QW eloralintide compared to placebo with respect to the percentage of participants achieving body weight reduction of 5% from baseline at Week 64 in JASSO population.

10.11.1.2. Analysis Set

For the Japan-specific analysis, in addition to the definitions provided in Section 9.4, the criteria for the JASSO subpopulation will be incorporated. Further details will be described in the SAP.

10.11.1.3. Statistical Analysis

In principle, any analysis will be conducted using the same methodology as described in the main body of the protocol, based on the attributes of each variable. Further details will be provided in SAP.

10.11.1.4. Sample Size Determination

It is assumed that at least 90% of the overall study population will meet the JASSO subpopulation criteria. Given the strong correlation between the percentage of participants achieving a $\geq 5\%$ reduction in body weight and the percent change in body weight, the former will be excluded from the power calculation. All other assumptions will be consistent with those described in Section 9.7. Under these conditions, the statistical power to detect a difference between the 1.5 mg, 3 mg, 6 mg, and/or 9 mg dose and placebo is expected to be at least 90%.

10.11.2. JASSO Subpopulation Criteria

The JASSO subpopulation criteria will be defined according to the criteria of both BMI- and obesity-related health disorders according to the obesity disease label in Japan. Specifically, it is defined as follows:

- 1) baseline BMI between ≥ 35 kg/m², with either hypertension or dyslipidemia or
- 2) baseline BMI between 27 to < 35 kg/m², with at least 2 obesity-related health disorders, of which at least one must be either hypertension or dyslipidemia, where the obesity-related health disorders are defined according to obesity disease treatment guideline (JASSO 2022).

Obesity-related health disorders (out of 11 excluding T2D):

- glucose intolerance disorder (impaired glucose tolerance)
- hyperuricemia and gout
- cardiovascular disease, myocardial infarction, and angina
- cerebral infarction and transient ischemic attack
- metabolic dysfunction-associated steatotic liver disease
- menstruation disorder and infertility
- obstructive sleep apnea syndrome and obesity-hypoventilation syndrome
- motor dysfunction: arthritis/osteoarthritis (knee, hip joint, supine, and so on)
- obesity-related renal disease
- hypertension, and
- dyslipidemia.

Eleven obesity-related health disorders

The JASSO treatment guideline (JASSO 2022) defines 11 health disorders for the diagnosis of “obesity disease” in participants who need weight reduction for medical reasons.

10.12. Appendix 12: Provisions for Changes in Study Conduct During Exceptional Circumstances

Implementation of this appendix

The changes to procedures described in this appendix are temporary measures intended to be used only during specific time periods as directed by the Sponsor in partnership with the investigator.

In an exceptional circumstance, the Sponsor's procedures ensure that relevant parties are kept informed of changes to the design and conduct of the study.

Exceptional circumstances

Exceptional circumstances are rare events that may cause disruptions to the conduct of the study. Examples include pandemics or natural disasters. These disruptions may limit the ability of the investigators, participants, or both to attend on-site visits or to conduct planned study procedures.

Implementing changes under exceptional circumstances

In an exceptional circumstance, after receiving the Sponsor's written approval, sites may implement changes if permitted by local regulations.

After approval by local Ethical Review Boards, regulatory bodies and any other relevant local authorities, implementation of these exceptional circumstance changes will not typically require additional notification to these groups, unless they have specific requirements in which notification is required (for example, upon implementation and suspension of changes). All approvals and notifications must be retained in the study records.

If the Sponsor grants written approval for changes in study conduct, the Sponsor will also provide additional written guidance, if needed.

Considerations for making a change

The prevailing consideration for making a change is ensuring the safety of study participants. Additional important considerations for making a change are compliance with Good Clinical Practice, enabling participants to continue safely in the study and maintaining the integrity of the study.

Informed consent

Additional consent from the participant will be obtained, if required, for

- a change in the method of study intervention administration
- dispensation of additional study intervention during an extended treatment period
- alternate delivery of study intervention and ancillary supplies, and
- provision of their personal or medical information required prior to implementation of these activities.

Changes in study conduct during exceptional circumstances

Changes in study conduct not described in this appendix, or not consistent with applicable local regulations, are not allowed.

The following changes in study conduct will not be considered protocol deviations.

Remote visits***Types of remote visits***

Telemedicine - Telephone or technology-assisted virtual visits, or both, are acceptable to complete appropriate assessments. Assessments to be completed in this manner include, but are not limited to, concomitant medications, AEs, C-SSRS since last assessed, lifestyle counseling, and diary review.

Other alternative locations – Local laboratories may be used to collect laboratory samples only for safety assessments.

Data capture

In source documents and the CRF, the study site should capture the visit method, with a specific explanation for any data missing because of missed in-person site visits.

Safety reporting

Regardless of the type of remote visits implemented, the protocol requirements regarding the reporting of AEs, SAEs, and PCs remain unchanged.

Return to on-site visits

Every effort should be made to enable participants to return to on-site visits as soon as reasonably possible, while ensuring the safety of both the participants and the site staff.

Local laboratory testing option

Local laboratory testing may be conducted in lieu of central laboratory testing. However, central laboratory testing must be retained for: Visits 1, 3, 10, 13, 19, ED, 801, and PK visits. The local laboratory must be qualified in accordance with applicable local regulations.

Study intervention and ancillary supplies (including participant diaries)

When a participant is unable to go to the site to receive study supplies during normal on-site visits, the site should work with the Sponsor to determine appropriate actions. These actions may include

- asking the participant to go to the site and receive study supplies from site staff without completion of a full study visit
- asking the participant's designee to go to the site and receive study supplies on a participant's behalf, and
- arranging delivery of study supplies.

These requirements must be met before action is taken:

- Alternate delivery of study intervention should be performed in a manner that does not compromise treatment blinding and ensures product integrity. The existing protocol requirements for product accountability remain unchanged, including verification of participant's receipt of study supplies.

- When delivering supplies to a location other than the study site (for example, participant's home), the investigator, Sponsor, or both should ensure oversight of the shipping process to ensure accountability and product quality (that is, storage conditions maintained and intact packaging upon receipt).
- Instructions may be provided to the participant or designee on the final disposition of any unused or completed study supplies.

Screening period guidance

To ensure safety of study participants, laboratory values and other eligibility assessments taken at screening visits (Visits 1 and 2) are valid for a maximum of 60 days, except for serum pregnancy tests which are only valid for a maximum of 30 days.

The following rules will be applied for active, nonrandomized participants whose participation in the study must be interrupted due to exceptional circumstances.

- If screening is interrupted for less than 60 days from screening visits to Visit 3, the participant will proceed to the next study visit per the usual SoA, provided that Visit 3 must be conducted within 60 days from first screening visit (Visit 1).
 - The site should conduct the next visit if the participant's eligibility criteria are confirmed, and the site should document the reason for delay.
 - The site should also reconfirm the impacted participant's consent and document this confirmation in the source documentation.
- If screening is interrupted for more than 60 days from screening visits to Visit 3, the participant must be discontinued because of screening interruption due to an exceptional circumstance. This is documented as a screen failure in the CRF. The participant can reconsent and be rescreened as a new participant. The screening procedures per the usual Schedule of Activities should be followed, starting at Visit 1 to ensure participant eligibility by Visit 3.

Adjustments to visit windows

Whenever possible and safe to do so, as determined by the investigator's discretion, participants should complete the usual Schedule of Activities. To maximize the possibility that these visits can be conducted as on-site visits, the windows for visits may be adjusted, upon further guidance from the Sponsor. This minimizes missing data and preserves the intended conduct of the study.

This table describes the allowed adjustments to visit windows.

Visit Number	Tolerance
Visit 3 (Randomization)	Within 60 days after V1
Visit 10 DXA	Within 30 days before or after the intended date
Week 28 PK only Visit	No adjustment to the visit window described in SoA
Week 40 PK only visit	No adjustment to the visit window described in SoA
Visit 19 DXA	Within 30 days before or after the intended date
Visits 4 to 19	Within 7 days before or after the intended date
Visit 801	Within 14 days before or after the intended date

Abbreviations: DXA = dual-energy X-ray absorptiometry; PK = pharmacokinetics; SoA = schedule of activities; V = visit.

For participants whose visits have extended windows, additional study intervention may need to be provided to avoid interruption and maintain overall integrity of the study.

Documentation

Changes to study conduct will be documented

Sites will identify and document the details of how participants, visit types, and conducted activities were affected by exceptional circumstances. Dispensing/shipment records of study intervention and relevant communications, including delegation, should be filed with site study records.

Source documents at alternate locations

Source documents generated at a location other than the study site should be part of the investigator's source documentation and should be transferred to the site in a secure and timely manner.

10.13. Appendix 13: Abbreviations and Definitions

Term	Definition
abuse	use of a study intervention for recreational purposes or to maintain an addiction or dependence
ADA	anti-drug antibodies
AE	adverse event
AFAB	assigned female at birth
AHM	antihyperglycemic medication
ALP	alkaline phosphatase
ALT	alanine aminotransferase
AMAB	assigned male at birth
ANCOVA	analysis of covariance
AST	aspartate aminotransferase
AV	Atrioventricular
blinding/masking	A single-blind study is one in which the investigator and/or the investigator's staff are aware of the treatment but the participant is not, or vice versa, or when the sponsor is aware of the treatment but the investigator and/the investigator's staff and the participant are not. A double-blind study is one in which neither the participant nor any of the investigator or sponsor staff who are involved in the treatment or clinical evaluation of the subjects are aware of the treatment received.
BMI	body mass index
BP	blood pressure
CEC	clinical endpoint committee
CFR	Code of Federal Regulations
CGM	continuous glucose monitoring
CI	confidence interval
C-SSRS	Columbia suicide severity rating scale
complaint	A complaint is any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, purity, durability, reliability, safety or effectiveness, or performance of a drug or drug delivery system.
compliance	Adherence to all study-related, good clinical practice (GCP), and applicable regulatory requirements.

CONSORT	Consolidated Standards of Reporting Trials
CoEQ	control of eating questionnaire
CRF	case report form; a printed, optical, or electronic document designed to record all of the protocol-required information to be reported to the sponsor for each trial participant.
CRP	clinical research physician: Individual responsible for the medical conduct of the study. Responsibilities of the CRP may be performed by a physician, clinical research scientist, clinical pharmacologist, global safety physician or other medical officer.
CT	computed tomography
CV	Cardiovascular
DMC	data monitoring committee. A data monitoring committee, or data monitoring board (DMB) is a group of independent scientists who are appointed to monitor the safety and scientific integrity of a human research intervention, and to make recommendations to the sponsor regarding the stopping of a study for efficacy, or for harm, or for futility. The composition of the committee is dependent upon the scientific skills and knowledge required for monitoring the particular study.
DPP-4i	dipeptidyl peptidase-4 inhibitor
DXA	dual-energy X-ray absorptiometry
EASD	European Association for the Study of Diabetes
ECG	Electrocardiogram
eCOA	electronic clinical outcomes and assessments
ED	early discontinuation
eGFR	estimated glomerular filtration rate
enroll	The act of assigning a participant to a treatment. Participants who are enrolled in the study are those who have been assigned to a treatment.
enter	Participants entered into a study are those who sign the informed consent form directly or through their legally acceptable representatives.
EQ-5D-5L	European quality of life-5 dimensions-5 level
FAS	fatigue Assessment Scale
FNQ	food noise questionnaire
GCP	good clinical practice
GDPR	EU General Data Protection Regulation
GI	Gastrointestinal
GIP	glucose-dependent insulintropic polypeptide
GIP/GLP RA	glucose-dependent insulintropic polypeptide/glucagon-like peptide-1 receptor agonist

GLP-1	glucagon-like peptide-1
GLP-1 RA	glucagon-like peptide-1 receptor agonist
HbA1c	hemoglobin A1c
HDL	High-density lipoprotein
hsCRP	high sensitivity c-reactive protein
IB	investigator's brochure
ICE	intercurrent event
ICF	informed consent form
ICH	International Council for Harmonisation
IEC	independent ethics committee
IMP	investigational medicinal product (see also "investigational product") A medicinal product which is being tested or used as a reference, including as a placebo, in a clinical trial.
informed consent	A process by which a participant voluntarily confirms their willingness to participate in a particular study, after having been informed of all aspects of the study that are relevant to the participant's decision to participate. Informed consent is documented by means of a written, signed and dated informed consent form.
interim analysis	An interim analysis is an analysis of clinical study data, separated into treatment groups, that is conducted before the final reporting database is created/locked.
IP	investigational product
investigational product	A pharmaceutical form of an active ingredient or placebo being tested or used as a reference in a clinical trial, including products already on the market when used or assembled (formulated or packaged) in a way different from the authorized form, or marketed products used for an unauthorized indication, or marketed products used to gain further information about the authorized form. See also "IMP."
INOCBP	individual not of child bearing potential
IOCBP	individual of child-bearing potential
IRB	institutional review board
IWRS	interactive web-response system
JASSO	Japan Society for the Study of Obesity
LDL	low density lipoprotein

medication error	Errors in the prescribing, dispensing, or administration of a study intervention, regardless of whether or not the medication is administered to the participant or the error leads to an AE. Medication error generally involve a failure to uphold one or more of the five “rights” of medication use: 1. the right participant 2. the right drug 3. the right dose 4. the right route, 5. at the right time.
misuse	Use of a study intervention for self-treatment that either is inconsistent with the prescribed dosing regimen, indication, or both, or is obtained without a prescription
MMRM	mixed models for repeated measures
NIMP	non-investigational medicinal product See AxMP. A medicinal product used for the needs of a clinical trial as described in the protocol, but not as an investigational medicinal product. Examples include rescue medication, challenge agents, agents to assess endpoints in the clinical trial, or background treatment.
participant	Equivalent to CDISC term “subject”: an individual who participates in a clinical trial, either as recipient of an investigational medicinal product or as a control
PC	product complaint
PGIC	patient global impression of change
PGIS	patient global impression of severity
PK/PD	pharmacokinetics/pharmacodynamics
PRO/ePRO	patient-reported outcomes/electronic patient-reported outcomes
QTc	corrected QT interval
QW	once-weekly
SAE	serious adverse event
SAP	statistical analysis plan
SC	Subcutaneous
screen	The act of determining if an individual meets minimum requirements to become part of a pool of potential candidates for participation in a clinical study.
SGLT-2	sodium-glucose co-transporter 2
SMBG	self-monitoring of blood glucose
SoA	schedule of activities
SU	sulfonylurea

SUSAR	suspected unexpected serious adverse reactions Refers to an adverse event that occurs in a clinical trial participant, which is assessed by the sponsor and or study investigator as being unexpected, serious and as having a reasonable possibility of a causal relationship with the study intervention.
T2D	type 2 diabetes
TBL	Total bilirubin
TE-ADA	treatment emergent anti-drug antibody
TEAE	Treatment-emergent adverse event: An untoward medical occurrence that emerges during a defined treatment period, having been absent pretreatment, or worsens relative to the pretreatment state, and does not necessarily have to have a causal relationship with this treatment.
ULN	upper limit of normal
UV	Unscheduled visit
VLDL	Very low density lipoprotein
WHtR	Waist to height ratio

11. References

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